How to Pay for Assisted Living in Illinois Without Going Broke

A Family Guide to Medicaid Spend-Down, the Five-Year Look-Back, VA Aid & Attendance, Spousal Protections, and Illinois' Supportive Living Program

Larisa Huhman

First Edition

Research reviewed through July 2026

COPYRIGHT

Copyright © 2026 Larisa Huhman

All rights reserved. No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the copyright holder, except for brief quotations used in reviews or as otherwise permitted by law.

This is an independently published consumer-education book.

Fictional and composite examples. The people, families, names, conversations, dollar amounts, and circumstances used in case examples are fictional or are composites created for educational purposes. They do not describe any one actual person or family. Any resemblance to an actual person, living or deceased, is coincidental. The examples are simplified to teach general principles and should not be used as predictions of an individual outcome.

Program rules, benefit amounts, forms, phone numbers, websites, and agency procedures can change. Readers should confirm current information with the responsible government agency and appropriately qualified professionals before acting.

Paperback ISBN: To be assigned

E-book ISBN: To be assigned

Cover and interior design credits: To be added

CONTACT INFORMATION

Website

chicagocareforseniors.com

Email

info@chicagocareforsenios.com
larisahuhman@gmail.com

IMPORTANT DISCLAIMER

Please read this section before using the book.

This book provides general educational information. It is not legal, financial, tax, insurance, investment, benefits, case-management, or medical advice, and it is not a substitute for individualized professional advice.

This book does not provide medical advice and no medical relationship is created. Reading this book, contacting the author, downloading a worksheet, or following a general example does not create a doctor-patient relationship or any other professional relationship. Nothing in this book diagnoses an illness, evaluates a person's ability to live safely in a particular setting, determines capacity, prescribes treatment, recommends a medication, or decides what level of care is medically appropriate.

Make medical and care-setting decisions with the older adult's licensed physicians and qualified health professionals. Consult them about symptoms, medications, mobility, cognition, behavior, rehabilitation, and the safety of a proposed setting. Do not disregard or delay medical advice because of this book. For an emergency, call 911.

This book does not create an attorney-client relationship. Medicaid, estate recovery, transfers, contracts, property, taxes, veterans' benefits, and spouse protections can have irreversible consequences. Before transferring, selling, retitling, or spending significant assets—or signing an admission agreement—consult an Illinois elder-law attorney. Use appropriately licensed tax and financial professionals.

For VA benefits, consult the U.S. Department of Veterans Affairs and a VA-accredited representative. Wartime service or a need for personal help alone does not establish eligibility.

Eligibility for Medicaid, the Illinois Supportive Living Program, VA benefits, insurance payments, admission to a residence, and continued residence depends on individual facts and the rules in effect when a decision is made. No result, approval, payment amount, savings amount, or length of stay is promised or guaranteed.

Reasonable efforts were made to use reliable sources, but laws, rules, forms, and practices change. References are provided for convenience and do not endorse any agency, professional, facility, service, or product.

Use this book as a map for asking better questions—not as permission to take an irreversible step without individualized advice.

HOW TO USE THIS BOOK

You do not have to read this book from beginning to end before taking action.

If your parent must leave a hospital, rehabilitation center, or unsafe home within the next few days, begin with the introduction and the **Act Now** boxes. Then complete the one-page financial snapshot in Chapter 4 and read the chapters that match your parent's situation.

If the family has several months, read Parts I and II before discussing Medicaid planning. Understanding the care setting and the family's financial runway will prevent many expensive mistakes.

If one spouse needs care while the other remains at home, do not start with the single-person examples. Go directly to the chapters on spousal impoverishment. A married couple's rules can be very different.

If your parent or a deceased spouse served in the military, preserve service records and read the VA Pension with Aid & Attendance chapters. Aid & Attendance is not a freestanding benefit and should not be treated as one.

Throughout the book, watch for these signposts:

ACT NOW — a step for families facing an immediate placement decision.

ILLINOIS RULE — information specific to Illinois.

CHANGING NUMBER — an amount or limit that must be verified for the current year.

DO NOT DO THIS YET — an action that may be difficult to reverse.

PAPER TRAIL — documents to obtain and preserve.

ASK A PROFESSIONAL — a point where individualized advice is especially important.

At the end of each chapter, the **Bottom Line** reduces the material to the decisions that matter most.

THE BOOK IN SEVEN PARTS

The parts follow the order in which families make safer decisions: define the problem, build a payment bridge, understand public programs, and turn the information into a family plan.

PART I — UNDERSTAND THE PROBLEM BEFORE SPENDING MONEY
Care settings, real prices, and the family financial snapshot.

PART II — BUILD THE PRIVATE-PAY BRIDGE
Financial runway, income, the home, insurance, family funding, and cost control.

PART III — ILLINOIS MEDICAID WITHOUT MYTHS
Eligibility pathways, assets, lawful spend-down, the look-back, applications, and estate recovery.

PART IV — PROTECTING THE SPOUSE AT HOME
Spousal protections, planning timelines, and Illinois couple case studies.

PART V — VA PENSION WITH AID & ATTENDANCE
Eligibility, safe applications, and coordination with Medicaid.

PART VI — THE ILLINOIS SUPPORTIVE LIVING PROGRAM
What SLP is, how to compare communities, and how transitions work.

PART VII — CREATE THE FAMILY PLAN
Thirty-day and long-range action plans, roles, meetings, and decision safeguards.

INTRODUCTION

Monday Is Coming

At 4:17 on Friday afternoon, Elena learned that her mother was being discharged on Monday.

“She can’t go home,” Elena said.

The discharge planner turned the computer screen slightly away and folded her hands. She had kind eyes and the exhausted calm of someone who had delivered bad news all week.

“Then we need another setting.”

Another setting. The phrase sounded as simple as changing hotel rooms.

Maria’s fall had not caused a serious injury, but it exposed what the family avoided naming. She had missed medication, kept expired food, and wandered the hallway at two in the morning looking for an elevator the building did not have.

Six weeks earlier, a neurologist had used the words *early-stage dementia*. Maria called it “a little forgetfulness” and accused Elena of turning one bad week into a federal investigation.

“Medicare is paying for rehabilitation,” Elena said. “Can’t she stay a little longer?”

“The therapy team believes she no longer needs this level of skilled rehabilitation.”

“But she still needs help.”

“Yes.”

It was the most important *yes* Elena had heard all day, and the least useful.

The planner slid over an alphabetical list of twelve communities. It had no prices and no explanation of which managed dementia or what *accepted Medicaid* meant.

Elena looked at the clock. The banks would close soon. The elder-law offices would close soon. Her brother Mark lived in California and was not answering.

And Monday was coming.

The first family argument

Mark finally called while Elena was standing beside a vending machine, eating crackers that had survived longer than several hospital administrators.

“You’re overreacting,” he said. “Mom sounded fine yesterday.”

“You spoke to her for seven minutes.”

“She knew who I was.”

“She also thinks she is going home to the house she sold eighteen years ago.”

There was a silence.

“Don’t sign anything,” Mark said.

This was Mark’s specialty: issuing excellent warnings from two thousand miles away.



Excellent warnings were Mark’s specialty.

“Then what should I do?”

“Can’t Medicare pay for assisted living?”

Elena closed her eyes.

By five o’clock, two loving children had confused Medicare with long-term-care coverage, treated assisted living as one uniform service, and begun arguing before either knew the price.

This book begins where Elena begins: without enough time, without a complete file, and without the luxury of learning everything before making the next decision.

It will not turn a family member into a doctor, lawyer, benefits specialist, or care manager. It will help families recognize which problem they are actually solving—and which professional should be at the table.

One emergency, five separate problems

Maria's emergency appears to be one question: *Where will she live on Monday?*

Inside that question are five different problems:

1. **Care:** What assistance and supervision does Maria require?
2. **Setting:** Which places can safely provide it?
3. **Price:** What will each realistic option actually cost?
4. **Payment:** Which personal resources or public benefits may apply to that setting?
5. **Timing:** What can the family arrange before discharge, and what requires a longer plan?

The problems cannot be solved randomly. Financial eligibility does not make an unsafe setting appropriate, and a clinically appropriate residence is not sustainable if its price exhausts the money. A beautiful apartment does not become Medicaid-funded because a salesperson says *Medicaid*.

The safest order is care, setting, price, payment, and timing.

Elena was about to learn this in reverse.

PART I

Understand the Problem Before Spending Money

CHAPTERS 1–4

First define the care, the setting, the real price, and the family's resources. Money decisions are safer after the problem is correctly named.

CHAPTER 1

The $4,300 Apartment That Cost $7,150

On Saturday morning, Elena toured Meadowbrook Senior Living.

The lobby had fresh flowers and a chandelier large enough to have its own retirement account. Marissa, the salesperson, opened a glossy folder.

“Our one-bedroom apartments begin at $4,300 a month,” Marissa said.

Elena’s shoulders dropped. Maria’s income and savings might cover that for several years.

“And that includes care?”

“We create an individualized service plan for every resident.”

It sounded like yes. It was not yes.

After the tour, a nurse asked about medications, personal care, falls, nighttime confusion, and resistance to help. Elena described difficult days, not Maria’s polished twenty-minute performance.

The nurse made notes and left the room. Marissa returned with a second sheet of paper.

Apartment and hospitality: $4,300.

Medication management: $550.

Personal-care level: $1,450.

Nighttime safety plan: $500.

Community and assessment fees, due before move-in: $3,200.

The new monthly total was $6,800, before several outside expenses. If Maria required a higher care level after another fall, the estimated monthly charge would become $7,150.

“I thought you said forty-three hundred,” Elena said.

“That is the apartment.”

“My mother would be living in the apartment.”

Marissa gave the sympathetic smile of a person who had heard this sentence before.

Housing plus care

Assisted living is usually two products sold together:

1. **Housing and hospitality:** the apartment, meals, housekeeping, activities, utilities, maintenance, and common spaces.
2. **Personal support and care:** help with medications, bathing, dressing, toileting, mobility, safety checks, and other services identified in the assessment and contract.

The advertised price may describe mostly the first product. The assessment determines the price of the second.

Care levels are not standardized. “Level two” may be called “level three,” priced separately, or unavailable elsewhere. Even *all-inclusive* requires a list of exclusions.

REMEMBER THIS
The apartment price and the care price are different numbers. A family must obtain both in writing.



The price was only the first surprise

Elena asked what would happen if Maria’s dementia progressed.

“We support aging in place,” Marissa said.

“Does that mean she can stay here no matter how much help she needs?”

“It means we do everything possible to support our residents.”

Again, it sounded like yes. Again, it was not yes.

The contract allowed reassessment. Meadowbrook could not retain someone needing frequent two-person transfers, continuous supervision, or certain nursing care. Maria might stay for years—or move after another hospitalization.

“What happens when her money runs out?” Elena asked.

Marissa paused.

“We are private pay.”

“The website says you accept Medicaid.”

“Residents may use Medicaid coverage for some outside medical services. We are not an Illinois Supportive Living Program community.”

The distinction might eventually save thousands. For now, the floor seemed to move.

Medicare could cover qualifying medical services, but not generally Maria’s ongoing apartment, meals, supervision, and custodial care. Illinois Medicaid might help later, but not as automatic payment at any private residence. Setting and program had to match.

Elena put the contract into her bag without signing it.

For the first time since Friday afternoon, she had made an important decision by refusing to make one too quickly.

REMEMBER THIS
A residence is affordable only if it can meet the person’s needs and the payment plan survives the next change.

Three families enter the same maze

While Elena drove home with the Meadowbrook folder on the passenger seat, two other Illinois families were opening different versions of the same problem.

In Naperville, Ellen Walsh faced a kitchen table covered with financial records. Her husband Robert had vascular dementia, and memory care cost more than their combined income.

Their daughter wanted to sell the house. Their son objected. Robert believed they were discussing someone else.

Ellen had one private fear she had not said aloud: if every dollar went to Robert’s care, what would happen to her?

In Springfield, Anne Carter found her late father’s discharge papers in a box labeled CHRISTMAS LIGHTS. An online company offered to secure “Aid and Attendance” if she first moved her mother’s savings into a financial product.

The representative sounded confident. Confidence was not accreditation—or loyalty.

The three women would encounter different parts of the Illinois payment system:

- Elena would try to match dementia care with an affordable setting.
- Ellen would learn that Medicaid has rules intended to protect a spouse who remains in the community.

- Anne would discover that VA Pension with an Aid & Attendance increase can be valuable, but it is not automatic, freestanding, or safely obtained through every person who offers to help.

They are fictional composites. Their confusion is real.

What Elena did before Monday

Elena did not solve Maria's future. She built a safer bridge. She collected the discharge notices, care recommendations, and power of attorney. Mark was assigned five years of financial statements. Elena listed Maria's income, savings, condominium expenses, insurance, debts, military history, and significant transfers. The list did not place Maria; it turned fear into problems that could be investigated.

On Sunday evening, Mark called again.

"I found a place that takes Medicaid," he said.

"Is it an Illinois Supportive Living provider?"

There was a long silence, followed by the sound of a keyboard.

Elena looked at the clock.

Monday was still coming. But now she knew the next question to ask.

Before Elena Signs Anything

By Monday morning, Elena had toured two more communities and learned that a polished answer was not always a complete one.

At one residence, "a nurse available twenty-four hours" meant a nurse was on call overnight, not necessarily in the building. At another, "aging in place" meant residents could remain through many changes—but not if they needed frequent two-person transfers, continuous supervision, or services outside the residence's license and staffing.

None of these phrases was automatically dishonest. They were incomplete until connected to Maria's actual needs.

The clinical assessment and the financial assessment were happening at the same time. A residence needed to know whether Maria required help with medications, bathing, dressing, toileting, eating, walking, transferring, behavior, and nighttime safety. Those same answers could determine the care charge.

Elena resisted two opposite temptations. She did not minimize Maria's needs to secure admission at a lower price. She did not describe Maria only by her worst night. She brought the medication list, therapy recommendations, fall history, and the neurologist's report and asked Maria's treating professionals to clarify what was medically necessary.

The result was not a perfect prediction. It was an honest starting point.

Scheduled help is not the same as immediate help

A brochure may promise staff availability twenty-four hours a day. The more useful questions are operational:

- How does a resident request help at night?
- Who responds, and what happens if several residents need help at once?
- Can staff provide the transfers and personal care Maria requires?
- Who evaluates a sudden change in condition?
- When is a licensed nurse physically present?
- Which services must come from an outside agency?
- What changes would require Maria to move?

The last question was the hardest. Elena wanted a home for her mother, not a temporary answer. But no residence could responsibly promise to meet every possible future need.

The contract is part of the care plan

Before paying a deposit, Elena requested the complete admission agreement. She looked for:

- every recurring charge and the process for changing care levels;
- notice before price increases;
- deposits, refunds, and charges during hospitalization or after death;
- responsibility for medications, supplies, private aides, and outside services;
- transfer and involuntary-discharge provisions;
- arbitration and dispute terms;
- the signature language and any request for a personal guarantee; and
- what happens when private funds decline.

Signing under a power of attorney did not mean Elena should casually promise to pay Maria's bills from Elena's own money. The wording mattered. A contract involving a large financial commitment, personal guarantee, arbitration clause, or unusual discharge provision deserved legal review.

Elena began to understand that affordability was not savings divided by the first month's price. A realistic forecast had to include price increases, higher care needs, medical and personal expenses outside the residence, the condominium's continuing costs, and the possibility of another move.

Bottom Line

Assisted living combines housing with care, but the advertised price may describe only part of the arrangement. The family needs a written total based on the resident's present needs, the residence's clinical limits, the complete contract, and a plan for what happens when either health or money changes.

CHAPTER 2

The Illinois Care and Payment Map

On Sunday afternoon, Maria's dining table became command headquarters.

Elena brought her notebook. Mark joined by video. Two cousins arrived carrying internet articles, each confident that the other article was wrong. Maria sat at the head of the table with tea and watched her family debate her future as though she had stepped out of the room.

"Medicaid takes the house," one cousin announced.

"Medicaid never takes the house," said the other.

"My neighbor put everything in a trust," Mark added from the laptop.

Elena rubbed her forehead. "I thought Medicare paid."

Maria raised one finger.

"Has anyone called someone who actually knows?"



The table fell quiet.

Maria's question became the organizing principle for the book. Internet stories often contain one true fact wrapped in missing facts: the person's marital status, type of property, timing, program, care setting, prior transfers, or state. A correct answer for one family may be dangerous for another.

The house question, for example, is not one question. It may involve eligibility while the owner is alive, whether a spouse remains in the home, how the property is titled, whether it is sold, and whether Illinois may later seek estate recovery. Those issues belong in later chapters. Moving the deed during a Sunday family meeting does not.

Two maps, not one

When families say, “We need assisted living,” they may mean a safer apartment with meals, occasional help with bathing, continuous dementia supervision, nursing care after a decline, or simply a setting they hope Medicaid can help pay for.

The **care map** asks where the person’s needs can safely be met.

The **payment map** asks which resources or programs may pay for that specific arrangement.

> REMEMBER THIS
> **A workable plan exists only where the care map and the payment map overlap.**

For Maria, the care map excluded an independent apartment with weekly housekeeping. Her clinical team believed she needed medication oversight, help with personal care, meal support, and a response available at night. Possible routes included home with reliable services, private assisted living or memory care, a suitable Illinois Supportive Living Program provider, or nursing-facility care if her needs became greater than an assisted setting could manage.

Her payment map included monthly income, savings, the condominium, possible long-term-care insurance that Elena had not yet located, and potential future Medicaid eligibility. It did not include unlimited private pay.

Option 1: remaining at home with support

Home can preserve familiarity and control, but “home” is not a care plan by itself. A real plan identifies who will be present, what each person or agency will do, how nights and emergencies are covered, and what change makes the arrangement unsafe.

Supports may include family or paid caregivers, home health, therapy, adult day services, meals, transportation, emergency-response systems, and home modifications. Medicare or other insurance may cover qualifying medical services under its rules; it does not become open-ended payment for round-the-clock custodial care.[1]

Illinois’ Community Care Program helps qualifying older adults who might otherwise need nursing home care remain at home. The Department on

Aging describes services such as care coordination, adult day services, in-home services, emergency home response, and automated medication dispenser services. Current eligibility must be verified.[2]

For Maria, returning to the condominium would require more than Elena stopping by before dinner. The schedule had to cover medication, meals, bathing, wandering risk, and nighttime response. A plan that depended on Elena never sleeping was not a plan.

Option 2: independent living

Independent living often offers housing, meals, activities, transportation, and freedom from household chores. It is not necessarily licensed or staffed to provide ongoing personal care.

Some communities allow residents to hire outside caregivers or offer several care levels on one campus. That does not automatically guarantee admission to the next level, fix the future price, or create Medicaid eligibility. The contract and the person's needs still control.

Option 3: private-pay assisted living and memory care

Private assisted living combines housing with personal, supportive, and certain health-related services. In Illinois, assisted living and shared housing establishments are licensed and surveyed through the Department of Public Health.[3]

Memory care may be a secured unit or specialized program within a larger community. The label alone does not reveal staffing, nursing capacity, response to wandering or agitation, or the circumstances that trigger discharge.

Many private assisted living and memory-care arrangements are paid from income and assets, qualifying long-term-care insurance, or family help. Some residents may also qualify for VA benefits. Covered medical services can still be billed to Medicare or other insurance, but that does not make the insurer responsible for the residence's entire monthly charge.

The critical question is what happens before private funds are exhausted. The answer must be obtained before admission, not during the final month of savings.

Option 4: the Illinois Supportive Living Program

The Illinois Supportive Living Program, usually called SLP, is an affordable assisted living model administered by the Illinois Department of Healthcare and Family Services. It is not the "Medicaid department" inside every private assisted living community.

Illinois describes SLP as a Medicaid alternative to nursing home care for low-income older persons and persons with physical disabilities. Through

a Medicaid waiver, the program can cover supportive services such as personal care, homemaking, medication assistance, activities, and twenty-four-hour response. The resident remains responsible for room and board and may have an additional cost-of-care obligation under current rules.[4]

Limited money alone does not establish eligibility. Current requirements include Illinois residency and qualifying status, age or disability criteria, financial eligibility, and a screening that finds nursing-facility level of care and determines that SLP is appropriate. A certified provider must also have an opening and be able to meet the person's needs.[5]

That creates four separate gates:

1. clinical and functional appropriateness;
2. financial and other program eligibility;
3. a certified provider with an available unit; and
4. the provider's ability to meet the applicant's needs.

Passing one gate does not open the others.

Elena called the community Mark had found. It was an SLP provider, but it had a waitlist. A second provider had an opening but needed to review Maria's dementia-related needs. For the first time, "Medicaid accepted" had become a specific pathway instead of a reassuring phrase.

Option 5: nursing-facility care

Nursing facilities provide a higher level of nursing and personal care. Some people enter for short-term rehabilitation and return home or move to another setting. Others require long-term nursing-facility care.

Medicare may cover a limited period of skilled nursing facility care when its requirements are met. It generally does not cover long-term custodial nursing home care.[1] Long-term care may be paid privately, through qualifying insurance, or by Medicaid for a person meeting medical and financial rules.

An end to Medicare payment does not automatically answer whether a resident must leave that day. Coverage, payment, clinical need, discharge rights, and bed availability are connected but distinct. Written notices and appeal information matter.

Ellen's Map: One Spouse Needs Care

While Elena searched for a setting, Ellen Walsh sat beside her husband Robert at their kitchen table in Naperville.

Robert had vascular dementia. He could still make coffee, although he sometimes put the grounds into the water reservoir. He could discuss the Cubs in exact detail and then forget Ellen had gone to the pharmacy. After

he walked outside one winter night without a coat, his physician and care team recommended continuous supervision.

The memory-care quote exceeded their combined monthly income. Ellen's daughter wanted to sell the house. Her son insisted Medicaid would take it anyway. Robert listened for several minutes and asked why everyone was discussing his brother.

Ellen's private fear was not greed. It was survival.

If Robert's care consumed everything, how would she pay the property taxes, utilities, groceries, insurance, and her own future care?

Medicaid's spousal impoverishment rules exist because the spouse who remains in the community still needs income and resources. The rules are technical and the amounts change. They do not mean a couple may keep everything, and they do not require the community spouse to become destitute before asking for help.

For now, Ellen needed one instruction: do not sell, gift, retitle, or spend major assets based on a family argument. A married couple should obtain individualized Illinois elder-law advice before irreversible moves.

Robert's care map and Ellen's survival map had to be considered together.

Anne's Map: The Benefit Hidden in a Box

In Springfield, Anne Carter found her late father's discharge papers under a string of Christmas lights. Her mother, Ruth, had moved into private assisted living after repeated falls. A neighbor mentioned "Aid and Attendance," describing it as money the VA gives veterans for assisted living.

The description was close enough to sound useful and incomplete enough to be dangerous.

Some wartime veterans and surviving spouses may qualify for VA Pension. A person meeting additional care-related requirements may qualify for an increased pension amount associated with Aid & Attendance. It is not a freestanding benefit for every veteran or every assisted living resident.

The VA considers service, discharge status, age or disability, income, net worth, unreimbursed medical expenses, and other rules. Anne also discovered that not everyone offering "VA planning" was accredited to represent claimants before the VA.

An online salesperson offered to help if Ruth first moved savings into a financial product. Anne stopped. Before changing an asset, she would verify the current rules and seek a VA-accredited representative who did not need to sell her mother something.

Aid & Attendance might become part of Ruth's payment map. It could not replace the care map or erase every other eligibility rule.

The Payment Lanes

Most families combine several sources over time:

Private income and assets. Social Security, pensions, savings, investments, retirement withdrawals, and property proceeds often create the first bridge. The question is how long they last after taxes, insurance, medical costs, home expenses, and rising care charges.

Long-term-care insurance. Coverage depends on the actual policy: benefit triggers, waiting periods, maximums, inflation protection, covered settings, and claims procedures. An old brochure is not the policy.

Medicare and health insurance. These may cover qualifying medical services. They generally are not comprehensive long-term housing and personal-care plans.

Medicaid. Medicaid can be central to long-term nursing-facility care and certain home- and community-based services, including SLP for eligible Illinois participants. Eligibility involves more than a low bank balance, and provider participation must be verified.

VA Pension with an Aid & Attendance increase. This may help an eligible veteran or surviving spouse, but it has its own service, financial, care, and application rules. Coordination with Medicaid requires care because the programs are not identical.

Family support. Money from adult children should be identified as a gift, loan, shared expense, or payment under an agreement. Parent and child funds should remain traceable and separate.

The bridge to Monday

Elena did not solve Maria's entire future before discharge. With Maria's clinical team, she arranged a short-term plan while the family evaluated longer-term settings and payment options. She did not sell the condominium, move Maria's savings into her own account, or sign a personal guarantee.

She kept the Meadowbrook contract, the SLP provider information, and the care assessment in one folder. Mark was assigned the financial records. Maria was included in every conversation she could meaningfully participate in.

The family still had hard decisions ahead. But the emergency had been divided into care, setting, price, payment, and timing. Each problem now had a next step.

> REMEMBER THIS
> **A benefit helps only when it fits the person, provider, timing, and care plan.**

The Illinois Long-Term Care Ombudsman Program can provide free, resident-directed information and advocacy to residents, prospective residents, former residents, and families considering long-term-care placement. Ombudsmen can help with rights, complaints, and involuntary transfer or discharge concerns.[6]

Bottom Line

Illinois senior living is not one straight ladder. Home care, independent living, private assisted living, memory care, Supportive Living, and nursing facilities serve different needs and use different payment structures.

The family's safest sequence is consistent: define the care, identify realistic settings, obtain the complete price, match payment sources to that setting, and plan the next transition before the present arrangement fails.

CHAPTER 3

What Will Care Really Cost?

At 8:14 on Monday evening, Elena received an email from Meadowbrook with the subject line **Your Personalized Monthly Estimate**.

She opened it expecting a number.

She found five attachments.

One described the apartment. One described the care assessment. One listed optional services, although several appeared less optional than the title suggested. One explained deposits and refunds. The fifth was a contract long enough to qualify as winter reading.

Elena spread the pages across Maria's dining table. The original brochure still sat near her elbow: **Studios from $4,300 per month.**

The new estimate looked different:

- apartment, meals, housekeeping, activities, and basic response: $4,300;
- Level Two personal care: $1,450;
- medication management: $550; and
- nighttime safety plan: $500.

The total was $6,800 per month.

The assessment also said that if Maria needed more hands-on help after admission, her care package could rise by $350. That would bring the monthly charge to $7,150. The figures were fictional, but the structure was painfully believable: the apartment had one price, Maria's present care had another, and her next decline had a third.

Mark appeared on Elena's laptop from his California patio.

"So it isn't forty-three hundred."

"The apartment is forty-three hundred."

"Does Mom come with the apartment?"

Elena gave him a long look. It was the first useful financial question he had asked all week.

The number on the brochure is not the budget

In CareScout's 2025 survey, the reported Illinois median for a private one-bedroom assisted-living community was $6,219 per month.[7] A statewide median is a planning reference, not a quote for a particular person. Geography, apartment size, staffing model, memory care, and service needs can move the actual price far above or below it.

Even a complete residence quote may omit health-insurance premiums, prescription copays, medical and dental care, clothing, personal supplies,

companion care, transportation outside the included schedule, cable or telephone service, and the cost of maintaining or selling the former home.

Elena needed three numbers, not one:

1. the **entry price** advertised before the assessment;
2. the **current-needs price** based on Maria's actual service plan; and
3. the **stress-test price** if Maria's needs increased by one care level or required memory care.

The entry price helped Elena decide whether to tour. The other two helped her decide whether Maria could afford to live there.

> REMEMBER THIS
> **The number on the brochure is not the budget. Price the resident's current needs—not only the apartment.**

How an assisted-living bill grows

Communities use different pricing models. One may charge a base rate plus a care level. Another may price individual services. Another may offer packages with limits. Memory care may have its own base rate and care charges. The labels are not standardized enough to compare safely by name alone.

The bill may grow through several doors.

A higher care level. More help with bathing, dressing, toileting, walking, transfers, eating, redirection, or nighttime needs may trigger reassessment and a new charge.

Medication services. Reminders, supervision of self-administration, administration by licensed personnel, pharmacy packaging, and the medications themselves are not the same service. A family needs to know which one is quoted.

Two-person assistance. A resident who once stood with one helper may later need two staff members or mechanical assistance. Some communities charge more; some cannot provide that care at all.

Incontinence care and supplies. The hands-on service, scheduled checks, laundry, and disposable supplies may be bundled together or billed separately.

Nighttime or safety support. Scheduled checks, escorts, behavior support, or private companions may carry separate charges. A twenty-four-hour response system does not necessarily mean continuous one-to-one supervision.

Transportation. A community bus may cover group outings or certain local appointments. Wheelchair transportation, escorts, wait time, mileage, and trips outside a service area may cost extra.

Outside providers. Therapy, home health, private-duty aides, podiatry, dentistry, and other services may be billed by separate providers. Insurance coverage depends on the service and the person's plan; the residence's monthly fee does not settle that question.

Apartment and household choices. A larger unit, second occupant, pet, parking space, storage area, or premium location can change the housing charge before care begins.

No single fee on that list was necessarily improper. The danger came from treating a partial price as a complete one.

Illinois gives families a place to look

For an Illinois assisted-living establishment, the written contract matters. Current Illinois law and administrative rules require the contract to identify the base rate and services included in that rate, additional services and their fees, billing and payment procedures, the amount and purpose of fees, charges, and deposits, refund policies, and residency-termination criteria and procedures.[8]

The rules generally require at least thirty days' written notice before a fee or charge changes. There is an important exception: that advance notice is not required for an increase tied to a change in the resident's service needs when the change is documented in the service plan.[8]

That exception made the care assessment financially important. Elena could not protect Maria by understating her needs. An inaccurate assessment might create an unsafe admission, a rapid price increase, or both. She could protect Maria by asking how the assessment worked, what facts placed a resident in each care level, who could change it, how the family would be informed, and how disagreements were reviewed.

"Can I see the written criteria for Level Three?" Elena asked Marissa, the sales director.

"Our nurse determines the level after assessment."

"I understand. I would like the service description and fee schedule that show what changes at Level Three."

There was a pause.

"I can send those."

The pause did not prove there was a problem. It proved that Elena had moved from brochure language to contract language.

First month, ordinary month, first year

Mark multiplied $6,800 by twelve and announced that Maria's annual cost would be $81,600.

It was a reasonable calculation and an incomplete budget.

Meadowbrook's fictional one-time charges included a $2,700 community fee and a $500 assessment fee. Elena estimated another $2,000 for movers, basic furniture, and setting up the apartment. Maria's condominium could cost roughly $1,650 per month for four months while the family prepared it for sale. Prescriptions, insurance premiums, personal supplies, and medical expenses outside the residence might average another $400 per month, although the real amount would vary.

Elena wrote the first-year working estimate:

- residence at the current care level: $81,600;
- one-time community and assessment fees: $3,200;
- moving and setup: $2,000;
- four months of condominium carrying costs: $6,600; and
- personal, insurance, medication, and medical costs outside the residence: $4,800.

Working first-year total: $98,200.

If the higher care level began in month four, the first-year total would increase by another $3,150, reaching approximately $101,350.

This was not a prediction. It was a stress test. The arithmetic gave the family time to ask whether they could carry the overlap, what could be reduced, and when another payment path might be needed.

REMEMBER THIS

Before signing, price one foreseeable decline and the first year—not only the first month.

The former home keeps sending mail

Families often count a house or condominium as an asset while forgetting that it remains an expense until it is sold, rented, or otherwise resolved. Mortgage payments, property taxes, assessments, insurance, utilities, repairs, lawn care, snow removal, security, and cleanout costs may continue during the transition.

The home may also be emotionally unavailable even when it is financially available. Maria's condominium contained forty years of photographs, furniture, recipes, and objects no spreadsheet could sort. Elena could estimate the property tax. She could not estimate how many evenings it would take Maria to decide what happened to her wedding china.

Rushing a sale could create regret or a poor price. Delaying indefinitely could drain cash needed for care. The solution was not to pretend the overlap did not exist. It was to give it a line in the budget and a person responsible for managing it.

Mark received that assignment.

"From California?" he said.

“The property tax website works there.”

Robert’s price affected two lives

Ellen and Robert Walsh received a memory-care quote of $8,400 per month. The sales counselor carefully explained that the amount included Robert’s apartment, meals, medication management, and his present care package. It did not include personal supplies, outside medical services, or a private companion if Robert later needed individual supervision.

Ellen’s daughter divided their savings by $8,400 and calculated the month Robert would run out of money.

Ellen looked at the calculation and felt herself disappear from it.

She still needed to live in their house. She still had groceries, insurance, property taxes, utilities, and medical expenses. She might eventually need care herself. A married couple’s plan could not treat every available dollar as Robert’s private-pay fund.

The later chapters on Medicaid and spousal impoverishment would address the governing rules. For now, Ellen wrote two budgets: Robert’s care budget and her community-living budget. The same income or asset could not be casually promised to both.

She also asked the community for its clinical ceiling. If Robert began wandering at night, needed two-person transfers, or became unable to participate safely in the program, could the community continue caring for him? If not, how much notice would the family receive and who would help plan the next setting?

A low price for a residence that could not meet his likely needs might become the most expensive option after two move-in fees, two moving bills, and two crises.

Anne refused to spend a benefit before approval

Anne Carter hoped VA Pension with an Aid & Attendance increase might help pay for Ruth’s assisted living. She was gathering discharge records, marriage documents, financial statements, and care information. The possibility was meaningful. It was not cash in Ruth’s bank account.

One residence encouraged Anne to move Ruth immediately because “the VA will reimburse you.” Anne asked for that promise in writing. The sentence became less certain.

Benefit eligibility, effective dates, evidence, processing time, and actual payment belong to later chapters. The budgeting rule was immediate: an unapproved benefit did not belong in the guaranteed-income column. Anne built a bridge using resources Ruth already controlled and treated possible VA money as a future improvement, not a present rescue.

That decision cost her nothing. Counting imaginary income could have cost her a deposit, a rushed move, and a monthly obligation Ruth could not carry.

Deposits, absences, refunds, and endings

The first-year estimate also depended on what happened when care did not proceed as planned.

Elena read the sections dealing with deposits and refunds. She wanted to know:

- whether a deposit held an apartment and under what conditions it was refundable;
- whether the community fee was refundable if Maria never moved in;
- what Maria would owe during a hospitalization or rehabilitation stay;
- how charges were handled after death;
- how much notice Maria had to give before a voluntary move;
- whether charges continued through the notice period after the apartment was emptied; and
- how a refund would be calculated and when it would be paid.

These details were not gloomy. They were ordinary contract planning. A family could love the community and still insist on understanding the exit.

Elena also asked what would happen if Maria's funds declined. Meadowbrook remained private pay and did not participate in the Illinois Supportive Living Program. A resident's later financial eligibility for Medicaid would not turn Meadowbrook into an SLP provider or make ordinary private assisted-living charges Medicaid-covered.

Marissa answered directly: Maria would need another payment source or another setting.

The answer hurt. It was also useful.

The one-page comparison

By Wednesday, Elena had stopped comparing communities by chandeliers, dessert menus, and the monthly number printed in large type. Each residence received one page in her notebook with the same categories.

Housing now: apartment rate, second-person fee, utilities, meals, housekeeping, laundry, parking, storage, and annual increase history.

Care now: assessed care level, tasks included, medication service, nighttime response, continence support, transfer assistance, and scheduled checks.

Care later: next care level, memory-care price, two-person assistance, private-companion expectations, and clinical reasons the residence could no longer serve Maria.

One-time money: application, assessment, community, move-in, pet, elevator, and setup fees; deposit amount; and refund terms.

Outside the bill: prescriptions, insurance, medical and dental care, supplies, transportation, private aides, telephone, clothing, and former-home costs.

Ending the contract: notice periods, charges during absence, voluntary and involuntary termination, refund timing, and responsibility after death.

Payment path: private-pay requirement, long-term-care insurance paperwork, VA documentation, and whether the setting was actually certified for the Illinois Supportive Living Program.

For every number, Elena recorded the document, date, and name of the person who provided it. “Included” was not a financial term until it was attached to a written service.

A practical price-check sequence

Before committing money to a residence, the family can follow a repeatable sequence:

1. **Obtain the complete fee schedule and contract.** Include attachments, service descriptions, deposit rules, and refund policies.
2. **Complete an honest care assessment.** Ask for the resulting service plan and current monthly total in writing.
3. **Price one foreseeable decline.** Request the next care level, memory-care rate, or other likely service increase.
4. **Build the first-year number.** Add one-time fees, moving costs, overlap with the former home, and expenses outside the residence bill.
5. **Identify the clinical ceiling.** Record which needs would trigger additional private help or a move.
6. **Confirm the payment path by name.** Private assisted living, SLP, nursing-facility Medicaid, Medicare, and VA benefits are not interchangeable labels.
7. **Have significant obligations reviewed.** A personal guarantee, unusual refund provision, arbitration clause, or unclear termination language may warrant review by a qualified Illinois attorney.

The purpose was not to predict every dollar. It was to prevent one attractive number from making the decision alone.

Bottom Line

The real cost of assisted living is the current care price plus the foreseeable increases, expenses outside the residence, transition costs, and the financial consequences of another move. Illinois contract rules give

families important written information, but the family still has to collect the documents and turn them into one honest first-year estimate.

Elena now knew that $4,300, $6,800, and $98,200 could all describe Meadowbrook accurately. They answered different questions.

The next question was no longer what the residence cost. It was what Maria actually owned, owed, received each month, and had already transferred. That financial snapshot would determine how long the bridge could hold—and which benefits or strategies were even available.

CHAPTER 4

The Family Financial Snapshot

Mark arrived from California on Wednesday with a roller bag, a laptop, and the confidence of a man who had not yet opened Maria's hall closet. Elena handed him a hard hat left over from a school fundraiser. "For safety," she said. Maria watched from the hallway, holding her tea.

The closet contained tax folders, expired insurance cards, paid utility bills, three shoeboxes labeled IMPORTANT, and a metal cash box whose key was taped to its lid. Mark lifted out a passbook from 1987 as if he had discovered a lost civilization. Maria smiled. "That account paid better interest than yours does."



"Carbon dating suggests the last deposit was during the Reagan administration."

The funny part was the passbook. The frightening part was that no one could say which accounts were open, which policies were active, what Maria received each month, or who could speak with the bank.

Elena stopped excavating and opened a blank page: Family Financial Snapshot—as of Wednesday. It did not need to solve everything. It needed to show what was true, what was missing, and what had to happen next.

A map, not a verdict

A financial snapshot is a one-page inventory of the facts that affect a care plan: income, assets, debts, insurance, property, military history, legal documents, and care needs. Date it, because balances, benefits, and care needs change.

It is not a Medicaid application, tax return, estate plan, or permission to move money. It makes the family's starting point visible enough to budget, compare care settings, and prepare useful questions for qualified advisers.

> REMEMBER THIS
> **The first job is not to move the money. It is to make the money visible.**

Build it in four passes

- **Cash flow.** Record reliable monthly payments and recurring bills. Use gross and net amounts when both matter. A Social Security benefit-verification letter can confirm current benefit information.[9]
- **Resources and obligations.** List accounts, retirement funds, investments, insurance cash value, trusts, debts, and unpaid bills. Record ownership and the latest statement location; do not collapse everything into one total.
- **Protection and authority.** Record health and long-term-care coverage, life insurance, property, and the location of powers of attorney, trusts, wills, and advance-care documents. The document—not its title—controls the authority granted under Illinois law.[10]
- **Care reality.** Describe the help needed now: medications, bathing, dressing, transfers, nighttime supervision, cognition, behavior, mobility, diet, and recent assessments. The wrong care level produces the wrong budget.

> DO NOT DO THIS YET
> **Do not retitle property, add names to accounts, give away money, cash out policies, close joint accounts, or transfer funds merely to simplify the snapshot. Record first; obtain qualified advice before acting.**

Three families, three missing facts

Elena needed Maria's cash flow and current statements. Ellen separated Robert's individual and joint accounts. Anne recorded her late father's branch and service dates without assuming a benefit existed; the VA explains how to request a missing DD214 and other service records.[11]

The One-Page Family Financial Snapshot

Working inventory — facts only, as of a specific date

Person: _____	Snapshot date: _____
Prepared with/by: _____	Next review date: _____
CATEGORY	**RECORD THE FACTS • AMOUNT • OWNER • LOCATION • MISSING ITEMS**
INCOME	Social Security (gross/net); pensions; annuities; VA payments; wages; rent; other regular income. Monthly total: $_____
ASSETS	Checking, savings, CDs; investments; retirement accounts; cash; trusts; insurance cash value; burial funds; business/other. Missing statements or access: _____
DEBTS	Mortgage/HELOC; credit cards; vehicle or personal loans; taxes; medical or facility balances; other obligations. Monthly debt payments: $_____
INSURANCE	Medicare; supplement or Medicare Advantage; Part D; Medicaid status; retiree coverage; long-term-care and life insurance. Note policy, benefit, waiting period, and premium status.
PROPERTY	Home: owner/title, estimated value, mortgage, taxes, insurance. Other real estate; vehicle; valuable property. Deed/title location: _____
MILITARY HISTORY	Who served; branch; service dates; discharge status; DD214/separation record; VA claim number; current VA benefits or letters. Record history—do not assume eligibility.
LEGAL DOCUMENTS	Property and health-care powers of attorney; will; trust; living will/advance directive; guardianship orders, if any. Record date, named agent/trustee, attorney, and signed-original location.
CARE NEEDS	Current setting and assessment date; bathing, dressing, toileting, transfers, eating; medications; cognition; nighttime/safety needs; mobility; behaviors; diet; skilled needs; likely next change. Current care estimate: $_____

PAPER TRAIL Keep the sheet private. Do not place full account numbers, passwords, Social Security numbers, or PINs on a shared copy.

Turn the inventory into decisions

By late afternoon, Elena's page was incomplete—but useful. She entered Meadowbrook's current-needs estimate from Chapter 3, Maria's dependable income, apparently accessible accounts, and every item still requiring proof. The page exposed a monthly gap and separated liquid funds from the condominium and old paper.

The snapshot converted "We cannot afford this forever" into three manageable tasks:

- **Measure the monthly gap.** Compare dependable income with the current-needs price, ongoing medical costs, premiums, and expenses continuing after a move. Keep the stress-test price beside it.
- **Separate value from cash.** A home, retirement account, insurance policy, and checking balance are not interchangeable. Record each without treating it as immediately spendable.
- **Name the missing fact.** A missing statement, unsigned document, unknown policy status, or service history can change the next step. Assign an owner and due date.

The sixty-minute first pass

Spend fifteen minutes on each group: income; resources and debts; insurance, legal documents, and military history; care needs and missing records. Mark uncertain items "unknown." A labeled unknown is safer than a confident guess.

> **REMEMBER THIS**
> **A one-page snapshot does not answer every question. It reveals which question must be answered next.**

What Elena did next

Elena asked Maria before contacting anyone. They requested a current Social Security letter, identified the calls Elena was authorized to make, and separated signed legal documents for review. Mark scanned ordinary statements. He was no longer advising from a patio; he was the document department.

Maria kept the 1987 passbook. "Historical artifact," she said, sliding it into her purse.

Bottom Line

A sustainable care plan cannot rest on scattered balances, memories, and assumptions. Build one dated page showing the money, obligations, protection, authority, and care needs. The snapshot is not the final answer. It is the first reliable map.

PART II

Build the Private-Pay Bridge

CHAPTERS 5–9

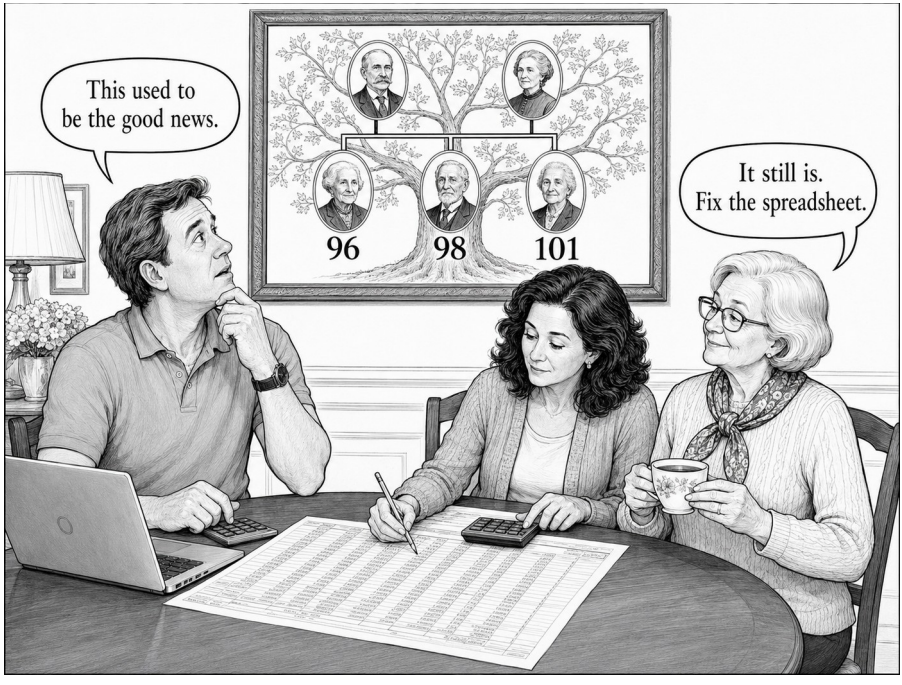
Turn the family snapshot into a bridge: measure the financial runway, organize income and assets, and prepare for the care and price changes that may come next.

CHAPTER 5

How Long Will the Money Last?

Mark's first spreadsheet ended when Maria turned eighty-eight. He had divided her savings by the current monthly gap, rounded down, and announced that the plan worked.

Maria looked past him at the framed family tree. Her mother had lived to ninety-six. Two aunts had reached ninety-eight and 101. Mark followed her eyes and slowly reopened the file.



Life expectancy is an average, not a personal deadline. The Social Security calculator itself describes an average number of additional years based only on sex and date of birth.[12] A care plan needs room for the possibility that the person lives longer than average—and needs more help along the way.

The question is not simply how many months the bank balance can pay today's bill. The useful question is: how much financial runway exists under several believable futures, and when must the family act before choices narrow?

Runway begins with the monthly gap

Elena returned to the dated snapshot from Chapter 4. For illustration, Maria's fictional numbers were:

- **Current care price:** $6,800 a month at Meadowbrook.
- **Costs outside the residence:** $400 a month for premiums, prescriptions, personal items, and other medical expenses.
- **Dependable monthly income:** $3,200 from income already received and verified.

THE CORE CALCULATION

Monthly gap = current care price + recurring outside costs + continuing home costs – dependable monthly income

After the condominium was resolved, Maria's working gap was $4,000 a month: $6,800 + $400 − $3,200. Elena did not count investment growth, a possible benefit, or sale proceeds that had not arrived.

Then identify usable funds

Suppose the snapshot showed $240,000 in liquid funds assigned to care. Elena first reserved $5,200 for Meadowbrook's community and assessment fees, movers, and basic setup. That left $234,800 for the continuing gap.

$234,800 ÷ $4,000 = 58.7 months. The flat calculation says the funds can cover fifty-eight complete monthly gaps, or a little under five years. It is useful—and incomplete.

REMEMBER THIS

Life expectancy is an average, not an expiration date. Build the plan for the long life you hope for.

All amounts in this example are fictional and rounded. They demonstrate the method; they are not a recommendation, eligibility result, tax projection, or promise of future cost.

One runway is not enough

The flat runway freezes every moving part. Real life does not. Residence prices can rise; care needs can change; the old home can remain costly; income may receive adjustments; and investment withdrawals may have tax consequences. General consumer inflation is measured by the federal Consumer Price Index, but assisted-living charges do not have to move at the same rate.[13]

Elena and Mark therefore tested the same $234,800 three ways:

SCENARIO	WHAT MARIA TESTED	RESULT
FLAT	The $4,000 monthly gap never changes.	**58 full months**
PLANNING	The gap rises 4% each year; $750 more care begins in year three.	**50 full months**
STRESS	The gap rises 6% each year; $1,200 more care begins in year two; the condominium costs $1,650 for four months.	**43 full months**

The 4% and 6% figures are planning assumptions, not forecasts. The additional care charges are fictional stress tests. Under those assumptions, the apparently five-year runway fell to roughly four years and two months in the planning case and three years and seven months in the stress case.

Test the change that could actually happen

- **Price change.** Use the contract's notice terms and recent written rate history if available.
- **Care escalation.** Add a realistic higher care level, medication support, nighttime help, or a different setting.
- **Transition delay.** Keep temporary home costs, a waitlist, claim delay, or sale delay in the bridge.
- **Liquidity and taxes.** Do not treat every asset value as spendable cash. Ask a qualified tax or financial professional about withdrawals and sales.

Cash, property, and promises are not interchangeable

A condominium's estimated value is not cash until the sale closes and costs are paid. An unapproved insurance claim or VA benefit is not dependable monthly income. A retirement account may be liquid but taxable. A family contribution is not a plan until the amount, timing, and limits are candidly understood.

Anne left Ruth's possible VA benefit out of the guaranteed-income column. She could add it after an award and actual payment. That made the early runway look shorter, but it prevented a hopeful assumption from financing a binding contract.

A couple needs a two-life runway

Ellen could not assign every joint dollar to Robert's care. Her worksheet needed Robert's care expenses and Ellen's continuing housing, food, insurance, taxes, transportation, and emergency reserve. Later chapters explain the rules that may protect a spouse at home; the immediate budgeting lesson is simpler: the same dollar cannot safely support two budgets at once.

Choose three dates—not one

- **Review date.** Recalculate on a regular schedule and whenever a trigger occurs.
- **Decision date.** Begin the next housing, benefit, legal, or financing step while choices still exist.
- **Estimated depletion date.** Keep it visible, but never use it as the date to begin planning.

> REMEMBER THIS
> **Do not plan to the day the money reaches zero. Set a decision date while choices still exist.**

Elena placed Maria's first decision date twelve months before the stress-test runway ended. Twelve months was not a legal rule. It was breathing room for research, advice, applications, waitlists, and a change of course.

The Financial Runway Worksheet

Working estimate — use current documents and write the "as of" date

INPUT	AMOUNT OR WORKING ASSUMPTION
A. MONTHLY COST	Current care price $_____ Outside recurring costs $_____ Temporary home costs $_____
B. DEPENDABLE INCOME	Social Security $_____ Pension/annuity $_____ Other dependable income $_____
C. MONTHLY GAP	A minus B = $_____ per month
D. USABLE FUNDS	Liquid funds assigned to care $_____ Minus one-time costs/reserve $_____ Net usable funds $_____
E. FLAT RUNWAY	Net usable funds ÷ monthly gap = _____ months This is a starting estimate—not the plan.
F. STRESS TEST	Annual price assumption _____ % Possible added care $_____ starting month _____ Other risk: _____
G. THREE DATES	Review date _____ Decision date _____ Estimated depletion date _____
H. TRIGGERS	Recalculate after a price/care change, benefit decision, home sale, major expense, or large withdrawal.

KEEP IT SAFE Use rounded figures on a shared worksheet. Keep account numbers, passwords, Social Security numbers, and tax documents elsewhere.

A runway is a living document

Mark wanted to perfect the spreadsheet. Elena wanted to know when it had to change. Maria wanted lunch. The three goals were compatible once the family agreed that the runway was a dated decision tool, not a prediction carved in stone.

Recalculate when the facts change

- **Care or price:** a new assessment, fall, hospitalization, care-level increase, annual rate notice, or move to another setting.
- **Money:** a home sale, major repair, large medical bill, insurance decision, benefit award or denial, investment withdrawal, or change in dependable income.
- **Time:** the scheduled review date arrives—even if nothing dramatic has happened.

What the family decided

The family kept three versions of Maria's runway on one page. Elena owned the monthly update. Mark maintained the calculations and attached the assumptions. Maria approved the reserve they would not spend without another conversation. A separate list named the professionals whose advice might be needed before selling property, changing investments, moving money, or applying for benefits.

Mark extended the spreadsheet to Maria's 105th birthday. Maria examined it and tapped the last column. "Add one more year," she said. "I dislike tight deadlines."

Bottom Line

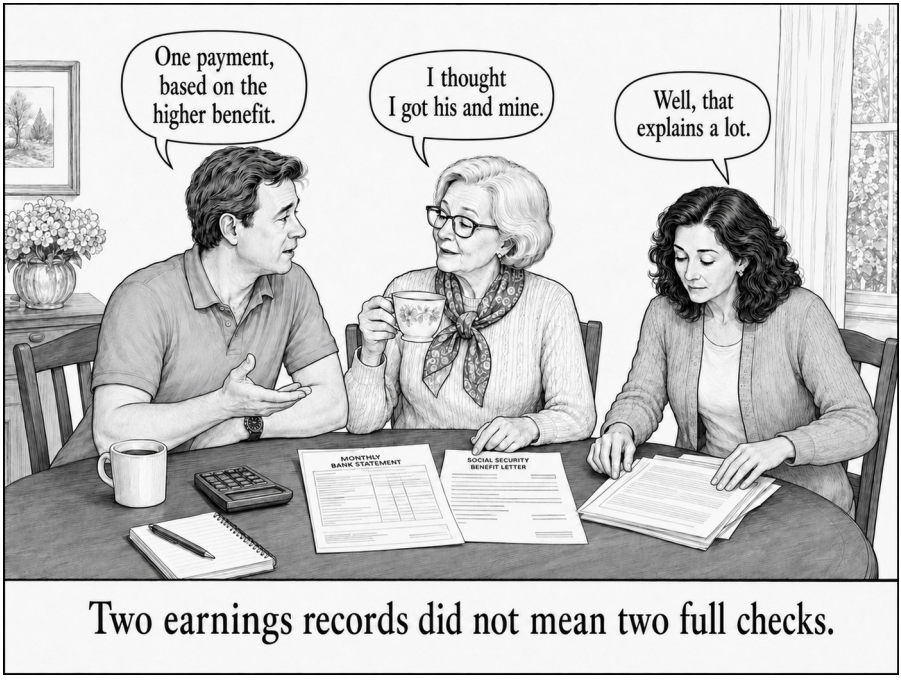
Start with the monthly gap, subtract one-time costs from usable funds, and calculate a flat runway. Then test price increases, care escalation, delays, taxes, and the needs of anyone who still depends on the same money. Most important, choose a decision date before the projected depletion date. A runway is valuable because it tells the family when to act—not because it predicts the future perfectly.

CHAPTER 6

Income Sources Families Overlook

Elena asked Maria to explain every deposit on the last three bank statements. Maria recognized Social Security, a small pension from the hospital, and an annuity payment. Then she pointed to the Social Security deposit. “That is his and mine,” she said, referring to Elena and Mark’s late father.

Mark opened the current benefit letter. The deposit was real. The family’s explanation was not.



Social Security explains that when a person is eligible for Survivor and another benefit, the payments are not simply added together; the person chooses the payment that is best and may be able to switch later.[14] Private pensions, annuities, and retirement accounts follow their own documents and rules.

Chapter 5 used Maria’s dependable income to calculate her runway. This chapter opens that number. Every deposit must be traced to its source, owner, gross amount, deductions, duration, and proof before the family relies on it.

Search in six directions

Families overlook income for ordinary reasons: old employers merged, a spouse handled the paperwork, deposits use abbreviations, benefits changed after a death, or a payment is so small that nobody asked what it was. Search records, not memory.

- **Social Security.** Obtain the current benefit-verification letter. Record the gross benefit, Medicare and tax deductions, net deposit, and whether the payment is based on the person's own, spouse's, or survivor record.
- **Employer pensions.** List every employer and union for both spouses. Find the plan administrator, benefit election, survivor option, cost-of-living terms, and tax withholding. PBGC maintains a searchable database for certain unclaimed benefits transferred after a retirement plan ended.[15]
- **Annuities.** Find the contract—not just the deposit. Record the owner, annuitant, beneficiary, payment frequency, guarantees, term, surrender provisions, and whether payments can change.
- **Retirement accounts.** Separate account value from withdrawals. Record regular distributions, required minimum distributions, ownership, withholding, and whether payments are automatic or discretionary.
- **Public programs.** Screen for cash assistance and programs that reduce food, medical, premium, or prescription costs. A cost reduction can extend the runway even when no new deposit appears.
- **Other verified income.** Check rent, royalties, wages, trusts, structured settlements, and recurring transfers. A transfer from another family account is not new income; label it accurately.

REMEMBER THIS

A regular deposit is not fully understood until the family knows who pays it, why, to whom, how long, and what can change it.

Elena assigned Mark the former-employer search. His confidence lasted until Maria produced a list of four hospital names—three of them apparently the same employer after mergers.

Gross, net, dependable, or possible?

One payment can occupy several categories. The gross amount may be larger than the bank deposit. The net amount may reflect Medicare premiums, tax withholding, insurance premiums, or other deductions. A payment can be dependable today but still change after a death, election, reassessment, or contract event.

Use four labels

- **Gross.** The amount before deductions. Keep it because benefit letters, tax forms, and eligibility rules may use gross rather than deposited income.
- **Net.** The spendable amount reaching the account. Use this carefully in the household cash-flow budget.
- **Dependable.** Approved, active, documented, and reasonably expected on its stated schedule. Record what could stop or change it.
- **Possible or pending.** Worth pursuing, but excluded from guaranteed income until eligibility, amount, start date, and actual payment are confirmed.

Benefits may appear as lower expenses

Illinois uses its Application for Benefits Eligibility system for programs including health coverage, SNAP, cash assistance, and Medicare Savings Programs.[16] Eligibility and reporting rules differ. A Medicare Savings Program or prescription-cost assistance may improve cash flow by reducing an expense rather than producing ordinary income.

The Illinois Senior Health Insurance Program provides free statewide counseling for Medicare beneficiaries and caregivers.[17] Screening is not the same as approval, and approval is not the same as money already available for a residence contract.

Avoid the double count

If Maria withdraws $1,000 from her IRA, the family cannot count the same $1,000 as both untouched savings and new outside income. The withdrawal converts an asset into cash flow. The runway must reduce the account balance and recognize taxes or withholding.

Tax-aware coordination is not do-it-yourself tax planning

Income sources interact. The IRS explains that other income is considered when determining whether Social Security benefits are taxable.[18] Pension and annuity payments may be fully or partly taxable depending on how the contract was funded.[19] Retirement-account owners may also face required minimum distribution rules that depend on account type, age, and other facts.[20]

The practical lesson is not to memorize tax formulas. It is to pause before changing withdrawals, withholding, ownership, or payment elections—and to bring the full income inventory to a qualified tax or financial professional.

Coordinate five moving parts

- **Timing.** When does each payment begin, arrive, adjust, or stop? Does the first payment cover prior months or only future months?
- **Taxes and withholding.** A larger gross distribution may not create the same increase in spendable income. Ask what tax forms and estimated payments may follow.
- **Benefits.** A withdrawal or income change may affect a means-tested program. Check the program's current rules before acting.
- **The spouse at home.** Ellen's income plan had to fund Robert's care without casually redirecting money needed for her own housing, insurance, food, and taxes.
- **Authority.** Confirm who may obtain information or make an election. A family relationship alone does not create authority over an account or contract.

REMEMBER THIS

The best income plan is not the one with the largest gross number. It is the one the family can verify, spend, and sustain after taxes and benefit interactions.

What Anne refused to count

Anne listed Ruth's possible VA benefit as pending, with no amount in the dependable column. She recorded the application question, evidence still needed, and the person responsible. Hope remained in the plan. Imaginary cash did not.

The One-Page Income Source Inventory

Working inventory — verify every entry and date the page

Person: _____ As of: _____ Next review: _____

SOURCE / OWNER	AMOUNT / FREQUENCY	PROOF • DEDUCTIONS • STATUS • NEXT STEP
SOCIAL SECURITY	Gross $_____ Net $_____ / mo.	Own, spouse, or survivor record? Benefit letter date: _____ Medicare/tax withholding: _____
PENSIONS	Gross $_____ Net $_____ / mo.	Employer/plan and owner: _____ Survivor terms verified? _____ Contact: _____
ANNUITIES	$_____ ☐ mo. ☐ qtr. ☐ yr.	Company/contract: _____ Fixed/variable; term/life; guarantees: _____
RETIREMENT WITHDRAWALS	$_____ ☐ regular ☐ RMD ☐ other	Account/owner: _____ Tax withholding and adviser question: _____
PUBLIC BENEFITS / COST OFFSETS	$_____ value ☐ cash ☐ cost reduction	Program and eligibility date: _____ Approved/paid, pending, or possible? _____
OTHER VERIFIED INCOME	$_____ ☐ mo. ☐ qtr. ☐ yr.	Rent, royalties, wages, trust, or other: _____ Proof, duration, deductions: _____

TOTAL DEPENDABLE NET MONTHLY INCOME: $_____ Do not include pending or merely possible amounts.

PRIVATE COPY Do not place full account numbers, Social Security numbers, passwords, or PINs on a shared worksheet.

What Maria did next

Maria, Elena, and Mark replaced one unexplained total with six labeled rows. It became reliable enough to use in the runway—and the former-employer search remained open.

Bottom Line

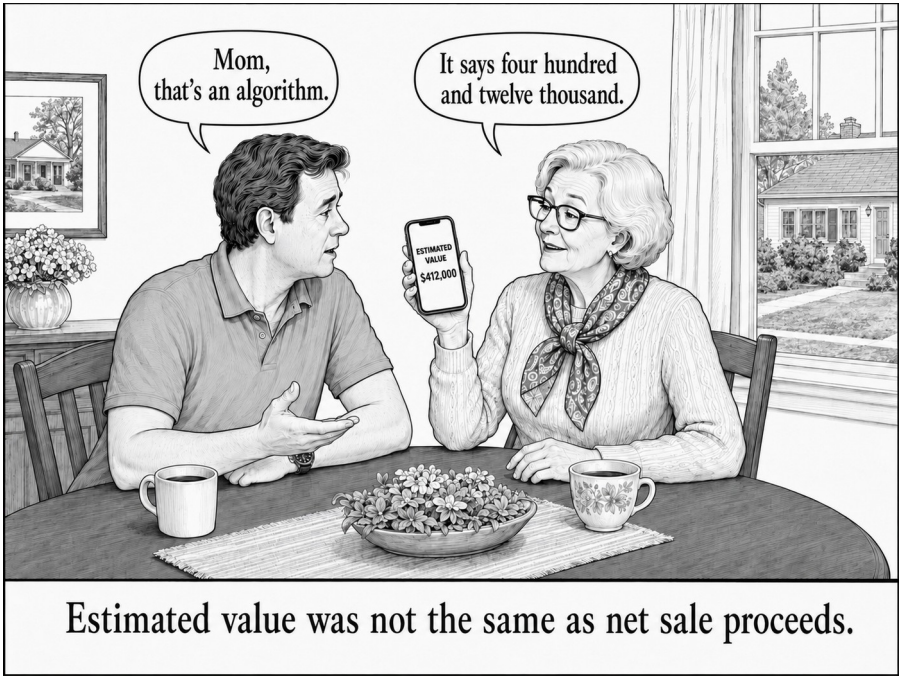
Trace every deposit and former employer. Separate gross from net, confirmed payments from possibilities, and income from asset withdrawals or cost reductions. Verify survivor and pension terms. Before changing withdrawals or elections, check taxes, benefits, the spouse at home, and legal authority.

CHAPTER 7

The House: Keep, Sell, Rent, or Borrow?

Maria held up her phone as if it were a winning lottery ticket. The screen estimated her condominium at $412,000. Mark, who had spent the morning studying recent listings, reminded her that the number came from an algorithm.

The estimate was a useful clue. It was not a buyer's offer, an appraisal, or the amount Maria could spend on care.



A home can be four things at once: a place someone lives, a major asset, a possible source of cash flow, and collateral for a loan. It can also carry debt, taxes, insurance, repairs, association fees, memories, legal restrictions, and benefit consequences.

The family's job is not to defend one choice on principle. It is to decide what work the house must do, for whom, for how long, and at what risk.

First decide what job the house must do

Elena wrote four verbs across a legal pad: keep, sell, rent, borrow. None was automatically wise. Each answered a different problem.

- **Keep.** Preserve housing for Maria or a spouse, buy time, or avoid a rushed transaction. The family must still fund taxes, insurance, association charges, utilities, security, and repairs—possibly while also paying for care elsewhere.
- **Sell.** Turn the property into liquid funds and end most carrying costs. The tradeoffs include preparation, closing costs, taxes, moving, timing, and the permanent loss of the home as housing.
- **Rent.** Retain ownership and seek recurring revenue. The family becomes a landlord, directly or through a manager, and accepts vacancy, repair, tenant, insurance, tax, and local-rule responsibilities.
- **Borrow.** Use equity without immediately selling. A loan creates a lien, costs, conditions, and repayment risk. It may solve a timing gap but can also move the family's deadline forward.

Collect five facts before comparing

- **Ownership and authority.** Confirm the deed, mortgage, liens, trust terms, and who has legal authority to sign. A child's willingness to help is not authority to sell or borrow.
- **Occupancy.** List everyone living there and anyone expected to return. A spouse's housing cannot be treated as an unused family asset.
- **Market evidence.** Use more than an online estimate: recent comparable sales, property condition, professional opinions, and realistic time to close.
- **Carrying cost and condition.** Annualize every recurring cost and identify repairs that cannot be postponed.
- **Legal and benefits context.** Before transferring, selling, or encumbering the home, check the current consequences for taxes, Medicaid, estate recovery, and any spouse who remains in the community.

> REMEMBER THIS
> **A home value is not care money until the family calculates what the decision produces after debt, costs, delay, taxes, and continuing obligations.**

Maria's $412,000 number therefore moved into the worksheet under "unverified estimate." It did not enter the bank-balance column.

Sell: plan with net proceeds, not the headline

Elena asked a local real-estate professional for a range based on the condominium's condition and recent comparable sales. She also requested written estimates for the work needed before listing. Only then could the family build a sale scenario.

THE SALE CALCULATION

Net sale proceeds = actual sale price – mortgage and liens – preparation and repairs – selling and closing costs – taxes and other reserves

For illustration, suppose Maria's condominium sold for $398,000—not the online estimate. The fictional calculation might look like this:

- **Sale price:** $398,000.
- **Mortgage payoff:** minus $46,000.
- **Repairs and preparation:** minus $12,000.
- **Selling, closing, and moving costs:** minus $26,000.
- **Tax and other reserve:** minus $8,000.

Working net proceeds: $306,000. Every figure is fictional and rounded. The tax reserve is not a tax opinion. Actual costs, gain, exclusions, basis, filing status, and documentation require current professional review. IRS Publication 523 explains federal rules for selling a home and the conditions that may allow eligible taxpayers to exclude part of the gain. [21]

Price is only one sale term

The family also estimated the listing date, likely closing date, carrying costs until closing, what would happen if the buyer requested repairs, and who would empty the home. Maria identified the possessions she would choose herself. Elena arranged help for everything else. A fast sale that overwhelms the owner can be financially efficient and personally brutal; both facts belong in the plan.

No one promised Meadowbrook that the sale would close by a certain date. Until the funds actually arrived, the family treated them as pending—not dependable.

Rent: the rent check is revenue, not profit

Mark found listings near $2,800 a month and called the condominium "passive income." Elena made a second list: association rental restrictions, landlord insurance, management fees, vacancy, repairs, taxes, recordkeeping, and the leaking water heater that had never respected anyone's spreadsheet.

THE RENTAL CALCULATION

Net rental cash flow = collected rent – vacancy – management – taxes, insurance, and association costs – repairs and capital reserve – owner-paid services

Renting may work when the property is suitable, the numbers remain positive after a realistic reserve, and someone can manage it. It is weaker when the care plan depends on every rent check arriving on time or when one major repair would consume several months of income.

Answer the landlord questions in writing

- **Who will manage?** Name the person or company responsible for marketing, screening, leases, collection, repairs, emergencies, inspections, and notices.
- **What will it really cost?** Obtain insurance and management quotes, review association and local rules, and fund a vacancy and repair reserve.
- **How will records be kept?** Separate rent collected from money transferred between family accounts. Preserve invoices, statements, contracts, and tax documents.
- **What is the exit plan?** Set the events that would trigger a sale: repeated vacancy, an unaffordable repair, a benefit change, or the need for liquid care funds.

IRS Publication 527 discusses rental income, expenses, depreciation, and reporting rules.[22] Converting a former residence to rental use can also complicate a later sale, so the family should coordinate the decision with qualified tax and real-estate professionals before the first tenant moves in.

Maria listened to the list and looked at Mark. "Passive," she said, "appears to have a large staff."

Borrow: equity is collateral, not free money

Borrowing can create cash without an immediate sale, but the house secures the debt. The correct comparison includes fees, interest, required payments, changing rates, occupancy rules, and what happens if the plan lasts longer than expected.

Home-equity loan

A home-equity loan generally provides a lump sum, often with a fixed interest rate and scheduled payments. The Consumer Financial Protection Bureau warns that the home is collateral and foreclosure is possible if the borrower cannot repay.[23] This structure may fit a defined, affordable need; it is dangerous when monthly payments depend on uncertain future funds.

Home-equity line of credit

A HELOC is a revolving line secured by the home. The borrower may draw, repay, and borrow again during the allowed period. Rates and payments may change, and the home remains at risk if repayment fails.[24] Families should examine the draw period, repayment period, rate formula, fees, minimum payment, freeze or reduction provisions, and worst-case payment—not merely the initial rate.

Reverse mortgage

A reverse mortgage is also a loan, not a benefit or sale. The federally insured Home Equity Conversion Mortgage program is generally for homeowners age sixty-two or older. HUD-approved counseling is required. At least one borrower must maintain the property as a principal residence, and obligations such as property taxes, insurance, and maintenance continue.[25]

A move to assisted living can therefore be central to the analysis. Loan documents, the length and reason for an absence, and the status of a co-borrower or non-borrowing spouse can affect whether the loan remains in good standing or becomes due. Before relying on reverse-mortgage proceeds, the family should meet with a HUD-approved counselor and obtain legal and financial advice that addresses the actual household.

BORROWING GUARDRAIL

If the plan works only while rates stay low, the home needs no repairs, and the owner returns from care on schedule, it does not yet have a safe stress test.

Elena left every borrowing option in the “possible” column until the family had written terms, verified authority, and a repayment or exit plan.

The spouse at home changes the assignment

When Ellen's nephew suggested selling the house to pay for Robert's care, Ellen answered from the kitchen where she still lived. The property was her housing, community, and monthly budget—not merely Robert's funding source. Any plan had to protect her occupancy and examine current spousal rules.

Medicaid questions must be separated

Families often hear that a home is "exempt" and assume the discussion is finished. It is not. Eligibility treatment, fair-market-value transfer rules, liens, spouse protections, and estate recovery are separate questions. Illinois warns that giving away or selling property below fair market value can affect eligibility for specified long-term-care services.[26]

Federal law requires states to seek recovery of certain Medicaid payments from estates of certain recipients, subject to protections and exceptions.[27] Illinois may pursue liens or estate claims in covered cases, will not recover more than it paid, and recognizes circumstances in which recovery is delayed, barred, or waived.[28] Favorable treatment during life does not eliminate the need for a later estate-recovery analysis.

Selling changes the asset from a home to cash, which may alter a future benefits analysis. When Medicaid may be part of the plan, obtain Illinois elder-law advice before changing title, selling below value, or spending proceeds.

REMEMBER THIS

An exempt home is not automatically an untouchable home. Eligibility, transfer rules, liens, spouse protections, and estate recovery are separate questions.

The House Decision Sheet

- **Verify:** deed, authority, occupants, debt, insurance, restrictions, condition, and carrying cost.
- **Calculate:** net sale proceeds; net rental cash flow; and every loan's costs, payments, conditions, and exit risk.
- **Protect:** lawful occupants, tax records, benefit eligibility, estate-recovery advice, reserves, and decision time.
- **Record:** the house's job, decision owner, reviews needed, next action, and review date.

Bottom Line

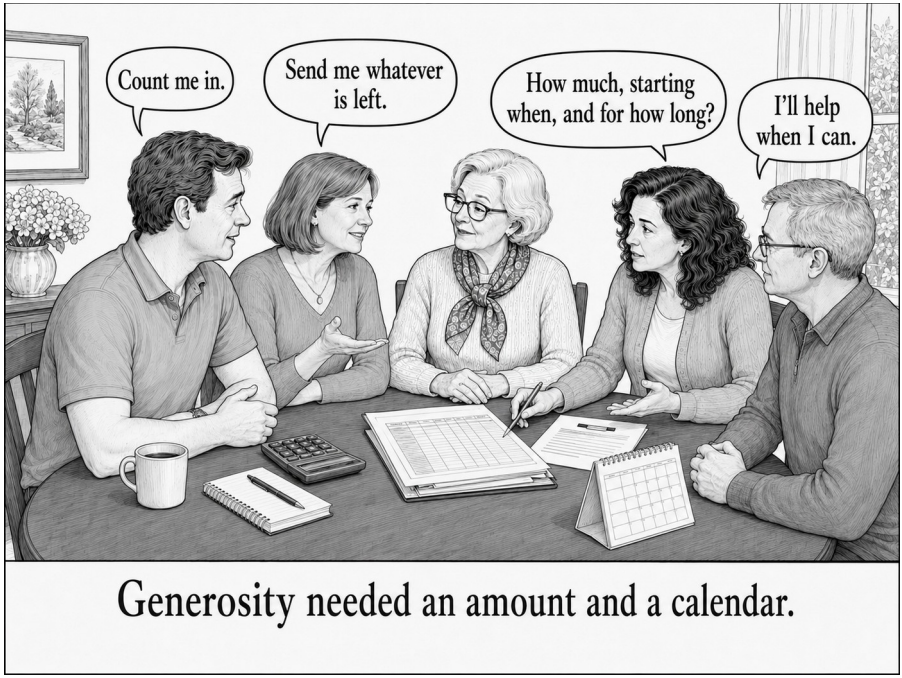
Keep, sell, rent, and borrow are tools—not verdicts. Verify ownership, occupancy, condition, debt, and carrying cost. Compare net results under realistic delays and risks. Protect the spouse at home. When Medicaid may matter, analyze eligibility, transfers, liens, and estate recovery before changing title or turning the home into cash.

CHAPTER 8

Insurance and Family Funding

After Chapter 7, Elena knew what Maria's condominium might contribute. Now the family needed to identify money that could arrive without selling another asset. Mark began with a promise: "Count me in." Two relatives followed with equally generous variations.

Elena looked at the Meadowbrook invoice, then at the calendar. Love was present. The payment schedule was not.



This chapter examines two different bridges. Insurance is a contract: it pays only under its written terms and claim procedures. Family funding is an agreement: it works only when people define the amount, purpose, timing, limits, records, and response to change.

Neither bridge belongs in dependable income merely because a policy exists or a relative means well. First convert possibility into documented, usable funding.

Long-term-care insurance: read the contract before the budget

Maria had paid premiums for years. Elena's first call was not "How much is the policy worth?" It was "What must happen before this policy pays for this residence?" Benefit triggers, elimination periods, covered services, and payment methods vary by policy.[29]

Build a one-page claim map

- **Policy status and authority.** Confirm the policy is in force, premiums are current, and the caller may receive information or file a claim.
- **Benefit trigger.** Find the exact physical or cognitive standard and required certification or plan of care. A diagnosis alone may not satisfy the contract.
- **Covered setting and provider.** Ask in writing whether the specific community, services, and provider type qualify—not merely whether a brochure mentions assisted living.
- **Elimination period.** Determine when counting starts, which days count, and what the family must pay before benefits begin.
- **Payment design.** Record reimbursement, indemnity, or cash terms; limits; inflation adjustments; remaining benefits; exclusions; and proof.
- **Claim workflow.** Name the forms, assessment, invoices, submission route, follow-up owner, appeal rights, and whether benefits can be assigned.

Elena budgeted Meadowbrook at full price until the claim and payment timing were confirmed. Reimbursement can create a cash-flow gap before covered expenses are repaid.

> REMEMBER THIS
> **A policy is not monthly income. It becomes usable funding only after the family verifies the trigger, setting, waiting period, benefit amount, claim status, and payment timing.**

Life insurance may contain living options

Term insurance usually does not build cash value. Some permanent policies permit withdrawals, surrender, or loans; riders may offer living benefits.[30] A life settlement transfers a policy to a third party for less than the expected death benefit.[31]

Options can affect premiums, loans, cash value, death benefits, beneficiaries, taxes, government benefits, privacy, or lapse risk. Anne requested an in-force illustration and written figures for Ruth's policy. She acted only after appropriate insurance, legal, and tax review explained what would remain.

Family funding needs terms before money moves

At the next meeting, Mark replaced “Count me in” with $600 on the first of each month for twelve months, paid directly to Meadowbrook, with a six-month review. The number was smaller than Elena had imagined—and real enough to budget.

Write down seven decisions

- **Who and how much.** Name each contributor, amount or service, payment method, start, review, end, and maximum commitment.
- **What it pays.** Assign the contribution to a stated purpose and decide where payment is sent.
- **Gift, loan, or services.** Define the legal character. Loans need repayment terms; caregiver pay needs duties, hours, rate, and records.
- **Authority and records.** Identify who may sign, approve, reconcile, receive statements, and preserve proof.
- **Change procedure.** Address missed payments, price increases, hardship, death, a move, benefit decisions, and disagreements.
- **Privacy and fairness.** Share enough for accountability without circulating account numbers or medical details. Capacity to contribute may differ.
- **Exit without ambush.** Require agreed notice so the care plan changes before a bill is missed.

REMEMBER THIS

Family generosity should be specific enough to survive a tired month: amount, purpose, start, finish, records, and what happens when someone cannot continue.

Paying a family caregiver is not merely “helping out”

Ellen wanted to pay a niece who was leaving weekend work to help Robert. Their agreement described services, schedule, rate, time records, substitutes, reimbursements, and termination—and required review before payment.

Caregiver and household-worker arrangements can create reporting or employment-tax questions.[32] Illinois Medicaid may examine transfers below fair market value when long-term-care eligibility is later requested.[33] A contract does not guarantee a tax or Medicaid result, but undocumented cash is harder to explain.

Unpaid work also deserves names: transportation, claims, visits, bookkeeping, home oversight, and emergency coverage. The family assigned each task so Elena’s invisible work did not become the default plan.

The Family Funding Agreement

Working summary — have an Illinois attorney and tax professional review when appropriate

DECISION	WRITE THE AGREEMENT HERE
PEOPLE & AUTHORITY	Care recipient: _____ Decision-maker: _____ Contributors and contact information: _____
CONTRIBUTION	Person: _____ $_____ ☐ one-time ☐ monthly ☐ other _____ Start: _____ End/review: _____ Maximum commitment: $_____
PURPOSE & PAYMENT	☐ residence ☐ insurance premium ☐ caregiving ☐ other _____ Paid directly to: _____ Due date/method: _____
LEGAL CHARACTER	☐ gift ☐ loan ☐ payment for documented services ☐ to be reviewed Repayment, interest, receipts, and signatures needed: _____
CAREGIVER SERVICES	Duties/hours/rate: _____ Timesheet, payroll/tax, insurance, backup-care owner: _____
IF SOMETHING CHANGES	Missed payment, price increase, hardship, death, move, or benefit decision: _____
RECORD & REVIEW	Document storage: _____ Monthly reconciler: _____ Next family review: _____ Professional review: _____

KEEP THE PROOF Preserve signed agreements, invoices, receipts, canceled checks, timesheets, insurer correspondence, and benefit statements. Never place passwords or complete account numbers on a shared worksheet.

What the three families counted

Maria counted Mark's signed twelve-month contribution only for the months promised. Anne kept Ruth's life-policy option at zero until written figures and advice arrived. Ellen recorded her niece's approved services as an expense—not as free family labor. Each plan became more modest and more dependable.

Bottom Line

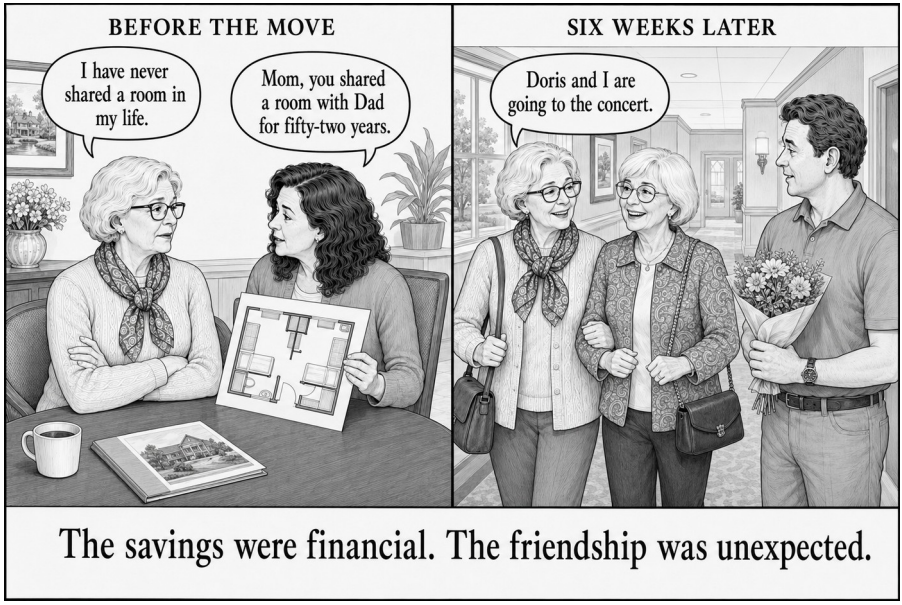
Read insurance contracts line by line and map the claim before counting benefits. Obtain written figures before changing life insurance. Convert family promises into defined contributions, document caregiver work, and review tax, employment, legal, and Medicaid consequences before money moves. A smaller amount that is verified and sustainable is more useful than a generous promise the budget cannot schedule.

CHAPTER 9

Lower the Cost Without Lowering Safety

Maria rejected the shared room before Elena finished describing it. Privacy mattered. So did the possibility of stretching Maria's runway. The family agreed to examine the option without treating price as the only fact.

Six weeks after the move, Mark arrived with flowers and discovered that Maria and Doris already had plans.



The cartoon shows one good match, not a guarantee. A shared room is safe savings only when both residents' needs, routines, privacy, personalities, communication, and supervision requirements can coexist. The family should see the actual room, meet the proposed roommate when possible, understand how belongings and visitors are handled, and ask what happens if the match fails.

Cost reduction is not a contest to find the lowest number. It is the disciplined removal of expenses that do not protect care, safety, dignity, or the family's ability to sustain the plan.

Negotiate the price that can move

Elena returned to Meadowbrook with the written quote and contract—not a vague request for a discount. Illinois rules require assisted-living contracts to identify the base rate and included services, additional services and fees, fee schedules, and the process for modifying the contract.[8] That gave her a clean list of questions.

- **One-time charges.** Ask whether the community, administrative, assessment, or move-in fee can be reduced, waived, credited, or divided. A facility may say no; the family is still entitled to understand the charge before signing.
- **Timing and availability.** Ask whether room location, move-in date, a shared unit, or a longer commitment changes the price. Do not accept a deadline that prevents contract review or a safe discharge plan.
- **Rate protection.** Request the recent written rate history, next expected review date, notice terms, and any written introductory-rate or rate-lock conditions.
- **Included versus optional.** Remove or decline only what is truly optional. Confirm that a lower package does not quietly remove transportation, meals, response services, supplies, or activities the resident will actually use.
- **Outside providers.** If the contract permits a third party, compare the total cost, coordination burden, insurance, scheduling, and responsibility when that provider does not arrive.

Choose the right care tier—not the cheapest tier

A lower tier is not a savings if staff cannot safely manage medications, transfers, bathing, nighttime needs, wandering risk, or changes in cognition. An unnecessarily high tier can also drain funds. Elena asked Meadowbrook to connect every charge to Maria's written assessment and service plan.

- **Match.** For each billed service, identify the assessed need, frequency, staff response, and documentation. Ask which services are bundled and which are individually charged.
- **Reassess.** Learn what triggers a new assessment, who participates, how the family receives notice, when a new price begins, and how a decision can be questioned.
- **Do not substitute family labor silently.** If the tier assumes Elena will provide transportation, supplies, escorts, or frequent supervision, place that time and cost in the family budget.

> **REMEMBER THIS**
> **Negotiate fees, timing, room type, and optional amenities. Never negotiate away the care that makes the placement safe.**

A cheaper ZIP code can carry a family surcharge

Mark found a lower price farther away. After adding travel, missed work, emergencies, winter driving, and fewer visits, the family kept it as a backup—not a price-only winner.

Screen for benefits before calling the gap permanent

A benefit may lower the family's net cost without changing the residence price. Elena reopened the Chapter 4 snapshot and screened every source systematically: long-term-care insurance, employer and union retiree benefits, public programs, military history, survivor records, and programs that reduce medical, premium, food, or prescription expenses.

Illinois uses the ABE system for applications involving Medicaid, SNAP, cash assistance, and Medicare Savings Programs; its screening tool can indicate possible eligibility but is not an approval.[34] VA Aid and Attendance or Housebound payments are additions to qualifying VA pension benefits for eligible veterans or survivors.[35] Each program has its own person, financial, care, provider, documentation, and timing rules.

Use three benefit columns

- **Approved and received.** Use the verified net amount and payment schedule in the dependable-income calculation.
- **Approved but not received.** Record the award, expected timing, follow-up owner, and any bridge needed before payment begins.
- **Possible or pending.** Pursue it, but do not let an unapproved amount finance a binding residence contract.

> REMEMBER THIS
> **The lowest monthly quote is not always the lowest safe cost. Compare care, family logistics, likely price changes, and only those benefits the family can verify.**

The Safe Savings Screen

- **Written savings:** What exact charge falls, by how much, for how long, and under what contract term?
- **Safety effect:** What service, staffing, supervision, privacy, transportation, or response changes with the lower price?
- **Family effect:** What new work, travel, missed income, supplies, or emergency responsibility shifts to relatives?
- **Failure plan:** What happens if the roommate match fails, the care tier rises, the introductory rate ends, or a pending benefit is denied?
- **Decision record:** Keep the quote, contract section, assessment, comparison date, decision-maker, next review date, and unresolved questions together.

What Maria chose

Maria chose the shared room after meeting Doris and reviewing the exit terms. Elena negotiated part of a one-time charge, kept the assessed care tier, and declined an optional upgrade Maria did not value. Mark's distant community remained the backup—not the winner of a price-only contest.

Bottom Line

Remove negotiable costs—not safe care. Count only verified benefits and choose a sustainable plan that works on a difficult night.

PART III

Illinois Medicaid without myths

CHAPTERS 10–15

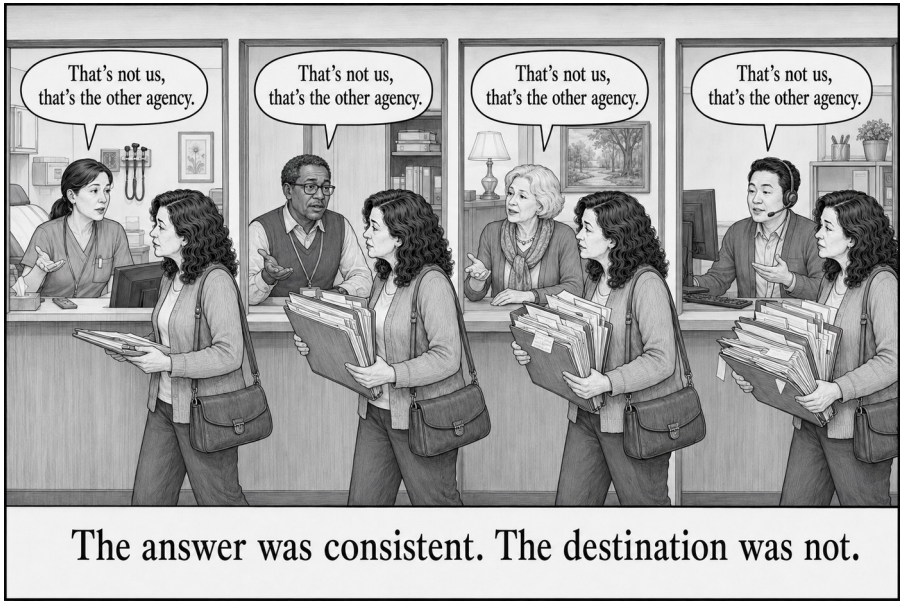
Replace the single word "Medicaid" with the exact Illinois program, setting, agency, eligibility pathway, and decision the family actually needs.

CHAPTER 10

Medicaid Is Not One Program

Elena began with what sounded like one question: “Will Medicaid help my mother?” By lunchtime she had spoken with a hospital office, a benefits caseworker, an aging-services counselor, and a program representative. Each understood a different piece of the question.

By the fourth referral, Elena’s folder had become a portable filing cabinet.



The professionals were not necessarily contradicting one another. “Medicaid” can mean ordinary medical coverage, payment for nursing-facility services, a home-and-community-based waiver, the Illinois Supportive Living Program, financial eligibility, a needs screening, an authorization, or a participating provider.

The word becomes useful only after the family adds a noun and a setting: Medicaid coverage for which service, through which pathway, in which place, decided by whom?

> **REMEMBER THIS**
> **Never ask only, “Does Medicaid cover this?” Ask for the exact program, service, setting, decision-maker, and next required step.**

This chapter supplies the map. Later chapters explain assets, spend-down, transfers, applications, and estate recovery.

The Illinois Agency Map

The office that answers one question may not control the next decision.

WHO	WHAT IT GENERALLY DOES	WHAT IT DOES NOT DECIDE
HFS	Illinois state Medicaid agency: program policy, administration, payment, and oversight; administers SLP and HCBS waivers.[36]	It is not the local financial caseworker, every functional assessor, or the facility licensor.
DHS / FCRC	Receives applications, requests verification, processes case eligibility, issues notices, and handles case status through ABE/FCRC channels.[37]	It does not decide whether a particular residence can safely meet the person's needs.
IDoA / CCU	For older adults, coordinates needs screening, care assessment, choices counseling, and programs including the Persons who are Elderly waiver.[38]	It does not replace the financial Medicaid determination.
DHS-DRS and other operating agencies	Assess and coordinate waiver services for specified disability or program populations.[39]	They are not one universal pathway for every older adult or disability.
IDPH	Licenses and regulates assisted-living establishments and nursing facilities and receives certain complaints.[3]	It does not approve Medicaid income, assets, transfers, or waiver enrollment.
Facility / health plan	Confirms participation, availability, care capability, service plan, authorization, billing, and practical admission requirements.	It cannot promise the State's final eligibility decision.

Why the referrals keep happening

HFS is the state Medicaid agency, but different agencies and contractors perform parts of the work. DHS/FCRC case processing can move separately from an IDoA or DHS needs assessment. IDPH can regulate a building without deciding whether Medicaid will pay. A facility can accept one Medicaid pathway and not another—or have no appropriate opening.

Elena began ending every call with five questions: What exact decision do you control? What program name belongs on my notes? What document or assessment is missing? Who owns the next step? What written notice should I expect?

Four pathways families commonly confuse

1. Ordinary Medicaid medical coverage

A person may have an HFS medical card and coverage for ordinary health services. That does not automatically establish eligibility for a nursing-facility stay, waiver services, or supportive living. HFS itself cautions that a medical card does not guarantee coverage for a particular service or time; providers verify eligibility.[37]

2. Institutional Medicaid for nursing-facility services

This is the long-term-care pathway families often mean when they say “nursing-home Medicaid.” The applicant must complete the Medicaid financial process and the required needs screening. The nursing facility must participate and be able to provide the required care. Approval for other medical benefits is not the same as approval of long-term-care payment.[38]

3. Home and Community-Based Services waivers

HCBS waivers permit specified services in a home or community setting as an alternative to institutional care. Illinois has multiple waivers for different populations. HFS states that waiver eligibility includes the financial criteria for that waiver, the required institutional level of care, cost and safety conditions, and program-specific requirements such as age or diagnosis.[39]

For the Persons who are Elderly waiver, the pathway includes Medicaid eligibility and a Determination of Need assessment showing risk of nursing-facility placement, plus a service plan that can safely maintain the person in the community.[40] A waiver is not a general promise to pay any caregiver, apartment, or assisted-living bill.

4. Illinois Supportive Living Program

SLP is Illinois’s distinct Medicaid-waiver alternative to nursing-home care at approved supportive-living providers. Medicaid pays specified services such as personal care and medication assistance; the resident remains responsible for room and board.[41] A private assisted-living community that says “we do not take Medicaid” may be answering a different question from an SLP provider that participates in this pathway.

REMEMBER THIS

A Medicaid approval does not make every residence a Medicaid provider, and a participating provider does not make every applicant eligible.

Medical and financial eligibility are separate doors

Ellen assumed Robert's dementia diagnosis proved eligibility. Mark assumed Maria's shrinking savings proved eligibility. Each had only half of the analysis. Long-term-care Medicaid pathways commonly require separate findings that must eventually meet in the same case.

The medical, functional, or programmatic decision

This side asks what help the person needs and whether the pathway's service criteria are met. Depending on the program, the decision may involve activities of daily living, cognition, supervision, level of care, age, diagnosis, safety in the proposed setting, a plan of care, and whether community services can meet the need. A physician's diagnosis can be important evidence without being the entire test.

The financial decision

This side applies the financial rules for the exact eligibility category: income, resources, ownership, availability, transfers, household or spousal rules, and required verification. Being unable to afford private care does not by itself prove Medicaid financial eligibility. The following chapters explain the rules and documentation without relying on changing annual limits here.

The provider and authorization decision

Even after both eligibility doors open, the family still needs a provider that participates in the correct pathway, has appropriate availability, can safely deliver the authorized services, and completes required enrollment or billing steps. A waitlist, service limit, or care mismatch can delay the practical result without changing the person's underlying eligibility.

THE TWO-DOOR TEST

Financially eligible + not functionally eligible = no long-term-care service approval. Functionally eligible + not financially eligible = Medicaid does not fund the service. Both decisions must match the same pathway and time period.

Run the tracks in parallel

- **Financial track.** Submit the correct application, identify long-term-care or waiver services, preserve the tracking number, answer verification requests, and read every notice and deadline.
- **Needs track.** Schedule the correct screening or assessment, provide accurate care information, identify the intended setting, and obtain the result or missing step in writing.
- **Provider track.** Confirm program participation, care capability, availability, room-and-board responsibility, required authorization, and who transmits admission or service information.

The Medicaid Route Sheet

Use program names, dates, and written notices—not "someone said Medicaid."

QUESTION	FAMILY'S CURRENT ANSWER
PERSON & AUTHORITY	Applicant: _____ Representative: _____ Authority document and contact: _____
SETTING NEEDED	☐ nursing facility ☐ supportive living ☐ home/community ☐ other _____ Current location and safe-until date: _____
EXACT PROGRAM	Program/pathway named by the agency: _____ Covered services—and room-and-board responsibility: _____
FINANCIAL CASE	ABE/tracking/case number: _____ Application date: _____ Caseworker/FCRC: _____ Verification deadline: _____
NEEDS SCREENING	Agency/assessor: _____ Date: _____ Result or missing step: _____
PROVIDER / PLAN	Participating provider or plan contact: _____ Availability, authorization, and care-fit status: _____
WRITTEN NOTICES	☐ pending ☐ approved ☐ denied ☐ more information requested Notice date: _____ Effective date/deadline: _____ Appeal noted: ___
NEXT ACTION	Owner: _____ Action: _____ Due: _____ Next follow-up and document-storage location: _____

What Elena wrote at the top

For Maria, the immediate question became: "Is she financially and functionally eligible for Illinois Supportive Living, at a participating provider that can safely meet her needs, and what will remain her room-and-board responsibility?" That sentence finally produced useful answers.

Bottom Line

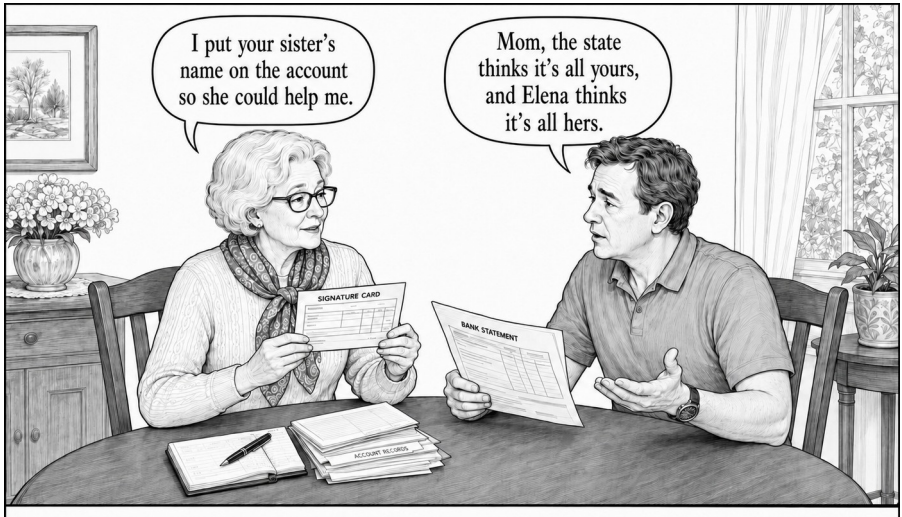
Medicaid is a family of pathways, not a single assisted-living payment. Identify the exact program and setting. Track the financial case separately from the needs assessment, then confirm the participating provider and authorization. Keep every notice, deadline, case number, assessor, and next step on one route sheet. Precision is the cure for the referral loop.

CHAPTER 11

Countable and Noncountable Assets

Maria remembered adding Elena to the checking account. She did not remember whether the bank had called Elena an owner, an authorized signer, a payable-on-death beneficiary, or something else. “She could pay the bills if I was sick,” Maria said. “That was the whole idea.”

Mark read the names printed across the statement and announced his interpretation. Elena read the same names and reached a different conclusion.



One account. Three ideas about who owned it.

The cartoon exaggerates Elena’s claim for the joke. The real problem was serious: purpose, family memory, bank access, legal ownership, and Medicaid classification are not interchangeable. A name on paper can create rights or presumptions the family never intended.

For Illinois Medicaid, the useful question is not merely “What does Maria own?” It is: What resource does she own or have power to make available, what is its verified value on the relevant date, does a rule exclude all or part of it, and can the family prove every answer?[42]

Start with ownership, access, value, and date

Illinois defines a resource broadly as cash or other personal or real property a person owns and has the right, authority, or power to liquidate. A resource is available when it is at the person's disposal or the person has the legal power to make it available for support, maintenance, or care.[42] Four facts therefore belong beside every asset:

- **Ownership.** Whose legal interest appears on the account, deed, title, policy, contract, or registration?
- **Access.** Who can withdraw, surrender, sell, borrow, close, or compel payment—and under what restriction?
- **Value.** What is the verified cash, market, surrender, or equity value after applicable debt?
- **Date and pathway.** Which eligibility month and exact Medicaid category are being tested? A balance or exemption can change.

Cash and ordinary accounts are usually the easy part

Cash, checking, savings, certificates of deposit, money-market accounts, and accessible investment accounts generally enter the resource analysis unless a specific exclusion applies. Money received as income in a month is not counted again as a resource for that same month; amounts retained into a later month may become resources under the Illinois rule.[42] Statements must therefore show dates and deposits, not merely an ending balance.

Joint ownership is not a family explanation

Illinois generally begins by treating the entire equity value of a jointly held resource as available, subject to stated exceptions. A person may document lack of access or a legal interest smaller than the whole with bank records, signature cards, trust documents, court papers, contribution records, or similar proof.[42] "We added her for convenience" is a fact to investigate, not a classification.

REMEMBER THIS

The name of the asset does not decide the answer. Ownership, legal access, value, date, program pathway, and proof decide the answer.

Elena requested the original signature card and account agreement. Until they arrived, the snapshot said "joint account—legal interest unknown." No one withdrew money, removed a name, or moved the balance to "clean things up."

A limit is not a list of everything a person may own

As of July 2026, Illinois states that the asset limit for AABD medical cases is $17,500, whether one person or more is in the household.[43] That number applies to nonexempt resources in the identified category; it is not a universal ceiling on total property, and married-couple protections can require a separate analysis. Verify the current standard before publication and again before relying on it.

ANNUAL UPDATE

Verify the AABD medical asset limit and every indexed exemption for the year of application. Never copy an old $2,000 figure—or any single limit—into every Medicaid pathway.

Retirement labels do not create automatic protection

Families often assume that an IRA, 401(k), deferred-compensation plan, or retirement annuity is untouchable because it was created for retirement. Illinois asks for IRA and 401(k) information on its resource form.[44] The analysis turns on the exact plan, owner, present access, cash or surrender value, payment status, and eligibility category—not the word "retirement."

- **Obtain the plan document or contract.** A monthly statement rarely explains withdrawal rights, surrender terms, assignment, annuitization, loans, beneficiaries, or restrictions.
- **Separate principal from payments.** An account value may be analyzed as a resource while distributions may also affect income calculations. Do not count or characterize either without the pathway's rule.
- **Calculate consequences before acting.** Liquidation can create income tax, penalties, loss of guarantees, market loss, or transfer issues. A Medicaid classification is not an instruction to cash out.
- **Treat annuities as specialized contracts.** Illinois requires disclosure and contains detailed availability, transfer, payout, and remainder-beneficiary rules. Obtain the complete contract and qualified review before changing it.[42]

Stocks, bonds, mutual funds, and CDs

Use the verified value for the relevant date and record ownership, restrictions, maturity, liens, and selling costs. A paper certificate in a drawer, an account marked "for the grandchildren," or a CD that has not matured can still require analysis. Family purpose does not replace legal control.

The home may be exempt without being irrelevant

Illinois exempts qualifying homestead property—property in which the person or spouse has an ownership interest and that serves as the principal residence. Occupancy, intent, a spouse or dependent relative in the home, a trust, and the move to long-term care can affect the analysis. Non-homestead real estate is generally available unless a rule applies, such as specified income-producing or good-faith sale treatment.[42]

Record the deed, every owner, mortgage or lien, current equity evidence, actual occupant, move date, intent, listing history, rental income, and expenses. “Mom’s house” is not enough. A life estate, fractional interest, trust-held home, vacation property, or land contract may produce a different answer.

DO NOT RETITLE YET

Do not add a child to a deed, remove a joint owner, transfer the home, or place it in a trust merely because someone called it exempt. Eligibility, transfer rules, taxes, creditor exposure, control, and estate recovery are different questions.

One vehicle can require several facts

Illinois vehicle exclusions depend on facts such as necessary employment, medical transportation, disability modification, essential daily transportation, and spousal rules. Additional vehicles and excess equity can be treated differently.[42] Keep the title, loan payoff, value evidence, use, modifications, and the identity of the driver or dependent who needs it.

Household goods are not an estate-sale assignment

Ordinary personal effects and household goods receive exclusion treatment under the Illinois rule.[42] Families generally do not need to price every chair, winter coat, and mixing bowl. They should separately identify unusually valuable collectibles, investment property, business equipment, vehicles, precious metals, or items held for resale, because labels and purpose matter.

Exempt for eligibility does not mean protected forever

A home or other asset may be noncountable during an eligibility determination and still create carrying costs, sale decisions, tax consequences, or potential estate-recovery questions later. Illinois explains that recovery is limited and subject to exceptions, but it is a separate process from initial eligibility.[46] Chapter 15 addresses that process directly.

Life insurance has at least three different numbers

Maria's policy page displayed a death benefit large enough to alarm Mark. Elena found a second number—cash surrender value—on the in-force statement. The two numbers served different purposes. For resource analysis, the policy type, total face value, ownership, loans, assignment, and accessible cash value matter.

Under the current Illinois exemption rule, term policies are excluded. Life policies with total face value at or below the stated threshold are also excluded; if total face value exceeds that threshold, cash surrender value is counted as a resource.[42] Because policy loans, dividends, riders, assignments, and changes can alter the result, request a current in-force ledger rather than relying on the original sales illustration.

Burial space, burial funds, and funeral contracts are not synonyms

- **Burial spaces.** The Illinois rule separately exempts qualifying spaces intended for the person, spouse, or immediate family.
- **Designated burial funds.** A bank account must be clearly identified for burial, and the available exclusion interacts with excluded life insurance and irrevocable arrangements.
- **Revocable contracts.** Refund rights and identified burial-space components matter; a revocable contract is not automatically excluded in full.
- **Irrevocable contracts or assignments.** The contract, itemized goods and services, licensed provider, assignment language, residual proceeds, and annually adjusted limits can control the treatment.[42]

REMEMBER THIS

"Exempt" is a conclusion supported by a rule and documents—not a label supplied by a bank, insurer, funeral home, relative, or internet checklist.

The safest sequence

- **Request.** Obtain the complete policy or contract, current values, refund and surrender terms, assignments, beneficiary information, and itemized goods and services.
- **Classify.** Match those facts to the exact Illinois program and application month, then confirm any annually adjusted amount.
- **Review before changing.** Surrender, borrowing, assignment, beneficiary changes, or converting a contract can affect taxes, death benefits, transfers, creditor rights, and eligibility.
- **Preserve.** Keep the document and written classification with the application file so a later reviewer can reproduce the answer.

The Asset Evidence File

Collect proof before asking anyone to classify the asset. Keep complete account numbers and passwords off shared worksheets.

ASSET	KEEP THIS PROOF	CONFIRM BEFORE CLASSIFYING
CASH & BANK	All statements; signature cards; ownership forms; closed-account records	Owner; access; balance on relevant date; deposits; joint-interest evidence
CDs, STOCKS & FUNDS	Statements; certificates; registration; redemption or transfer terms	Owner; market/cash value; restrictions; penalties; maturity date
RETIREMENT & ANNUITY	Current statement; plan or contract; payout and surrender terms; beneficiary page	Access; cash/surrender value; payment status; tax and Medicaid treatment
HOME & REAL ESTATE	Deed; mortgage; tax bill; appraisal/CMA; occupancy and listing proof	Legal interest; equity; principal residence; occupant; sale or income status
VEHICLES	Title; loan balance; current value; use and modification records	Owner; equity; necessary use; number of vehicles; spouse/dependent facts
LIFE INSURANCE	Policy; current in-force ledger; face value; cash value; loans; assignment	Term or permanent; owner; insured; accessible value; assignment status
BURIAL	Contract; itemized goods/services; revocable/irrevocable terms; burial-space deed	Owner; designated use; refund rights; assignment; current exemption amount
HOUSEHOLD & PERSONAL	Inventory only for unusual items; appraisal if genuinely valuable	Ordinary personal use versus investment, business, or collectible property
JOINT OR DISPUTED	Signature card; contribution history; trust/deed; court or divorce papers	Who owns what; who can withdraw or sell; how access or legal interest is proved

DATE THE FILE Classification can change when a balance, owner, occupant, contract, accessibility term, or program pathway changes. Preserve the evidence used for each decision.

The Asset Classification Record

Complete one record for every material asset. "Unknown" is an acceptable first answer; an unsupported guess is not.

QUESTION	FAMILY'S DOCUMENTED ANSWER
IDENTIFY	Asset/institution: _____ As-of date: _____
OWNERSHIP	Legal owner(s): _____ Applicant's claimed interest: _____
ACCESS	Who can withdraw, surrender, sell, borrow, or close? _____
VALUE	Gross value: $_____ Debt/encumbrance: $_____ Equity/cash value: $_____
PATHWAY	Exact Medicaid category/setting and eligibility month: _____
CLASSIFICATION	☐ countable ☐ exempt/noncountable ☐ partly countable ☐ unavailable ☐ unknown
AUTHORITY	Rule/notice/adviser: _____ Proof stored: _____
NEXT ACTION	Do not transfer. Obtain: _____ Owner: _____ Due: _____

What Maria's signature card showed

The bank records showed that Maria had added Elena as a joint owner with withdrawal authority. Maria had supplied the money. Elena had used the account only to pay Maria's documented expenses. Those facts did not permit the family to erase Elena's name or declare the balance divided. They gave the eligibility reviewer evidence to apply the Illinois joint-resource rule—and gave counsel a precise problem to review before anyone acted.

Mark updated the snapshot: "Joint checking—full balance presumed available unless documented exception or smaller legal interest is accepted." Maria corrected his handwriting. Elena stored the signature card, contribution history, and statements behind the record.

Bottom Line

Classify assets one at a time. Identify ownership, access, verified value, date, Medicaid pathway, possible exclusion, and proof. Cash and accessible accounts are often countable; homes, vehicles, life insurance, burial arrangements, household goods, retirement funds, and joint property require their own facts. Do not transfer, retitle, surrender, or spend merely to make the list look simpler. Build the evidence file first, then obtain the decision the documents support.

CHAPTER 12

Spend-Down: What It Really Means

Elena had reduced Maria's accounts to a single total. Then she looked at the Medicaid resource standard from Chapter 11 and made the sentence every exhausted family eventually makes: "So the goal is to spend all her money."

Mark moved the calculator aside. "The goal is to spend it on her. If we do it right, that's not the same sentence."



That correction became the family's rule. Spend-down was not a shopping spree, a race to zero, or permission to move money to the children. It was a controlled conversion of Maria's countable resources into care, services, property, protection, and obligations that genuinely benefited Maria—at fair market value and with proof.

First, identify which "spend-down" you mean

Illinois uses spenddown as a formal Medicaid term when countable income or nonexempt resources exceed the applicable standard. For long-term-care eligibility, incurred or projected medical expenses may be used under the applicable rule.[47] Families also use "spend down" more loosely to mean reducing excess countable assets before an application. The two ideas overlap, but they are not interchangeable.

Before paying anything, Elena wrote four facts at the top of the plan:

- **Program and setting.** Institutional Medicaid, Supportive Living, a waiver, and ordinary medical coverage can apply different rules and timelines.
- **Target date.** Which month is the family trying to establish financial eligibility? Balances, ownership, and bills must be tied to dates.
- **Verified excess.** What countable resources remain after exclusions, spousal protections, and other applicable rules are correctly applied?
- **Decision authority.** Who may legally sign, purchase, sell, contract, or withdraw—Maria, an agent under power of attorney, a guardian, a trustee, or someone else?

The family did not begin with a dollar amount copied from the internet. They began with Maria's program, application month, marital facts, resource classification, and documents. Only then could they calculate the amount that actually needed attention.

REMEMBER THIS
Spend-down is not a command to become poor. It is a plan to use excess countable resources lawfully for the applicant's benefit while preserving care, dignity, and proof.

A last-minute plan can be especially dangerous. Illinois reviews transfers during the applicable 60-month look-back for long-term-care services, and a transfer for less than fair market value can create a penalty period rather than eligibility.[48] "We spent it" is not the answer. The file must show what Maria received in return.

Pay real obligations and meet real needs

Maria already had expenses that were neither artificial nor invented for Medicaid. Elena listed them in priority order: health and safety first, then legally enforceable debts, then improvements that would make Maria's daily life safer or more comfortable.

Debts belonging to Maria

Paying a genuine debt for full value generally differs from giving money away. The file should identify the creditor, the underlying obligation, the balance, the due date, and the payment. Examples may include credit cards used for Maria's expenses, medical bills, taxes, a mortgage, insurance premiums, or an assisted-living balance. Do not pay another person's debt from Maria's account merely because the family shares a surname.

Care and health needs

Current care can be the most direct use of Maria's money: assisted-living charges, nursing or personal care, dental work, hearing aids, eyeglasses, therapy, prescriptions, transportation to treatment, or other needed services. Coverage and eligibility treatment still depend on the exact program, but the family's first question remains practical: Does this expense meet Maria's documented need?

- **Confirm the service.** Obtain an assessment, order, care plan, estimate, or contract when appropriate.
- **Confirm the price.** Compare ordinary market rates and identify what is included.
- **Confirm delivery.** Keep invoices, dates of service, time logs, proof of payment, and any warranty or equipment serial number.

Equipment and personal property

A walker, wheelchair, lift chair, adjustable bed, communication device, clothing, computer, television, or safer furniture may improve life. But buying an object does not automatically make it noncountable or medically necessary. Record why Maria needs it, who owns it, where it will be used, what it cost, and what value it may retain. If the item is primarily for someone else, paying from Maria's funds can look like a transfer.

Improve Maria's property—without improving everyone else's balance sheet

The back steps at Maria's home were cracked, the bathroom had no grab bars, and the furnace was approaching the end of its life. Those facts made home work plausible. They did not make every remodeling idea a Medicaid strategy.

A defensible project begins with ownership and benefit

- **Ownership.** Confirm that Maria owns the property or has a documented legal interest. If she owns only part, ask who should bear the rest of the cost.
- **Purpose.** Connect the work to safety, accessibility, preservation, or Maria's reasonable use—not to a child's future resale plans.
- **Scope and price.** Use written estimates, specifications, permits when required, invoices, proof of payment, and photographs before and after the work.
- **Timing and occupancy.** A repair to a home Maria or a protected spouse occupies presents different facts from a luxury renovation to a house already promised to a child.

Fair market value is the hinge

Illinois long-term-care transfer rules focus on whether Maria received fair market value. Gifts, bargain sales, debt forgiveness, adding a name to an account or deed, and paying an inflated price to a relative may create uncompensated value. The State's own application asks for the item transferred, recipient, date, amount received, market value, proof of that value, and reason for the transfer.[49]

Buying at fair market value is not limited to medical items. Maria may buy ordinary goods and services for herself. But the purchase must be genuine: a real item or completed service, reasonable value, legal authority, and actual benefit to Maria. Converting cash into another countable asset may not reduce countable resources at all; buying an exempt or excluded item requires correct classification, not wishful labeling.

FAIR-VALUE TEST
Could a neutral reviewer identify what Maria received, why it benefited her, when she received it, and why the price was reasonable? If not, pause before paying.

Some good expenditures require specialized documents

Legal and professional services

Maria could pay reasonable fees for advice and documents that served her: Medicaid eligibility analysis, powers of attorney, estate or tax review, guardianship work, contracts, real-estate services, accounting, appraisal, or benefits advocacy. Elena requested an engagement letter naming the client, the scope, billing method, and responsible payer. A professional's invoice is not proof that every strategy is lawful; it is proof of the service purchased.

Burial and funeral planning

Preplanning can honor Maria's wishes and may receive special resource treatment, but "prepaid funeral" is not a single legal category. Illinois distinguishes burial spaces, designated funds, revocable contracts, irrevocable contracts, and life-insurance-funded arrangements. Itemization, refund rights, assignment language, provider status, residual proceeds, and current limits matter.[50] The family should obtain the complete contract and written classification before funding or changing it.

Paying a family caregiver

A child may be paid for real future care at a reasonable market rate, but informality is dangerous. Illinois presumes care previously provided by friends or relatives was gratuitous unless credible documentary evidence created before the care establishes the terms, and contemporaneous records prove delivery. Paying later for years of unbilled "love and affection," or paying more than prevailing rates, may be treated as a transfer for less than fair market value.[51]

- **Before care starts,** use a written agreement describing duties, schedule, rate, payment timing, termination, and who supervises the work.
- **As care occurs,** keep dated time logs, task records, invoices or payroll records, cancelled checks, and required tax documentation.
- **When family circumstances change,** amend the agreement prospectively. Do not rewrite history after a Medicaid question arrives.

Elena could still provide unpaid help. She simply would not later invent a retroactive bill. Chapter 8's Family Funding Agreement and this chapter's spend-down record belong in the same file.

Build the paper trail before the bank statement arrives

Medicaid reviewers do not sit at Maria's kitchen table. They see statements, transfers, checks, withdrawals, contracts, and explanations. The safest file allows a stranger to reconstruct each transaction without relying on family memory.

For every material expenditure, preserve:

- **Need and authority.** Assessment, care plan, request from Maria, power of attorney, guardianship order, or other proof that the signer could act.
- **The bargain.** Written estimate, contract, itemized scope, vendor identity, market comparison, and fair-value support.
- **The exchange.** Invoice, receipt, delivery confirmation, dates of service, time logs, photographs, warranty, title, or serial number.
- **The money path.** Account statement, cancelled check, card record, electronic confirmation, and ledger entry. Avoid unexplained cash withdrawals.
- **The Medicaid explanation.** A brief note stating what Maria received, why it benefited her, whether value remains, and which adviser reviewed any specialized treatment.

Elena assigned each expenditure a number—SD-001, SD-002, and so on—and wrote that number on the plan, invoice, receipt, and bank-statement annotation. The system was simple enough to use and strong enough to audit.

REMEMBER THIS
A receipt proves money left. A complete paper trail proves what the applicant received, why the price was fair, and why the expenditure served the applicant.

Do not manufacture documents after the fact, backdate an agreement, split one payment into smaller checks to make it look ordinary, or describe a gift as "care." If records are missing, disclose the gap and seek advice about credible reconstruction. A clean-looking fiction is worse than an honest incomplete file.

The Spend-Down Plan

List proposed transactions before making them. “Review” is a valid decision; not every idea belongs in the payment column.

Expense or purchase	Benefit to Maria	Proposed cost	Proof / fair value	Decision
Existing debt		$	Statement / obligation	Pay / Review
Care, health, or equipment		$	Plan / invoice / receipt	Pay / Review
Home repair or accessibility		$	Ownership / bids / scope	Do / Review
Legal, professional, or burial		$	Contract / classification	Hire / Review
Other		$		

Control totals

Verified countable resources: $_____ Applicable resource amount: $_____

Planned reduction: $_____ Projected remaining resources: $_____

Before the final transaction

- **Recalculate and protect liquidity.** Confirm the eligibility month and reserve enough for care, taxes, insurance, personal needs, and emergencies.
- **Review red flags.** Stop before gifts, bargain sales, retroactive caregiver payments, title changes, annuities, trusts, loans, or large related-party transactions.
- **Preserve the closing snapshot.** Keep statements showing post-transaction balances and every pending refund, reimbursement, or deposit.

Bottom Line

Spend-down is not the goal. Maria’s care, safety, dignity, and lawful eligibility are the goals. Pay real obligations, purchase real value for her, and connect every dollar to Maria with proof. If a transaction chiefly benefits someone else—or the family cannot prove what Maria received—stop and obtain Illinois-specific advice.

CHAPTER 13

The Five-Year Look-Back

Maria stood at the wall calendar and began flipping backward. One year passed, then another. The pages moved faster than the family's memory.

"How far back do they look?" she asked. Mark held the account file. "Far enough to see the thing you're about to remember."



The five-year look-back is not a rule that makes every old transaction improper. It is a review period. Illinois examines transfers by an institutionalized person or spouse for less than fair market value during the applicable 60 months, and the rule reaches long-term-care facility services and specified home- and community-based waiver services.[52] The family's first job was therefore not to defend anything. It was to reconstruct the facts.

Sixty months is a records project, not a memory test

Elena began with the month in which Maria might apply and built a month-by-month timeline backward. She did not assume that "five tax years" and "the prior 60 months" produced the same dates. The exact look-back window must be anchored to the application and long-term-care facts.

Illinois's application asks directly about transfers during the last five years, people who helped manage finances, discussions about Medicaid or nursing-home planning, trusts, reverse-mortgage proceeds, and supporting tax returns.[53] Bank statements alone are rarely the complete file.

- **Accounts and cash.** Checking, savings, certificates, brokerage accounts, electronic-payment services, cash withdrawals, cashier's checks, and closed accounts.
- **Property and ownership.** Deeds, vehicles, business interests, life estates, trusts, annuities, loans, beneficiary changes, and additions or removals of names.
- **Income and rights.** Gifts of income, waived inheritances, forgiven debts, redirected payments, uncollected loans, and assets the person was entitled to receive but did not pursue.
- **People and authority.** Actions by Maria, her spouse, an agent, guardian, trustee, joint owner, or another person acting at their direction can require review.
- **Purpose and value.** For every unusual movement, record what left, what Maria received, the recipient, date, market value, reason, and proof.

The Illinois rule allows the Department to request records needed to determine whether and to what extent property was transferred. It also calls for reasonable measures that consider the age, significance, relevance, difficulty of obtaining records, and the applicant's condition and capacity.[54] Missing records should be identified and pursued—not replaced with confident guesses.

REMEMBER THIS
The State reviews transactions, not family labels. "Gift," "loan," "help," "inheritance," "joint account," and "payment for care" are starting descriptions—not legal conclusions.

The issue is uncompensated value

A transfer becomes dangerous when Maria gives up an asset, income, ownership right, or enforceable claim and receives less than roughly equivalent tangible value in return. Illinois defines uncompensated value as the fair market value of the transferred asset, after applicable encumbrances, minus the compensation actually received.[52]

Common examples that require analysis

- **Outright gifts.** Checks to children or grandchildren, tuition, charitable gifts, holiday cash, or paying another person's bills from Maria's funds.
- **Bargain transactions.** Selling a car, house, stock, or business interest below fair market value; forgiving a loan; or accepting less than the balance owed.
- **Ownership changes.** Adding someone to a deed or account, changing title, creating a life estate, placing assets in some trusts, or surrendering control without receiving equivalent value.
- **Action or inaction.** Waiving pension income, disclaiming an inheritance, declining a settlement, or failing to pursue an asset may count when Maria or someone acting for her caused the asset not to be received.

How the penalty works

The penalty is not simply a fine equal to the gift. Illinois aggregates nonallowable transfers and divides total uncompensated value by the applicable average private-pay cost of long-term care in the relevant community. The result is a period of ineligibility for long-term-care services, including months, days, and a portion of a day; the rule does not round the result down.[52]

PENALTY LOGIC
Total uncompensated value ÷ the current Illinois private-pay divisor = the penalty period. Verify the divisor and calculation at the time of application; never reuse an old example amount.

Timing is the painful part. For current-rule transfers, the penalty generally begins at the later of the transfer month or the date the person is otherwise eligible and would be receiving long-term-care services but for the penalty, after any spenddown is met. That can create a coverage gap when the money is already gone. The notice must include appeal information.[52]

“But it was three years ago” is not an exception

Maria finally remembered helping her nephew. He had used the money toward a boat. The transfer was old enough to feel settled inside the family—and recent enough to sit squarely inside a 60-month review.



The cartoon compresses a difficult process into one deadline. In real life, returning a gift can require tracing the original transfer, identifying the recipient, documenting the amount returned, recalculating available resources, coordinating application timing, and confirming how Illinois will treat the return. A promise to repay is not the same as returned funds.

Relatives receive scrutiny, not automatic condemnation

A transfer to a child, nephew, sibling, friend, church, or charity is not improper merely because the recipient is familiar. But love and affection are not fair-market-value compensation under the Illinois rule. Related-party sales, family loans, and payments for earlier informal care need objective records and realistic value.

Adding a name can transfer more than convenience

Putting a child on a deed or account “so she can help” may change ownership, withdrawal rights, survivorship, creditor exposure, or control. Removing a name later can create another transfer question. Before changing title, obtain the signature card or deed, contribution history, market value, and an explanation of the rights that moved. Perform Chapter 11’s ownership analysis and this chapter’s transfer analysis together.

Family care needs a before, during, and after file

Elena had driven Maria to appointments, organized medication, handled bills, and stayed overnight after illnesses. Those services had value. But the family had never discussed payment. A check written years later could not automatically convert loving help into a past debt.

Illinois presumes that care, services, or accommodations previously provided by a friend or relative were gratuitous unless credible documentary evidence created before the services establishes the compensation terms and contemporaneous records prove that the services were delivered. Payment above prevailing community rates can be treated as a transfer for less than fair market value.[55]

A defensible caregiver arrangement

- **Before care begins.** Use a written agreement identifying the parties, duties, schedule, rate, payment timing, term, termination, substitute care, and authority to sign.
- **Set a market rate.** Use comparable agency or independent-caregiver rates for similar work in the community. Explain unusual duties or higher rates.
- **During the work.** Keep dated time logs, task records, mileage or expense support, invoices or payroll records, proof of payment, and any required tax filings.
- **Separate roles.** Distinguish unpaid daughter, agent under power of attorney, landlord, household employee, independent contractor, and paid caregiver. One person may have several roles, but the records must say which hat she wore.
- **Change prospectively.** If duties or rates change, amend the agreement before the new arrangement begins. Do not backdate or create a retroactive salary.

A caregiver agreement is not a magic shield. The work must be real, the rate reasonable, the contract followed, the signer authorized, and the records consistent with bank activity. Tax and employment-law consequences also require review. Chapter 8's family agreement and Chapter 12's spend-down file provide the supporting structure.

THE THREE-DATE TEST
The agreement date must precede the paid care; the service dates must match the logs; and the payment dates must match the bank record. If the dates tell different stories, stop and investigate.

Exceptions are narrow doors with evidence requirements

The five-year look-back does not mean every transfer creates a penalty. Illinois recognizes specific exceptions and rebuttals, but each depends on exact relationships, property, timing, purpose, legal instruments, and proof. Families should not compress them into "transfers to family are allowed."[56]

Important categories to review with qualified help

- **Spouse.** Certain transfers to a spouse or for the spouse's sole benefit may avoid penalty, but spousal-resource rules, timing, and the legally binding arrangement still matter.
- **Blind or disabled child or beneficiary.** Specified transfers to a qualifying child, qualifying trust, or for the sole benefit of a person determined disabled can receive special treatment. "Sole benefit" requires a binding written structure and precise compliance.
- **Home-transfer exceptions.** A homestead may be transferred in specified circumstances to a spouse; a child under 21; a qualifying blind or disabled child; a sibling with an equity interest who satisfies the residence requirement; or a caregiving son or daughter who satisfies the two-year residence, level-of-care, and care-prevention evidence requirements.
- **Fair market value intended or received.** Good-faith marketing, an arm's-length price, comparable sales, expert valuation, and a binding exchange can matter. A subjective statement that "we thought it was fair" is not enough.
- **Exclusively another reason.** The rule permits rebuttal when credible tangible evidence shows the transfer was exclusively for a reason other than qualifying for assistance. The person's health, finances, living arrangements, expected support, timing, and later unexpected events matter.
- **Returned asset or hardship.** A returned transfer and an undue-hardship request have their own rules. Neither should be assumed from fairness alone.

A diagnosis, family relationship, residence, or caregiving history may be necessary but not sufficient. For example, the caregiver-child home exception calls for evidence of institutional-level need, the child's two-year residence, and care that prevented institutionalization. Build the proof around every element—not merely the family's overall story.[56]

REMEMBER THIS
An exception is not a sympathetic explanation. It is a rule with elements. Identify each element, attach evidence to each one, and leave unsupported conclusions out of the application file.

Curing a transfer is a legal process, not a family promise

When Elena spoke to the nephew, she did not say that selling the boat would automatically erase every problem. She said Maria needed the money returned, then asked for advice about the correct sequence.

Illinois distinguishes returns made before a penalty is imposed from returns after imposition. For transfers on or after January 1, 2012, all transferred assets generally must be returned to eliminate an imposed penalty. Before imposition, full or partial returns can reduce the uncompensated portion; after imposition, the rule describes elimination when all assets are returned. Returned assets become available resources and may then require lawful spend-down or other treatment.[57]

The return file should include the original transaction, recipient, original and returned amounts, dates, cancelled checks or wire confirmations, sale documents if property funded the return, post-return account statements, and the adviser's calculation. Do not route the return through another relative or immediately give it away again.

Hardship is actual, not merely inconvenient

Illinois provides for waiver of all or part of a penalty when application would deprive the person of medical care so that health or life is endangered, or of food, clothing, shelter, or other necessities. The applicant bears the burden of proving actual hardship and may need to show efforts to recover the asset, the circumstances of transfer, the person's condition and capacity, and the real effect of the penalty.[57]

Professional-help triggers

- **Before filing or responding,** seek Illinois elder-law or Medicaid counsel for a deed change, trust, life estate, annuity, promissory note, large gift, business transfer, spouse transaction, disputed capacity, or missing ownership records.
- **After a notice,** get help immediately when a transfer penalty, denial, incomplete-verification request, hardship issue, or appeal deadline appears. Preserve the notice and envelope.
- **When recovery is difficult,** document good-faith efforts and obtain advice before suing a relative, selling property, compromising a debt, reporting exploitation, or claiming hardship.

The goal is not to make every transaction look acceptable. It is to identify the correct treatment early enough to protect care and preserve every available remedy.

The 60-Month Transfer Review

Enter every unusual transaction before classifying it. Continue on additional sheets and cross-reference the supporting file.

Date / asset	Recipient or new owner	FMV / amount	Value received and proof	Status / adviser

File controls

Look-back start: _____ **Date used:** _____

Statements obtained: Yes / No **Missing months:** _____ **Potential uncompensated value: $**_____

Final review before filing

- **Do not hide or relabel.** Disclose responsive transfers and explain them with records. A confusing transaction is not improved by an inaccurate answer.
- **Separate facts, conclusions, and deadlines.** Record the transaction and proof first; identify the adviser's classification; calendar verification requests, penalty notices, hardship submissions, appeals, and expected coverage.

Bottom Line

The look-back reaches sixty months, but the family controls the file. Reconstruct every transfer, calculate uncompensated value, test every exception, document returns, and seek help before a transaction becomes a care-coverage gap. The worst surprise is not that the State looked back. It is that the family waited until the application to look back themselves.

CHAPTER 14

Applying and Surviving the Paper Chase

The mailbox held three State envelopes on the same day. Elena read the labels aloud: “Approved, a request for more documents, and a notice—all today.” Mark looked at the stack. “Open them in the wrong order and you’ll have a heart attack for no reason.”



The letters were not contradictory. They concerned different pieces of Maria’s case. One confirmed a result, one demanded proof, and one started a deadline. The family’s first rule became simple: open every envelope on arrival, save the envelope, write the received date on the first page, scan the entire packet, and identify what action—if any—is required.

REMEMBER THIS
A notice is not ordinary mail. Date it, read every page, find the deadline and appeal instructions, and preserve the envelope. The deadline on the notice controls the next move.

Start the right processes at the right time

Maria's family wanted one application that would solve everything. Illinois long-term-care planning rarely works that way. For nursing-facility payment, a person generally applies for medical benefits through the Illinois Department of Human Services and also completes a needs screening through the agency responsible for that assessment. Home- and community-based waiver pathways likewise require financial eligibility and a program-specific level-of-care or functional determination.[58]

Run three tracks together

- **Financial application.** Identify the exact Medicaid pathway, applicant, spouse, residence, requested coverage, and application date. Use Illinois ABE, a paper form, or an authorized application channel, and keep the confirmation or case number.[59]
- **Needs screening.** Schedule the required functional or level-of-care assessment with the correct operating agency. A financial approval does not substitute for a clinical or functional determination.
- **Care and provider fit.** Confirm that the facility or community program participates in the applicable Medicaid pathway and can meet the person's needs. An approved benefit does not make every setting payable.

Apply neither blindly nor indefinitely late. Filing too early can create avoidable verification demands or expose unresolved transfer issues; waiting for a perfect file can delay eligibility, care coordination, or an appealable decision. Before filing, identify the requested program, likely eligibility month, missing records, urgent-care facts, and professional-help triggers from Chapters 10–13.

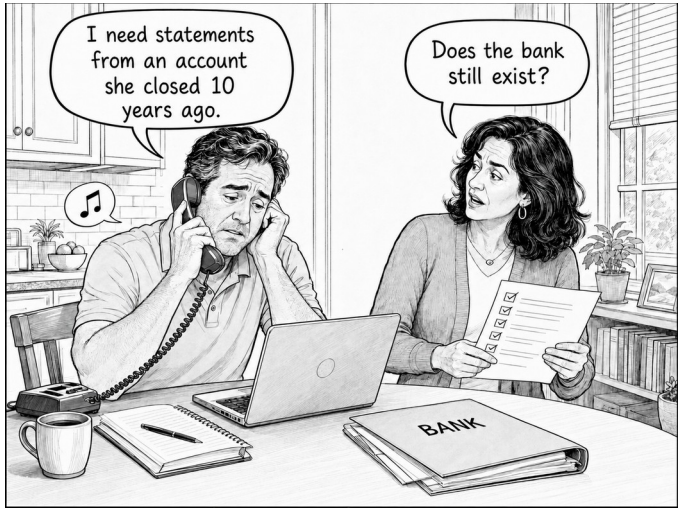
The application-day control sheet

- **Record the submission.** Save the full application exactly as filed, all attachments, the date and method, confirmation page, tracking number, office, case number, and the person who submitted it.
- **Authorize communication.** Confirm who may speak for Maria and what document proves that authority. A helper is not automatically an authorized representative for every agency or provider.
- **Create one calendar.** Enter screening appointments, requested verification dates, renewal dates, expected follow-up, facility deadlines, and every deadline stated on a notice.

If an application is made by phone, Illinois instructs applicants to request and preserve the case number. Online Manage My Case can be used to check status, upload documents, report changes, and view benefit information after the case is linked.[59]

The missing statement is usually not missing by itself

Mark called the bank with Maria's account list beside him. The problem was not the current checking account. It was an account that had been closed long before the family knew it might matter.



Old statements can reveal linked accounts, transfers, cashier's checks, recurring deposits, or an owner the family forgot. Ask for the exact date range, all pages, relevant check images, account-opening and closing records, signature cards, ownership history, and proof of closure. If a bank merged or failed, trace the successor and document every attempt.

Build the verification file by category

- **Identity and authority.** Identification, Social Security and Medicare information, marital status, address history, guardianship or power-of-attorney records, and representative forms.
- **Income and health coverage.** Benefit letters, pensions, annuities, retirement distributions, wages, premiums, and health or long-term-care coverage.
- **Resources and ownership.** All statement pages, retirement accounts, deeds, vehicles, insurance cash values, burial arrangements, trusts, annuities, business interests, and jointly held property.
- **Transfers and spending.** Cancelled checks, receipts, contracts, invoices, appraisals, caregiver logs, gift records, sales, title changes, and Chapters 12–13 explanations.

Illinois's mail-in medical application asks about resources, five-year transfers, people who helped manage finances, and supporting records. [60] Use it as a checklist even when applying another way, but confirm the current program's requirements.

Make each unusual transaction tell one documented story

A bank statement proves that money moved. It rarely proves why. The reviewer may see a $4,800 deposit, a $2,200 check to Elena, repeated cash withdrawals, or a transfer between accounts and reasonably ask for an explanation. The family should answer with a short, neutral statement tied to evidence—not a long argument and not a guess.

The transaction explanation

- **Identify it.** State the date, amount, account, payer or recipient, and transaction type exactly as shown on the statement.
- **Explain the source or purpose.** Examples include an insurance reimbursement, tax refund, sale proceeds, account-to-account transfer, returned purchase, payment for fair-value services, or returned gift.
- **Trace both sides.** For transfers between Maria's accounts, provide the sending and receiving records. For a sale, attach ownership, valuation, contract, deposit, and use-of-proceeds evidence.
- **Separate known from unknown.** If the family cannot identify a transaction, say what was checked, what records were requested, and who was contacted. Do not manufacture certainty.

Package the response so a stranger can review it

Put the agency request first. Follow it with a one-page index matching each requested item to an attachment. Number the pages or files, label the applicant and case number, and use the agency's accepted upload, fax, mail, or in-person method. Submit only through a channel the agency recognizes; retain an exact copy and proof of delivery.

> **REMEMBER THIS**
> Never send the only copy. The case file should show what was submitted, when, by whom, through which channel, and what proof confirms receipt.

One submission can answer several questions, but do not bury the answer. If Attachment 7 explains a deposit, say "Deposit dated May 4—see Attachment 7, pages 1–3." A clear cross-reference can prevent another request and makes an appeal record easier to reconstruct.

A follow-up request is a deadline, not a verdict

Elena sorted every letter into one of four actions: information only, provide proof, correct or clarify, or challenge the decision. “Approved” could still require checking the effective date and scope. “More documents” required a complete response or a documented request for help or additional time. A denial or adverse notice required immediate review of the facts, rule cited, effective date, and appeal instructions.

When more proof is requested

- **Read the exact request.** List every item, deadline, acceptable alternative, office, case number, and delivery method. Do not answer only the easiest part.
- **Respond even when records are unavailable.** Before the deadline, provide what exists, explain what does not, attach record-request efforts, and ask the agency what substitute evidence it will accept. Preserve the response and delivery proof.
- **Correct the record clearly.** Identify the prior answer, state the corrected fact, explain why it changed, and attach support. Silent replacement documents can look inconsistent.

When a notice is adverse

- **Do not rely on a phone call alone.** A useful call may explain the issue, but it does not necessarily preserve appeal rights. Follow the notice’s written instructions and keep confirmation.
- **Calendar two deadlines.** Record the final appeal deadline and any earlier deadline that may apply to continuation of existing assistance or services. Different program and managed-care routes can have different steps.
- **Build the appeal packet.** Keep the notice and envelope, application, submissions, delivery proof, call log, governing rule, disputed calculation, missing evidence, and the remedy requested.

Illinois’s general assistance-appeal rule states a 60-calendar-day filing period for many DHS actions, while continuation of existing assistance can require earlier action; managed-care and waiver procedures may differ. The ABE Appeals portal can accept requests for a State Fair Hearing. The safest instruction is the one on the actual notice: read it immediately, meet its deadline, and seek qualified help when care or coverage is at risk.[61]

Submission and Notice Log

Use one line for every application, upload, fax, mailing, call, screening, request, notice, or appeal. Continue on additional sheets; keep the proof in the same numbered file.

Date / item	Agency / contact	Case no. / method	What happened or was sent	Deadline / next step

Case controls

Applicant: _____ **Program/pathway:** _____

Application date: _____ **Case number:** _____ **Authorized representative:** _____

Weekly paper-chase review

- **Mail and portal.** Check both; scan new items; preserve envelopes; update deadlines; and confirm the current address, phone, and authorized contact.
- **Open requests.** List missing items, the person responsible, record-request dates, partial responses, delivery proof, and the next follow-up date.
- **Decisions.** Compare approval dates, scope, cost sharing, and provider payment with what was requested. Escalate adverse or unclear notices before appeal time expires.

Bottom Line

The paper chase becomes survivable when every event leaves evidence. Apply through the correct pathway, run the screening in parallel, explain transactions with documents, answer requests before the deadline, and treat every notice as a decision that may start a clock. The family cannot control every agency letter. It can control whether the file proves what happened next.

CHAPTER 15

Estate Recovery and Liens

Mark held the old eligibility letter in one hand and the new recovery letter in the other. “They told us the house didn’t count.” Elena looked at the house through the window. “It didn’t count while she was alive.”



Both statements could be true. A home may receive favorable treatment when Medicaid decides whether an applicant is financially eligible. That does not answer whether Illinois may later file an estate claim, whether an older lien already affects the property, or whether a survivor protection or hardship waiver applies. Those are separate questions asked at separate times.

REMEMBER THIS
“Exempt for eligibility” does not mean “immune from every later claim.” Identify the program, owner, title, date, estate, survivors, and any recorded lien before drawing a conclusion.

Recovery begins with the correct program and payment history

Federal law requires states to recover specified Medicaid payments for certain people age 55 or older and permits broader recovery in defined circumstances. Illinois's rules and public guidance focus on assistance provided through Aid to the Aged, Blind or Disabled and related medical-assistance categories. For many AABD medical-only recipients, Illinois may claim medical assistance paid after the person's fifty-fifth birthday. Other histories—including cash assistance or an older lien tied to permanent institutionalization—can produce a different calculation.[62]

The claim is a calculation, not a slogan

- **Start with paid assistance.** Request the State's itemization and identify the program, service dates, payment dates, age, and any amount already repaid. Illinois will not seek more than it paid, but the claim can still exceed the estate's value.
- **Measure the estate.** List the property Maria owned at death, its title, gross value, secured debt, selling costs, and governing estate route. For deaths on or after July 1, 2022, Illinois waives recovery against the first $25,000 of gross estate value.[63]
- **Apply estate priorities.** The Illinois guide states that funeral costs, legal costs, and a home mortgage are paid before an estate-recovery claim. Probate counsel should determine the actual priority and documentation for every expense.[63]
- **Test exclusions and special rules.** HFS gives life insurance payable to a named person and payable-on-death bank accounts as examples of property it will not collect from. But title, beneficiary language, trusts, joint ownership, and qualified long-term-care partnership rules can change the analysis.

For most cases, the Illinois regulatory definition begins with property inside the individual's estate under the Probate Act. A special expanded definition can apply when benefits were received under a qualified long-term-care insurance policy for which assets were disregarded. Do not assume that "avoids probate" always answers the Medicaid question—or that every nonprobate transfer is automatically recoverable.[64]

THE RECOVERY CEILING
The practical payment is limited by the valid claim, the property legally reachable, estate priority, the $25,000 protection when applicable, survivor rules, hardship relief, and the value actually available. Each limit needs proof.

A claim, a lien, and a notice are not the same thing

After a Medicaid customer dies, Illinois may send the estate representative or heirs a notice of intent and request information before deciding whether to file a claim. The notice begins a response process; it is not proof that the stated amount is correct or collectible from every asset.[63]

Survivor protections come before collection

- **Surviving spouse.** Illinois does not enforce a medical-assistance estate claim against the deceased recipient's property while a spouse survives. The rule also addresses claims involving a deceased spouse's estate, so later exposure and title questions belong with Illinois probate and elder-law counsel.
- **Protected child.** Recovery is not enforced while there is a surviving child under 21 or a child of any age who is blind or permanently and totally disabled under the applicable standard.
- **Proof and timing.** Do not assume the State knows the survivor exists or qualifies. Return the requested information, attach proof, preserve delivery evidence, and calendar every deadline.

The lien question now has a date

A lien is a recorded security interest in real property; an estate claim is a demand against property after death. Illinois HFS states that Public Act 102-1037 restricts the filing of new real-property liens in these cases effective June 2, 2022, while liens filed before that date remain subject to pursuit. Therefore, search the county land records instead of relying on memory or an old benefits letter.[65]

- **If an older lien exists,** obtain the recorded instrument, filing and renewal history, payoff calculation, property description, ownership chain, and any release. HFS states it does not force a sale solely to enforce such a lien; payment generally arises when the property is sold.
- **Protected occupancy can matter.** The older lien rules identify protections for a spouse, a child under 21, a blind or disabled child, and in some circumstances a sibling with equity and qualifying residence. A return home after institutionalization may require release of the lien.
- **Never change title casually.** A deed signed to "avoid recovery" may create a transfer penalty, tax consequences, creditor exposure, loss of control, or a title defect without solving the estate issue. Review Chapter 13 before any conveyance.

Sometimes the estate is smaller than the letter

Mark and Elena waited for their appointment with the recovery letter between them. Mark read, “They’re seeking recovery from the estate.” Elena opened the checkbook. “The estate is this lamp and a checkbook with forty dollars. Good luck.”



A large claim does not create assets that do not exist. Still, the family must answer accurately. The estate representative should inventory property, trace title, document values and debts, preserve the notice and envelope, and avoid distributing property until probate counsel confirms the proper process.

Hardship must be requested and proved

Illinois may waive recovery when it would cause an heir or beneficiary to become or remain eligible for a public-benefit program. HFS also identifies a qualifying family business, farm, or ranch that was the estate property for at least twelve months before death and is the heirs’ main income source, as well as other circumstances that may create hardship. Relief is not automatic.[66]

Each person seeking hardship consideration must file a separate application with supporting proof. Current HFS instructions require submission within 60 calendar days from the date stated on the notice accompanying the application. The actual notice and current form control; seek help immediately rather than waiting for probate to finish.[66]

Estate Recovery and Lien Review

Bring this sheet, the notices, and the documents to Illinois probate or elder-law counsel. Record the answer and the source—not merely "yes" or "no."

Question	Known facts / documents	Counsel answer / next step
Which program and payments support the claim?		
What property is legally inside the recoverable estate?		
Does the $25,000 protection apply, and how is value measured?		
Is a spouse or protected child living, and what proof is required?		
Is any lien actually recorded, valid, renewed, payable, or releasable?		
Does an heir qualify for hardship, and what is the filing deadline?		

File controls

Date of death: _____ **Notice date:** _____ **Response / hardship deadline:** _____

Gross estate value: $_____ **State claim: $**_____ **Recorded lien search completed:** Yes / No

Questions before anyone signs or pays

- **Who has authority?** Identify the executor, administrator, trustee, surviving joint owner, or other fiduciary and the document proving authority.
- **What does the notice require?** Separate information requests, claim deadlines, hardship deadlines, appeal rights, payoff requests, and probate deadlines.
- **What should remain untouched?** Ask before selling the home, distributing cash, paying lower-priority debts, signing a deed, closing probate, or paying the State from a survivor's own funds.

Bottom Line

Eligibility, liens, and estate recovery are three different analyses. Verify the assistance history, title, survivors, recorded liens, claim calculation, $25,000 protection, hardship evidence, and deadlines. Neither panic at a large letter nor ignore a small estate; obtain counsel before property moves.

PART IV

Protecting the spouse at home

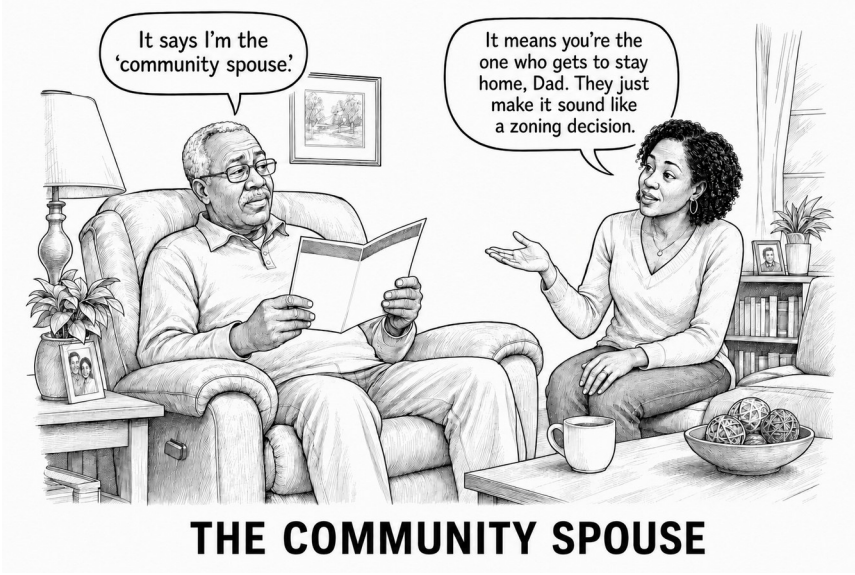
CHAPTER 16

Preserve a workable home, income, and resource base while the other spouse receives long-term care.

CHAPTER 16

Spousal Impoverishment in Plain English

James read the pamphlet from his recliner. “It says I’m the ‘community spouse.’” His daughter, Renee, translated. “It means you’re the one who gets to stay home, Dad. They just make it sound like a zoning decision.”



James’s wife, Lillian, needed nursing-facility care. In Medicaid language, she was the long-term-care or institutionalized spouse. James remained in the community. The labels do not decide who owns the marriage’s money or who deserves protection. They identify which spouse is receiving long-term-care services and which spouse needs enough resources and income to remain at home.

REMEMBER THIS
Spousal-impoverishment rules are not a reward for one spouse or a punishment for the other. They divide one financial problem into two goals: qualify the long-term-care spouse and keep the community spouse financially stable.

Start with the pathway, not the nickname

Illinois applies the Community Spouse Resource Allowance to long-term-care customers who have a community spouse. The current policy includes people in nursing homes and Supportive Living Programs, and people applying for or receiving the Illinois Department on Aging home- and community-based waiver. Other Medicaid categories can use different financial rules.[67]

Name the two roles

- **Long-term-care spouse.** The spouse applying for or receiving the covered nursing-facility, supportive-living, or qualifying waiver service. “Institutionalized spouse” is the federal term often used even when a waiver allows care in the community.
- **Community spouse.** The husband or wife who is not receiving that long-term-care pathway and remains in the community. If the community spouse later enters a nursing home, Supportive Living Program, or qualifying waiver, the allowance analysis changes.

Four dates control the file

- **Care-pathway date.** When did the long-term-care spouse enter the facility or begin the qualifying waiver process?
- **Application date.** Which annual CSRA and CSMNA standards apply to the filing and decision?
- **Initial determination date.** Which resources and income were reported, verified, and used in the first eligibility calculation?
- **First redetermination.** Was ownership of the protected resource allowance transferred and documented as required by the notice?

Do not assume the rules begin only after admission or that every assisted-living resident is covered. The facility type, Medicaid program, screening, application month, marital status, and living arrangement must match the pathway. Chapter 10’s Medicaid Route Sheet should be completed before any spouse calculation begins.

The information duty belongs to both spouses

Illinois requires the long-term-care customer and community spouse to provide essential information about income and resources owned by either spouse or both. Refusing to disclose an account because it is “his,” “hers,” or jointly titled does not protect it from the initial review.[68]

For resources, begin by putting both columns on one page

At the initial determination, Illinois looks at total nonexempt resources owned by Lillian, James, or jointly by both. The current account title still matters for proof and transfer mechanics, but it does not let the couple omit one spouse's nonexempt property from the starting inventory.[67]

Step 1 — classify before calculating

- **Exclude what the program excludes.** Analyze the home, vehicle, household goods, burial arrangements, insurance, retirement funds, and other property under Chapter 11. Do not subtract an asset merely because the family calls it untouchable.
- **Count each nonexempt resource once.** List owner, account number, verification date, gross value, debt, equity, accessibility, and any pending transaction. Reconcile statements to the application date.
- **Separate resource from income.** A bank balance is a resource; a monthly pension is income. Depositing income into an account can change its form over time, so use the program's measurement date.

Step 2 — protect the current Illinois allowance

For applications received on or after January 1, 2026, Illinois allows a Community Spouse Resource Allowance of up to $143,172. It is the portion of combined nonexempt resources that may be held by or transferred to the community spouse or another person for that spouse's sole benefit. The number is a maximum standard—not a State payment and not permission to create assets the couple does not own.[69]

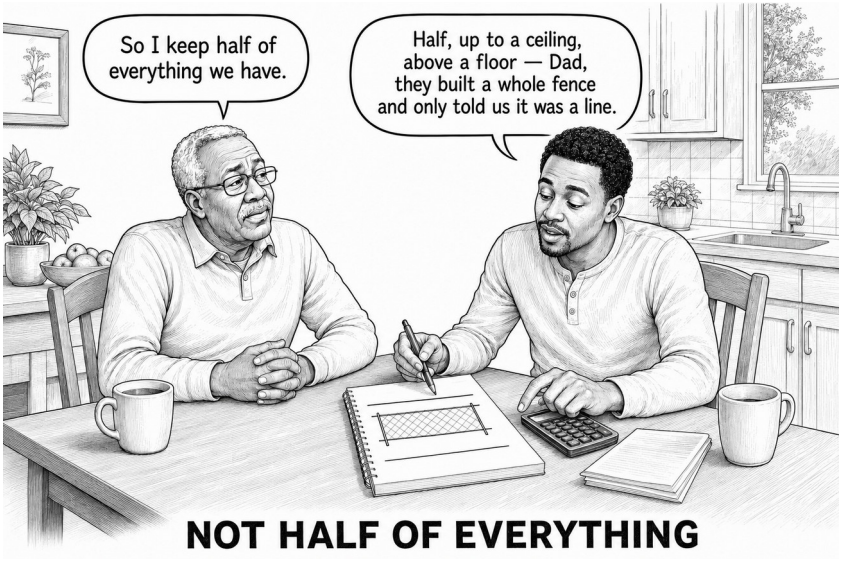
Step 3 — make ownership match the decision

To keep a nonexempt resource excluded as part of the CSRA, Illinois requires the protected ownership to be transferred to the community spouse or for that spouse's sole benefit. The decision notice and HFS 3190 show the protected amount and the portion remaining for the long-term-care spouse. Verify the transfer at the first redetermination and preserve the statements, deeds, assignments, and confirmation.[70]

OWNERSHIP STILL MATTERS
The initial calculation combines nonexempt resources. The completed plan then assigns the protected share to the community spouse. A calculation without the required ownership change can fail at redetermination.

"Half" is shorthand, not the Illinois answer

James tried to simplify the calculation. "So I keep half of everything we have." His son, David, looked at the worksheet. "Half, up to a ceiling, above a floor—Dad, they built a whole fence and only told us it was a line."



The joke reflects the confusing federal vocabulary, but current Illinois practice is clearer when written as a state-standard calculation. Use the published Illinois CSRA for the application year, not a remembered "half" rule from another state or an older article.

Illustration using the 2026 standard

- **Combined nonexempt resources:** $180,000 after every asset is identified, valued, and classified.
- **Maximum 2026 CSRA:** $143,172 protected for James, subject to the actual decision and proper ownership transfer.
- **Remainder:** $36,828 remains outside the CSRA and must be analyzed for Lillian's eligibility and lawful spend-down. Her separate applicant resource standard and other rules still apply.

If the couple had only $100,000 of nonexempt resources, the $143,172 maximum would not create another $43,172. If James already owned part of the protected amount, only the additional ownership needed to reach the approved CSRA would be transferred. Court orders and fair-hearing decisions can produce a higher allowance in limited circumstances.[69]

Income follows a different road from resources

The resource calculation begins with both spouses' nonexempt property. Monthly income is handled differently. James keeps his own income. After Lillian becomes eligible, her income is applied under post-eligibility rules, with allowed deductions that may include an amount actually contributed for James's maintenance.[71]

The 2026 maintenance calculation

Beginning January 1, 2026, Illinois uses a maximum Community Spouse Maintenance Needs Allowance standard of $4,066.50 per month. The ordinary calculation subtracts the community spouse's gross monthly income from that standard. The result is the potential deduction from the long-term-care spouse's income, limited by available non-SSI income and the amount actually contributed.[71]

- **James's gross income:** $2,500 per month.
- **2026 maximum standard:** $4,066.50 per month.
- **Potential diversion:** $1,566.50 per month, if Lillian has sufficient eligible income and actually contributes it. This is an illustration, not an eligibility decision.

Housing costs belong in the evidence file

Illinois's published CSMNA is a maximum standard, not a promise to reimburse every household bill. Still, the family should document James's rent or mortgage, property tax, homeowner's or renter's insurance, utilities, condominium charges, repairs needed for safe occupancy, medical costs, and debt service. Those records test whether the permitted income is enough and help counsel evaluate a court order, fair hearing, or another available remedy.

A court order or fair-hearing decision may establish a different maintenance allowance. Illinois also permits a higher CSRA after an appeal when additional income-producing resources are needed to raise the community spouse's income to the maximum CSMNA, using the State's prescribed method. That is a professional-help issue—not a do-it-yourself transfer.[69][71]

DO NOT BLEND THE TWO ALLOWANCES
CSRA protects resources. CSMNA can divert monthly income. One is a balance-sheet allowance; the other is a cash-flow allowance. Calculate and document them separately.

The figures expire; the file should not

Illinois increased both standards for 2026 under Public Act 102-1037. The CSRA is $143,172 and the CSMNA maximum is $4,066.50. Illinois states that both will increase annually based on federal poverty-level amounts. Every worksheet should therefore display the year, application date, source URL, and date verified.[72]

What happens after the initial decision

- **Community-spouse resources.** Illinois generally does not review resources newly acquired by the community spouse after initial eligibility is established. An inheritance received later by James ordinarily does not reopen the established CSRA under the cited policy.
- **Long-term-care spouse resources.** A resource later received by Lillian cannot simply be transferred to James to top up an unused portion of the earlier maximum. Stop and obtain advice before moving it.
- **Change in care status.** If James begins a qualifying waiver or enters a nursing home or Supportive Living Program, the community-spouse allowance may no longer apply in the same way.
- **Income changes.** Report pensions, Social Security changes, annuity payments, work income, or other changes as required. Recalculate the potential diversion using the current notice and effective month.

Professional-help and appeal triggers

- **Request review immediately when** an account was omitted, ownership is disputed, a valuation is wrong, a retirement account is misclassified, or the notice uses the wrong application year or care pathway.
- **Seek Illinois elder-law counsel when** a court order, fair hearing, higher CSRA, higher CSMNA, annuity calculation, trust, deed, business, farm, separation, prenuptial agreement, or incapacity affects the plan.
- **Preserve the deadline when** the determination understates the allowance or overstates the amount available for care. A phone call may help, but it does not necessarily protect appeal rights.

The family should keep the application, HFS 3190, decision notice, resource inventory, transfer proof, income calculation, housing-cost file, call log, and every annual standard used. The arithmetic can be recreated only if the evidence survives.

Community Spouse Protection Record

Complete this record before filing, update it after the decision, and bring it to the first redetermination. Attach supporting schedules when a line is too small.

Item	LTC spouse	Community spouse	Joint / proof / treatment
Nonexempt accounts			
Investments / retirement			
Other nonexempt property			
Gross monthly income			
Housing and fixed costs			

Decision controls

Care pathway: _____ Application date: _____ Standard year: _____

Combined nonexempt resources: $_____ Approved CSRA: $_____

Community-spouse gross income: $_____ Approved monthly diversion: $_____

Before the first redetermination

- **Match the notice.** Confirm every resource, owner, valuation, excluded item, CSRA amount, income source, and effective month.
- **Complete the ownership work.** Transfer only the amount and property approved for the community spouse or that spouse's sole benefit; preserve proof and tax or title advice.
- **Reconcile cash flow.** Confirm that the approved income diversion is actually paid and visible in both spouses' bank records.
- **Calendar change and appeal duties.** Record redetermination, reporting, transfer-verification, fair-hearing, and court dates.

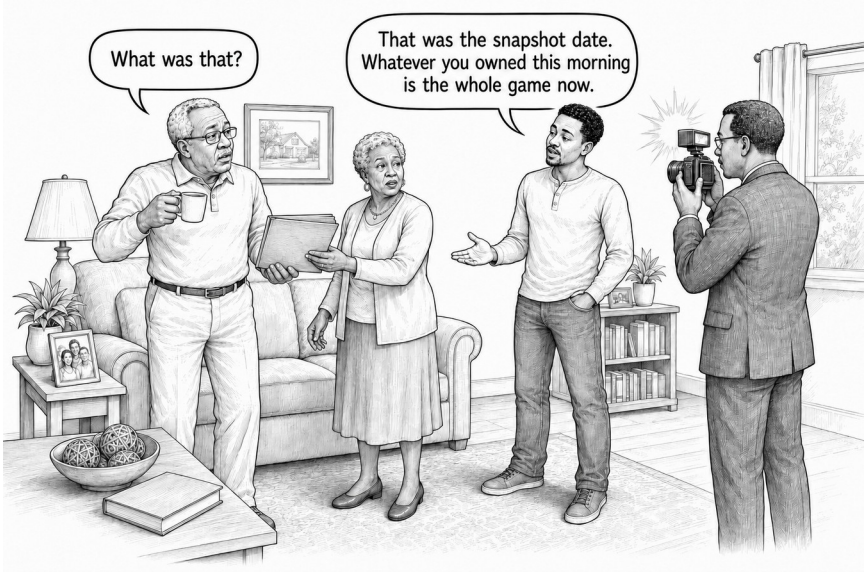
Bottom Line

Protecting the spouse at home requires two separate calculations and one disciplined file. Pool and classify nonexempt resources, apply the current Illinois CSRA, complete the approved ownership transfer, preserve each spouse's income treatment, calculate any CSMNA diversion, document housing reality, and challenge a wrong decision before the deadline.

CHAPTER 17

The Snapshot Date and the Application Date

An official lifted a camera while James and Lillian stood frozen in the living room. “What was that?” James asked. David answered, “That was the snapshot date. Whatever you owned this morning is the whole game now.”



David’s sentence made the date memorable, but it was deliberately too absolute. Federal spousal-impoverishment law uses the beginning of the first continuous period of institutionalization to measure and document a couple’s resources for the resource assessment. The later Medicaid application uses the resources and rules relevant to the eligibility determination. The two dates serve different jobs.[73]

REMEMBER THIS
The snapshot is an evidence date. The application is a decision date. Neither replaces the other, and neither authorizes a transfer, gift, concealment, or hurried rearrangement of ownership.

First identify the event before reconstructing the money

"Snapshot date" is convenient family shorthand. The federal statute speaks of the beginning of the first continuous period of institutionalization. For a nursing-facility case, this is generally anchored to the first day of the qualifying continuous stay—not the day the family first hears the phrase, hires counsel, spends down, or submits an application.[73]

Do not pick the date from memory alone

- **Build the care chronology.** List hospital admissions and discharges, rehabilitation stays, nursing-facility admission, moves between facilities, days at home, waiver assessments, and the date long-term care was expected to continue.
- **Confirm the pathway.** A nursing home, Supportive Living Program, and Department on Aging waiver do not create identical fact patterns. Ask the agency or qualified Illinois adviser which event activates the spousal rules for this application.
- **Resolve interruptions.** A return home, short rehabilitation episode, change of setting, or later readmission can make the "first continuous period" question technical. Preserve the records instead of guessing that a new building always creates a new snapshot.
- **Record the source.** Write down who supplied the date, which admission record supports it, and whether the State accepted it. Keep the determination or assessment with the permanent Medicaid file.

The snapshot can matter even when the application comes later. Federal law allows either spouse to request a resource assessment at the beginning of the period, upon providing relevant documentation. The State must assess and document the total value, give the requesting spouse a copy, retain a copy, and include fair-hearing information. A request outside an application can carry a reasonable fee at the State's option.[73]

If no assessment was requested promptly, the family may have to reconstruct the date later. That is why old statements, admission papers, and account histories become more valuable than anyone's recollection.

Build one dated balance sheet, not a box of statements

The resource assessment starts with the total value of resources in which either spouse had an ownership interest. Excluded property must be classified under the applicable rules; joint property must not be counted twice; and account titles do not excuse missing proof. Chapter 11's asset inventory supplies the categories. Chapter 17 adds a measurement date. [73]

For each resource, capture five facts

- **Identity.** Institution, last four digits, property description, policy number, or other unique identifier.
- **Ownership and access.** Whose name was on it? Who could withdraw, sell, surrender, borrow, or demand payment on the snapshot date?
- **Gross value and debt.** Use the relevant close-of-day balance, cash value, market value, equity, or other program valuation—not an estimate copied from today's screen.
- **Classification.** Countable, potentially excluded, unavailable, disputed, or awaiting professional review. State the legal or policy basis; do not mark "safe" because a relative said so.
- **Evidence.** Statement, ledger, deed, title, appraisal, loan payoff, insurance illustration, retirement-account record, trust, business record, or written explanation.

When one statement is not enough

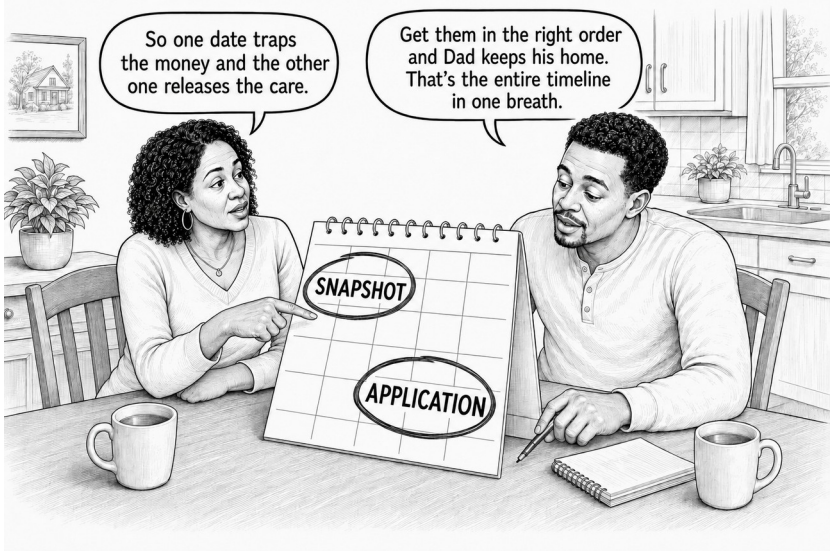
A monthly bank statement may straddle the date. Obtain transaction detail for the exact day and reconcile deposits, checks, transfers, and pending transactions. For securities, preserve the positions and pricing date. For a closed account, obtain the closing statement and destination of the funds. For cash, document the amount and source honestly; do not manufacture precision after the fact.

Prepare a one-page reconciliation from the evidence. It should show the snapshot total, every excluded item, every unresolved issue, and the location of the supporting record. A reviewer should be able to reproduce the total without calling the family.

A DATE WITHOUT PROOF IS ONLY A STORY
The strongest snapshot file connects each number to a dated record and each classification to a rule. Missing proof can turn a correct memory into an unusable claim.

The application date starts a different calculation

Renee pointed to an oversized calendar. “So one date traps the money and the other one releases the care.” David answered, “Get them in the right order and Dad keeps his home. That’s the entire timeline in one breath.”



Again, the joke compresses the law. An application does not automatically release care, and a snapshot does not freeze every later dollar. At initial eligibility, federal law attributes resources held by either spouse or both, then protects the applicable community-spouse allowance. Illinois applies the CSRA to the couple’s total nonexempt resources and uses the standard in effect for the application. For applications received on or after January 1, 2026, the published Illinois maximum is $143,172; annual figures must be rechecked.[74][75]

What must be updated for the application

- **Current resources.** Reconcile every account and property item from the snapshot through the application month. Identify what still exists, what changed form, and where money went.
- **Current program facts.** Confirm marital status, community-spouse status, care pathway, medical or functional approval, residence, and the year’s resource standards.
- **Current authority.** Confirm powers of attorney, guardianship, signatures, authorized representatives, and the mailing or electronic address for notices.
- **Current request.** State the coverage month sought and retain the filing confirmation. A complete resource assessment is not itself an application for benefits.

Every change between the dates needs a name and a receipt

Money can change between the snapshot and application for lawful reasons: facility charges, medical bills, debt payments, taxes, home maintenance, equipment, legal services, burial planning, fair-market-value purchases, market gains or losses, and ordinary living expenses. The task is not to preserve the earlier total. The task is to explain the bridge from the earlier total to the later one.

Use four labels for every material change

- **Spent for value.** Identify the payee, purpose, amount, date, benefit to Lillian or James, invoice, proof of payment, and fair-market-value support.
- **Changed form.** Show the trail when cash becomes a burial contract, vehicle, home repair, annuity, prepaid service, or another asset. Reclassify the new asset instead of treating the money as vanished.
- **Transferred.** Flag gifts, added names, deed changes, joint accounts, loans to relatives, caregiver payments, and property sold below value. Cross-reference Chapter 13 before assuming the transfer is harmless.
- **Changed without action.** Document interest, dividends, market fluctuation, fees, insurance-value changes, debt reduction, and other automatic movement.

After eligibility, the dividing line changes again

Federal law generally stops deeming the community spouse's resources available after the eligibility month during the continuous institutionalization period. Illinois policy similarly says resources later acquired by the community spouse are not subject to review after initial determination, while a later resource received by the long-term-care spouse cannot simply be transferred to fill unused CSRA room. A jointly received later resource is treated separately under Illinois policy.[74][75]

The approved CSRA still requires follow-through. Illinois sends the HFS 3190 with the decision, allows the stated protected transfer period, and verifies ownership by the required redetermination. Copy the notice exactly; do not improvise a transfer from a worksheet alone.[76]

Stop for professional help when the date is disputed, records are missing, ownership is unclear, a trust or business is involved, a spouse died or moved, a court order exists, a transfer occurred, or the notice uses the wrong pathway, year, resources, or allowance. Preserve the appeal deadline while corrections are discussed.[74]

Two-Date Resource Record

Complete this record from primary documents. Attach an account-by-account schedule when a line is too small, and update the application column until the eligibility decision is issued.

First continuous-period date: _____ Source document: _____

Application date: _____ Coverage month requested: _____

Resource / owner	Snapshot value / proof	Application value / proof	Change and treatment

File-closing checks

- **Reconcile the totals.** Snapshot total, additions, subtractions, form changes, and application total must connect without unexplained gaps.
- **Match the decision.** Compare the agency's resource list, exclusions, CSRA, effective month, and transfer instructions with the family record.
- **Preserve the proof.** Keep the resource assessment, application, HFS 3190, notice, statements, explanations, ownership-transfer evidence, and appeal calendar together.

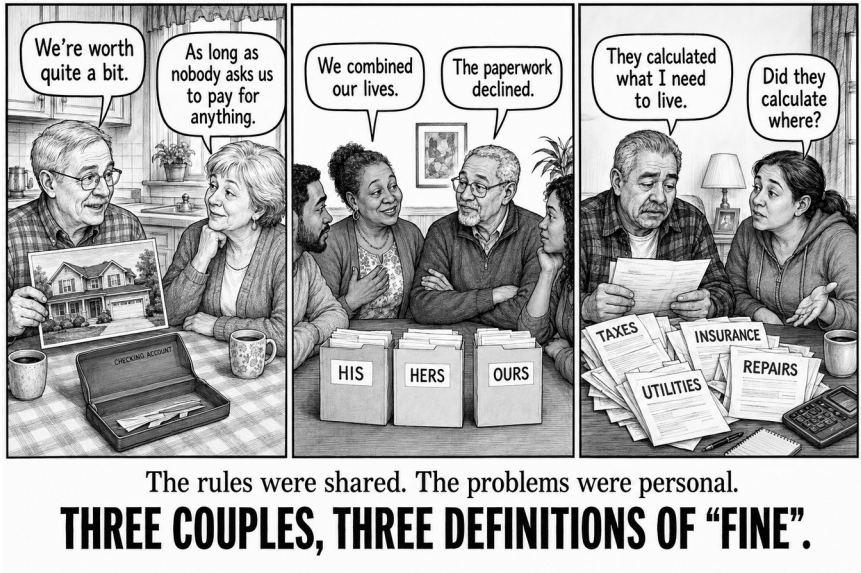
Bottom Line

The snapshot and application dates are not rival answers. The first anchors the historical resource assessment; the second anchors eligibility under the facts and standards then in force. Identify both dates, prove both balances, explain every change between them, follow the written decision, and obtain Illinois-specific advice before a transfer or missed deadline changes the result.

CHAPTER 18

Three Illinois Couple Case Studies

Three couples reached the same Illinois rules through very different doors. One had a valuable home and limited cash. One brought children and property expectations from earlier marriages. One had little income and a house that cost too much to maintain.



These are teaching files, not predictions. Each calculation assumes an Illinois long-term-care pathway covered by the spousal-impoverishment rules, an application received in 2026, complete disclosure, and no undisclosed transfer, trust, business, annuity, lawsuit, or tax problem. Change one fact and the result may change.

REMEMBER THIS
A case study becomes useful only after the reader separates the rule from the facts. Copy the questions and method—not the fictional couple's answer.

Case 1 — Rosa and Miguel: house rich, cash tight

Rosa entered a nursing facility. Miguel remained in the suburban home they had owned for thirty-one years. A broker estimated the home at $510,000; the mortgage payoff was $65,000. They also had $205,000 in nonexempt accounts. Miguel received $2,900 monthly income, and Rosa received $2,100.

First question: does the home enter the resource total?

Because Miguel continued to occupy the home, Illinois homestead policy kept it exempt even if the equity exceeded the ordinary home-equity limit. That did not make the home free to transfer, sell, rent, borrow against, or ignore. Title, mortgage, taxes, insurance, maintenance, later occupancy, and estate-recovery consequences still required separate review.[77]

Second question: what happens to the liquid resources?

Using the published 2026 figures, the couple began with $205,000 of nonexempt resources. Up to $143,172 could be protected as Miguel's CSRA, subject to the decision and required ownership work. Rosa's separate $17,500 AABD Medical resource standard still applied. The illustration therefore left $44,328 requiring classification, lawful spend-down, or other approved treatment before ordinary financial eligibility: $205,000 − $143,172 − $17,500.[78][79]

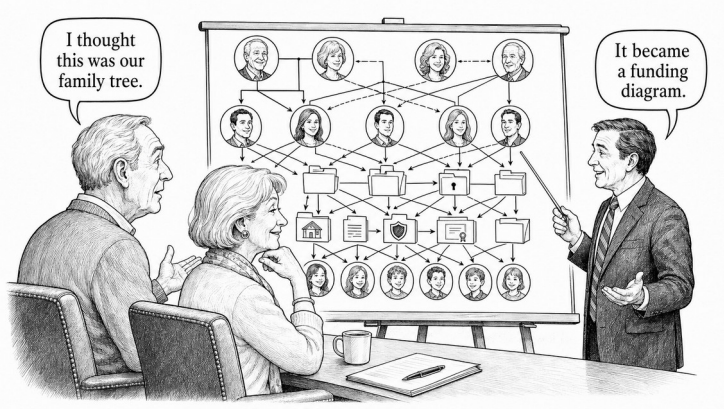
The plan

- **Preserve the house evidence.** Deed, mortgage payoff, tax bill, insurance, occupancy proof, valuation method, and repair forecast.
- **Protect cash flow, not just equity.** Build Miguel's twelve-month housing and living budget before spending liquid funds that may be needed to keep the home safe.
- **Spend only for documented value.** Facility charges, debts, repairs, equipment, burial arrangements, legal services, and other fair-market-value purchases must connect to receipts and the Chapter 12 file.
- **Do not add a child to the deed.** An ownership change can create transfer, tax, creditor, inheritance, and recovery problems. Obtain advice before changing title.

Rosa and Miguel were not poor on paper. Their danger was converting all available cash into eligibility while leaving Miguel with an expensive house and no repair reserve.

Case 2 — Elaine and Thomas: the blended-family ledger

Elaine and Thomas married in their late sixties and each had two adult children. Thomas owned a $90,000 certificate of deposit intended for his children. Elaine had $45,000 in savings and later needed Supportive Living Program care. They held a $25,000 joint household account. Their wills, powers of attorney, beneficiaries, and account titles had been created at different times.



In a second marriage, ownership and inheritance plans could not be treated as the same question.

THE FAMILY TREE BECOMES A FLOWCHART

For initial Medicaid analysis, "his," "hers," and "ours" did not create three separate eligibility silos. Illinois begins with total nonexempt resources owned by the long-term-care spouse, community spouse, or jointly by both. The $160,000 total had to be classified together before applying the current CSRA and Elaine's applicant resource standard.[79]

The conflict was not arithmetic

- **Eligibility objective.** Protect the approved CSRA for Thomas and complete only the ownership changes authorized by the decision.
- **Estate-plan objective.** Decide whether Thomas's children, Elaine's children, the surviving spouse, or a trust should receive each asset after death—and whether the current documents actually produce that result.
- **Authority objective.** Confirm who may act for each spouse, whether the agent faces divided loyalties, and who receives notices and account records.
- **Communication objective.** Document the couple's instructions without asking adult children to vote on property they do not own.

A Medicaid-allowable transfer does not resolve tax, beneficiary, probate, or family questions. Elaine and Thomas had to coordinate the plan before acting.[80]

Case 3 — Curtis and Anne: protected on paper, squeezed at home

Anne entered a nursing facility. Curtis remained in their modest home with gross monthly income of $1,650. The mortgage, property tax, homeowner's insurance, utilities, and required condominium assessment totaled $2,380 before food, transportation, prescriptions, repairs, or personal expenses. Anne received $3,100 per month from Social Security and a pension.

The income calculation

Illinois's published 2026 maximum Community Spouse Maintenance Needs Allowance is $4,066.50. Subtracting Curtis's $1,650 gross income produced a potential $2,416.50 monthly diversion from Anne's eligible non-SSI income. The deduction is not automatic: it is limited by Anne's available income, the ordering of other deductions, the amount actually contributed, and the final agency calculation.[81]

2026 maximum standard: $4,066.50 **Curtis's gross income:** − $1,650.00 **Potential diversion:** $2,416.50

The housing problem did not disappear

Even if the full potential diversion was approved and paid, Curtis needed a real cash-flow plan. Property tax could rise, the furnace could fail, and Anne's income might not be sufficient after required deductions. The State's formula was an eligibility protection; it was not a home-maintenance guarantee.

- **Prove every income source.** Gross award letters, deductions, pensions, annuities, and actual transfers into Curtis's account.
- **Build the housing file.** Mortgage, taxes, insurance, utilities, association charges, accessibility needs, and urgent repairs—even when the ordinary Illinois formula uses a published maximum.
- **Screen cost reductions.** Property-tax relief, energy assistance, Medicare savings, prescription help, veterans benefits, and other programs must be verified before being treated as dependable income.
- **Escalate a genuine shortfall.** A court order or fair-hearing strategy can be fact-specific. Preserve the deadline and obtain Illinois elder-law advice when the approved amount cannot sustain the community spouse.[79][81]

INCOME PROTECTION IS A CASH-FLOW TOOL
The CSMNA does not transfer ownership of resources, guarantee every housing bill, or create income the long-term-care spouse does not have. Track the approved amount and the payment itself.

Three cases, one disciplined method

Use the same sequence for every couple: confirm the Medicaid pathway, identify the two spouses' roles, establish the snapshot and application dates, classify every resource, apply the current standards, test monthly cash flow, complete only the approved ownership changes, and preserve notice and appeal deadlines.

Case	Pressure point	Rule that matters	Next document
Rosa / Miguel	Valuable home; limited liquid reserve	Occupied-home exclusion; CSRA; applicant standard	House file and 12-month cash-flow plan
Elaine / Thomas	Prior children and conflicting expectations	Total nonexempt resources; approved spousal transfer	Coordinated title, beneficiary, authority, and estate-plan review
Curtis / Anne	Low income; high fixed housing expense	CSMNA, actual contribution, notice and appeal rights	Income proof, housing-cost file, payment reconciliation

Case-Study Transfer Sheet

Couple / roles: _____

Care pathway: _____ **Application year:** _____

Snapshot resources: $_____ **Application resources: $**_____

Approved CSRA: $_____ **Approved CSMNA diversion: $**_____

Three questions before acting

- **What keeps the community spouse safe for twelve months?** Include income, liquid reserve, housing, transportation, health costs, and realistic emergencies.
- **What family expectation could the eligibility step disrupt?** Identify deeds, beneficiaries, prior-marriage promises, jointly held property, and decision authority.
- **What must be verified before the deadline?** List missing statements, valuation disputes, transfer proof, current annual figures, notice corrections, and professional-help triggers.

Bottom Line

The homeowners needed liquidity, the blended family needed coordination, and the low-income spouse needed sustainable cash flow. None could solve the case by memorizing one allowance. The durable method is to connect the current rule to the couple's property, income, family structure, housing reality, documents, and deadlines.

PART V

VA Pension with Aid & Attendance

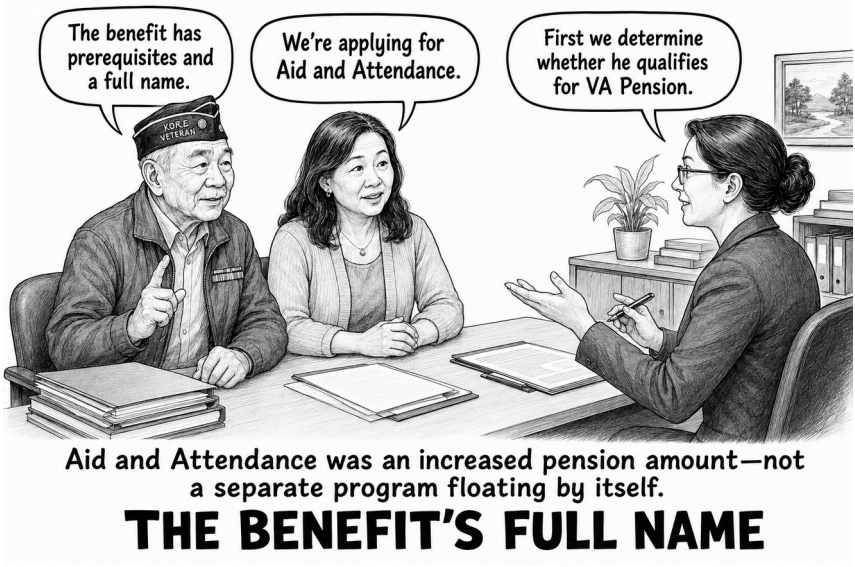
CHAPTER 19

Name the benefit correctly, identify the claimant, and prove every eligibility layer before counting an award.

CHAPTER 19

The Benefit Many Families Miss—or Misunderstand

Samuel Park served during the Korean War. Decades later, his daughter Grace heard that "Aid and Attendance" might help pay for care. She arrived at a benefits appointment ready to apply for it as though it were a separate program.



The representative's order mattered. VA Pension is the benefit pathway. Basic Pension is the starting payment level. Housebound and Aid and Attendance are higher pension levels for people who meet additional medical or functional criteria. A family cannot skip the pension requirements and qualify only because care is expensive.[82][84]

REMEMBER THIS
Say the whole question: "Does this Veteran—or this surviving spouse—qualify for VA Pension, and if so, for the Housebound or Aid and Attendance rate?"

Two doors begin with different questions

Samuel had always called every VA payment “a veterans benefit.” That everyday label concealed two different entrances. Grace needed to learn which claim she was screening before she gathered evidence or estimated cash flow.



VA Pension

Pension is needs-based. A Veteran generally must have qualifying wartime service and discharge, meet the current income and net-worth rules, and be age 65 or older or meet a qualifying disability or care condition. The disability does not have to be connected to military service. The payment is based on the applicable Maximum Annual Pension Rate, reduced by income for VA purposes.[82][85]

VA disability compensation

Compensation is based on a disability connected to military service. It may arise from wartime or peacetime service, does not use pension’s income and net-worth tests, and is driven by the VA disability rating and other compensation rules. A person cannot receive pension and compensation at the same time; if eligible for both, VA pays the greater benefit.[83]

For a survivor, keep a third door visible: Survivors Pension is needs-based, while Dependency and Indemnity Compensation is a separate survivor-compensation program. Do not use one program’s name, evidence, or financial rules as shorthand for the other.

One pension pathway, three possible payment levels

After Samuel passed the pension gateway, the next question was which rate matched his condition. The levels do not stack. A claimant may receive the applicable Basic Pension, Housebound, or Aid and Attendance rate—not all three added together.[84]

Basic Pension

The claimant satisfies the pension pathway but has not established the additional condition required for Housebound or Aid and Attendance. "Basic" describes the rate category, not whether the need is serious.

Housebound

The claimant receives pension and spends most of the time at home because of a permanent disability. Housebound is an increased pension level, not reimbursement for every hour spent inside.

Aid and Attendance

The claimant receives pension and meets at least one VA criterion, such as needing another person's help with activities of daily living, being bedridden for a large part of the day, living in a nursing home because of loss of mental or physical abilities, or having eyesight within the stated limits.[84]

Build evidence around function, not adjectives

- **Medical examination.** Ask the clinician to complete VA Form 21-2680 with diagnoses, limitations, mobility, cognition, and the help actually required.
- **Nursing-home evidence.** When applicable, obtain VA Form 21-0779 and supporting facility information.
- **Daily-care record.** Describe bathing, dressing, toileting, eating, transferring, medication supervision, safety cueing, and who provides each service.
- **Cost record.** Preserve invoices, contracts, proof of payment, insurance reimbursements, and the person for whom each unreimbursed medical expense was paid.

THE RATE FOLLOWS THE EVIDENCE
A diagnosis alone does not describe daily assistance. Match the form, care record, and expenses to the exact pension level being requested.

First choose the claimant: Veteran or survivor

Grace could not simply move Samuel's future claim into her mother's name. A Veteran's Pension claim and a surviving spouse's Survivors Pension claim have different claimants, forms, relationship evidence, income records, and effective-date questions.[82][86]

Veteran pathway

- **Service and discharge.** Obtain the DD-214 or other separation records; verify active-duty dates, character of discharge, required service length, and at least one day in a recognized wartime period.
- **Age or condition.** Confirm age 65 or older, permanent and total disability, qualifying nursing-home status, or Social Security Disability Insurance or Supplemental Security Income status.
- **Financial screen.** Inventory household income, assets, dependents, deductible unreimbursed medical expenses, and transfers during VA's 36-month look-back period.
- **Rate evidence.** Decide whether the file supports Basic Pension, Housebound, or Aid and Attendance before selecting forms and medical proof.

Surviving-spouse pathway

- **Relationship.** Prove the marriage and death. The current general rule requires the surviving spouse not to have remarried after the Veteran's death; confirm current exceptions rather than assuming.
- **Veteran's service.** Prove the deceased Veteran met the applicable wartime-service and discharge requirements.
- **Survivor's finances and care.** Use the survivor's income, net worth, dependents, medical expenses, and functional evidence—not the deceased Veteran's old household budget.
- **Claim date.** Ask whether an intent to file should be submitted while evidence is gathered. A timely intent to file may protect a potential effective date, but it is not an approval.[86]

A VA-accredited representative can help identify the correct claim and review the evidence. Accredited Veterans Service Organization representatives provide their services free; attorneys and claims agents may charge only in circumstances allowed by VA rules.[87]

VA Pension Screening Record

Complete this record before treating a possible benefit as part of the care budget. Verify current annual figures on VA.gov or with a VA-accredited representative; rates and limits change.

Question	Evidence / answer
Claimant and path	☐ Veteran Pension ☐ Survivors Pension ☐ Compensation/DIC question referred
Service record	Branch, active-duty dates, discharge, wartime day, DD-214 location
Age / condition	Age 65+, disability, nursing-home status, SSDI/SSI, or survivor status
Requested pension level	☐ Basic ☐ Housebound ☐ Aid and Attendance
Household finances	Gross income, dependents, assets, primary home/car, medical expenses
Prior 36 months	Gifts, below-market sales, retitling, trusts, annuities, asset transfers
Evidence and forms	21-2680; 21-0779 if applicable; care record; bills; relationship/death proof
Dates and help	Intent-to-file date, application date, accredited representative, follow-up log

Three funding labels

- **Possible.** The family sees a plausible pathway but has not filed. Keep the projected amount out of dependable income.
- **Pending.** The claim or intent to file has been submitted. Track dates and requests, but do not promise the money to the residence.
- **Awarded.** The written decision states the benefit, amount, effective date, and payment details. Reconcile the actual deposit and report later changes as required.

Professional-help triggers

Get individualized help when discharge or service dates are uncertain; compensation, DIC, or pension pathways overlap; assets approach the current limit; transfers occurred during the prior 36 months; marriage or remarriage history is complicated; trusts, annuities, businesses, or property are involved; or a denial or effective date may require review.

Bottom Line

The overlooked benefit is not a shortcut. It is a sequence: identify the claimant, separate pension from compensation, establish service and financial eligibility, prove the correct pension level, file on time, and count only the amount VA actually awards.

CHAPTER 20

Four Eligibility Tests

Samuel looked at the four locked checkpoints and tightened his grip on the pension application. “I thought being a veteran was the eligibility test.” Grace pointed to the first gate. “It’s the first eligibility test.”



The picture is simplified, but the sequence is useful. Gate 1 tests service and discharge. Gate 2 tests the claimant’s age or qualifying disability status. Gate 3 determines whether the evidence supports Basic Pension, Housebound, or Aid and Attendance. Gate 4 tests income, deductible expenses, net worth, and recent transfers under rules effective for the relevant claim period.[88][89]

REMEMBER THIS
Do not begin with the hoped-for payment. Begin with the four gates. A strong medical file cannot replace missing wartime service, and low income cannot replace the age-or-disability requirement.

Gate 1 — prove the service before calculating money

Grace found Samuel's cap, photographs, and medals immediately. The document that mattered first was his separation record. Family memory could describe the service; the DD-214 or other official record had to prove the dates, active-duty status, and character of discharge.

Length and wartime-day test

A Veteran who entered active duty before September 8, 1980 generally needs at least 90 days of active duty, including at least one day during a recognized wartime period. Later entrants are generally subject to longer minimum-duty rules—often 24 months or the full period called or ordered to active duty—with exceptions. Officers beginning active duty after October 16, 1981 can face a related rule. Verify the actual record instead of applying the 90-day shorthand to everyone.[88]

Wartime is a statutory date test

Combat is not required. Service must overlap a period Congress recognizes as wartime. Current periods most relevant to living Veterans include:

Recognized period	Dates used by VA
World War II	December 7, 1941–December 31, 1946
Korean conflict	June 27, 1950–January 31, 1955
Vietnam era—service in Republic of Vietnam	November 1, 1955–May 7, 1975
Vietnam era—service outside Republic of Vietnam	August 5, 1964–May 7, 1975
Gulf War	August 2, 1990–future ending date set by law or proclamation

Character of discharge

The current VA eligibility page says the claimant must not have received a dishonorable discharge. An other-than-honorable, bad-conduct, or dishonorable status may require a discharge-upgrade application or a VA Character of Discharge review. Do not promise eligibility or abandon the claim based only on a family label for the discharge.[88][94]

Gate 1 file: separation records; active-duty entry and discharge dates; branch; service-number variations; reserve or National Guard activation orders; discharge documents; and any upgrade or character-review decision.

Gate 2 — age or disability opens the pension pathway

Samuel was over 65, so Grace did not need to prove permanent and total disability merely to pass this gate. A younger claimant would need a different route. Under the current VA eligibility description, at least one of these conditions must be true:[88]

- **Age 65 or older.** Prove the date of birth. Age can satisfy this gate even when the Veteran's disability is not service connected.
- **Permanent and total disability.** Build medical and vocational evidence appropriate to the claim rather than assuming a diagnosis automatically establishes permanence and totality.
- **Nursing-home patient.** The Veteran is receiving long-term care in a nursing home because of a disability.
- **SSDI or SSI recipient.** Current receipt of Social Security Disability Insurance or Supplemental Security Income can satisfy the stated condition; preserve the award evidence.

Do not confuse this gate with compensation

For Pension, the qualifying disability does not have to arise from military service. Disability compensation asks whether a disability is service connected and uses a different payment system. If both routes may apply, screen both; VA does not pay Pension and disability compensation simultaneously and generally pays the greater benefit when a claimant is eligible for both.[95]

Gate 2 does not decide the pension level

Age 65 may establish the Basic Pension threshold, but it does not establish Housebound or Aid and Attendance; Gate 3 requires additional evidence. A Veteran may also need extensive daily help yet fail Pension at Gate 1 or Gate 4.

Gate 2 file: identity and birth-date proof; diagnoses, prognosis, functional limits, treatment records, and the appropriate clinician-completed form when disability is the route; current SSDI or SSI verification; or nursing-home documentation connecting long-term care to disability.

ONE DISABILITY, TWO DIFFERENT QUESTIONS
Pension asks whether age or a qualifying disability condition opens the needs-based pathway. Compensation asks whether a disability is connected to service. Do not answer one question with evidence prepared for the other.

Gate 3 — prove the care need at the level requested

Samuel's age could support the Basic Pension pathway. Grace requested the Aid and Attendance rate because he needed hands-on help and safety supervision. The residence invoice proved cost; it did not fully describe function.

Aid and Attendance

A claimant receiving VA Pension may qualify when at least one current criterion is met: another person is needed for daily activities such as bathing, feeding, or dressing; illness requires the claimant to remain in bed for a large portion of the day; the claimant is in a nursing home because of loss of mental or physical abilities; or vision falls within VA's stated limits.[90]

Housebound

A pension claimant may qualify for the Housebound level when a permanent disability causes the person to spend most of the time at home. Housebound and Aid and Attendance cannot be paid at the same time.[90]

Translate care into evidence

- **Activities of daily living.** Record bathing, dressing, eating, toileting, transferring, and ambulating—not merely "needs assistance."
- **Cognition and safety.** Describe cueing, wandering risk, medication supervision, judgment, and protection from hazards, with frequency and examples.
- **Who provides the help.** Identify residence staff, paid attendants, clinicians, and family; connect schedules and invoices to the care actually delivered.
- **Forms.** Use VA Form 21-2680 for the examination information and VA Form 21-0779 when the claimant is in a nursing home.[90]

Care setting is not the test by itself

Living in assisted living does not automatically establish Aid and Attendance, and living at home does not rule it out. The decision follows the claimant's condition and evidence. Facility contracts should separate lodging, meals, health care, custodial care, and optional services so the medical-need and expense analyses can be supported.[90][92]

DESCRIBE THE TUESDAY, NOT THE DIAGNOSIS
A diagnosis names the condition. The stronger file explains what happens during bathing, medication time, transfers, meals, nighttime safety, and an ordinary Tuesday without help.

Gate 4A — turn gross income into income for VA purposes

VA starts with household income sources such as Social Security, retirement payments, annuities, earnings, and dependent income, then applies authorized exclusions and deductions. Pension is generally the applicable Maximum Annual Pension Rate minus income for VA purposes. The MAPR depends on dependents and whether the claimant qualifies for Basic Pension, Housebound, or Aid and Attendance.[89]

A dated illustration—not a promise

For December 1, 2025 through November 30, 2026, the MAPR for a Veteran with no dependents is $17,441 at the Basic level, $21,313 at Housebound, and $29,093 at Aid and Attendance. The published five-percent medical-expense threshold for that claimant is $872. Different household and rate facts produce different figures.[89]

Illustration: no-dependent Veteran at Aid and Attendance level

Calculation	Amount
Current MAPR	$29,093
Gross annual income	$30,000
Paid qualifying unreimbursed medical expenses	$16,000
Published medical-expense threshold	− $872
Potential expense deduction	$15,128
Illustrative income for VA purposes	$14,872
Illustrative annual pension	$29,093 − $14,872 = $14,221

The example assumes the expenses qualify, were paid in the relevant annualization period, were not and will not be reimbursed, and are properly attributable. VA makes the actual calculation.

What can belong in the medical-expense file

- **Common categories.** Health and long-term-care insurance premiums; medical providers; medications, supplies, and equipment; qualifying medical transportation; and certain institutional or in-home care expenses.[92]
- **Assisted-living caution.** Meals, lodging, and assistance with instrumental activities are not automatically medical expenses. The regulation connects deductibility to health care, custodial care, protected-environment evidence, and the claimant's A&A or Housebound status.[92]
- **Proof.** Preserve the invoice, service period, recipient, payer, payment date, amount, purpose, provider, reimbursement, and supporting care statement. VA Form 21P-8416 reports paid medical or dental expenses that were not and are not expected to be reimbursed.[93]

Gate 4B — net worth needs both a number and a date

Grace found three saved rate sheets. Each was accurate for a different period. The largest figure was not necessarily the correct one; the claim needed the rule effective for its own measurement period.



Current dated rule

From December 1, 2025 through November 30, 2026, the VA Pension net-worth limit is $163,699. VA's public explanation combines assets with annual income for VA purposes and includes the claimant's dependents as applicable. The limit and MAPRs are adjusted, so every worksheet should show the source page, access date, and effective period.[89]

Classify before totaling

- **Commonly included.** Bank and investment accounts, non-primary real estate, boats, and other countable property at fair market value, less qualifying debt tied to the property.
- **Commonly excluded.** The primary residence under the applicable rule, a vehicle, and basic household items. Sale proceeds and later transactions can change the analysis.[89][96]
- **Income belongs in the net-worth analysis too.** Do not compare assets alone with the published limit and declare the gate passed.
- **Transfers require their own review.** VA reviews certain transfers during the three years before the claim. A below-market transfer that pushed net worth above the limit may create a penalty period of up to five years.[89]

Do not move, gift, retitle, sell, or annuitize property merely to reach the published number. Coordinate VA Pension, Medicaid, tax, creditor, housing, and estate consequences before acting.

Four-Gate Eligibility Record

Mark a gate "verified" only when the evidence is in the file. A plausible answer is a lead, not a decision.

Gate	Verification question	Evidence / status
1	Do service, wartime day, minimum duty, and discharge pass?	☐ Verified ☐ Missing ☐ Review
2	Does age, disability, nursing-home status, SSDI, or SSI pass?	☐ Verified ☐ Missing ☐ Review
3	Which level does the functional evidence support?	☐ Basic ☐ Housebound ☐ A&A
4A	What is income for VA purposes after supported deductions?	$_____ Period: _____
4B	What is net worth under the effective rule?	$_____ Limit: $_____

Current-Rule Date Card

Source page: _____

Accessed on: _____ **Rule effective:** _____ to _____

MAPR category / amount: _____

Medical-expense threshold: $_____ **Net-worth limit:** $_____

Medical-Expense Record

Paid / person	Provider and purpose	Gross / reimbursed	Proof
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

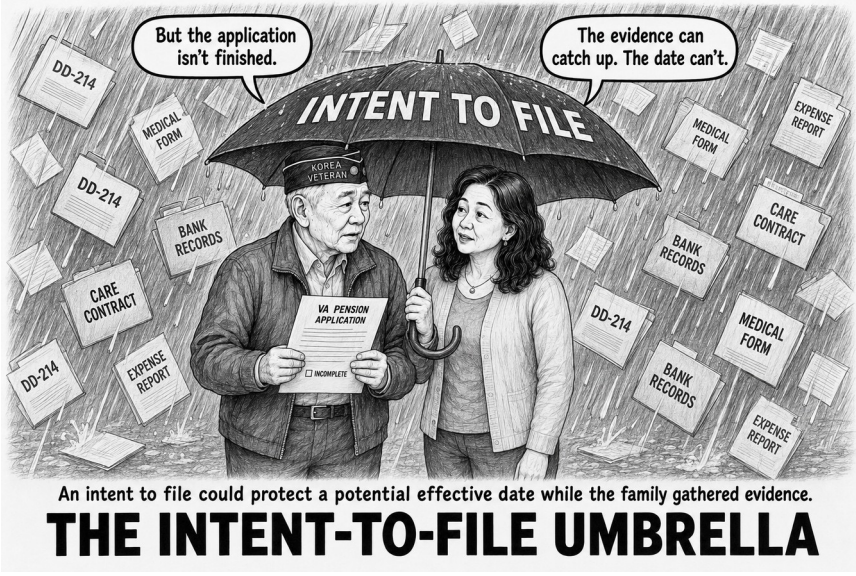
Bottom Line

The durable screening method is sequential: prove service and discharge, prove age or qualifying disability, prove the requested care level, then calculate income and net worth with supported medical expenses under a dated rule. Four verified gates create a claim worth filing; four assumptions create a surprise.

CHAPTER 21

Applying Safely

Samuel watched service records, medical forms, care contracts, bank records, and expense reports fall around him. “But the application isn’t finished.” Grace opened the intent-to-file umbrella. “The evidence can catch up. The date can’t.”



The umbrella buys organization time, not eligibility. VA Form 21-0966 or another recognized intent method can establish a potential effective date, but the correct completed claim must follow on time and VA must approve it. The safer strategy runs on two tracks: protect the date and build a claim that can be reproduced, verified, and defended.[98]

REMEMBER THIS
An intent to file is not the application, an effective date is not an approval date, and a projected award is not dependable income. Record each one separately.

Build one evidence file before choosing the submit button

Grace created a master folder before opening the online application. That prevented the form from becoming the family's only copy of the facts and exposed contradictions while they could still be corrected.

The six-part Pension evidence file

- **Identity and authority.** Claimant's legal name, Social Security number, VA file number if any, birth information, contact details, and proof that anyone signing or acting has authority.
- **Military service.** DD-214 or other separation records, active-duty dates, branch, wartime overlap, discharge character, and any reconstructed-record or character-review evidence.
- **Family status.** Marriage and prior-marriage history for both spouses, dependent information, death records when relevant, and names reconciled across documents.
- **Income and assets.** Gross monthly income by source, account and property values, ownership, debts, dependents' finances, transfers, and the effective-date worksheet from Chapter 20.
- **Medical and functional evidence.** Diagnoses, prognosis, daily assistance, cognition, safety, setting, provider, and clinician-completed forms tied to the requested rate level.
- **Care expenses.** Contracts, invoices, service periods, proof of payment, reimbursements, insurance, provider credentials, and the Medical-Expense Record.

Application and supporting forms

- **VA Form 21P-527EZ.** Current Application for Veterans Pension. A surviving-spouse claim uses the appropriate survivor application instead.[97]
- **VA Form 21P-0969.** Income and Asset Statement when the application or VA instructions call for it, including a separate form for each year when multiple years must be reported.[97]
- **Supporting forms.** Use 21-2680, 21-0779, 21P-8416, appointment forms, and other forms only for the function they actually serve.

VA currently permits online filing, PDF upload through QuickSubmit, mailing the paper application, in-person delivery, or help from an accredited professional. Select one controlled submission path, preserve the receipt or tracking evidence, and keep an exact copy of everything sent.[97]

The medical form must describe care, not merely name illness

Grace brought Samuel's clinician a one-page factual summary: diagnoses, medications, mobility, cognition, and the help he required with bathing, dressing, eating, toileting, transferring, walking, and safety. She did not ask the clinician to sign a blank form or copy a sales brochure.

VA Form 21-2680

For an Aid and Attendance or Housebound request, a medical examiner completes the examination-information section. The final form should be legible, internally consistent, and specific about limitations, frequency of help, ability to leave home, and the reason assistance is needed. Compare it with the residence assessment and care plan before submission.[99]

VA Form 21-0779

When the claimant is in a nursing home, VA identifies Form 21-0779 as additional facility information. Do not use assisted-living terminology as a substitute for the form's nursing-home facts.[99]

VA Form 21P-8416 and the expense packet

Form 21P-8416 reports paid medical or dental expenses for the claimant or a household family member when the expenses were not reimbursed and are not expected to be reimbursed. The form summarizes; the evidence packet proves.[100]

- **Connect charge to care.** Match the invoice or contract to health care, custodial care, qualifying transportation, insurance, equipment, medication, or another supported category.
- **Connect care to person and period.** Show who received it, who provided it, dates of service, and the annualization period used in the Pension calculation.
- **Connect charge to payment.** Bank, card, or canceled-check evidence should show who actually paid; subtract insurance, refund, family repayment, or other reimbursement.
- **Reconcile recurring estimates.** Keep monthly proof and report material changes. A projected facility cost is not automatically the final deductible amount.

Medical-Form Quality Check

Diagnosis stated: ☐ **ADLs described:** ☐ **Cognition / safety:** ☐
Frequency of help: ☐ **Provider identified:** ☐ **Form signed / dated:** ☐
Care plan agrees: ☐ **Invoice agrees:** ☐ **Copy retained:** ☐

An intent to file starts a clock, not a benefit

The current VA guidance gives the claimant one year after notifying VA of an intent to file to complete and file the claim. After the completed claim is filed, that intent is no longer active. Only one intent can be active at a time, and a claimant planning different benefit types may need a separate intent for each.[98]

Choose the correct date lane

- **Verified online Pension application.** Starting the form while signed in with an identity-verified account can automatically reserve an earlier potential start date; VA says a separate intent is not needed for that online path.[97][98]
- **PDF, mail, or unverified account.** Consider a separate intent before gathering the complete paper claim. Confirm VA processed it and record the potential effective date VA provides.
- **Wrong benefit type.** An intent connected to compensation does not automatically preserve a Pension date. Name the intended benefit accurately.
- **One-year deadline.** Calendar reminders should begin well before the anniversary. Missing the deadline does not necessarily bar a later claim, but the protected potential date can expire.

Three dates that families confuse

Date	What it means	What it does not mean
Intent received	Potential effective-date anchor; one-year completion clock	Not a completed application or eligibility decision
Claim received	VA has the formal application; evidence and follow-up continue	Not necessarily the eventual effective date or award date
Decision / award	Written determination states outcome, rate, and effective date	Not proof that every projected deposit has arrived

Intent-to-File Control Card

Benefit type: _____ **Method:** _____

Submitted: _____ **VA processed / potential date:** _____

One-year deadline: _____ **90-day warning:** _____

PROTECT THE DATE AND THE PROOF

Save the confirmation screen, mailed-form copy and tracking, call record, or representative confirmation. A date remembered but not documented is a date the family may have to reconstruct.

A helper needs accreditation—not your password

The consultant's badge sounded official. His request did not. When he asked for Samuel's VA login, bank password, and Social Security number, Grace closed the laptop.



VA recognizes three accredited claim helpers: VSO representatives, attorneys, and claims agents. Search the VA directory before sharing the claim. A VSO representative's claim services are always free. Attorneys and agents may charge only as VA rules allow; understand the appointment, scope, and fee documents before signing.[101]

Appointment must match the helper

- **VSO representative.** VA Form 21-22 appoints the organization; ask which organization name belongs on the form.
- **Attorney or claims agent.** VA Form 21-22a appoints the individual. Confirm the exact person—not merely the marketing company—is accredited.

Access boundaries

- **The claimant controls the account.** Do not share VA.gov, email, banking, or identity-provider passwords. Use lawful appointment, authorization, and document-sharing methods instead.
- **Share the minimum needed.** Redact unrelated account numbers where appropriate, transmit securely, and keep a list of what was given to whom and why.
- **Review before submission.** Never sign a blank form or allow an unknown person to change direct deposit, contact details, ownership, or beneficiaries without informed authorization.[102]

Separate claim help from a product sale

VA warns that "pension poaching" can involve pressure to hide or move assets into trusts or annuities sold by the adviser. The maneuver may create fees, transfer penalties, repayment exposure, Medicaid problems, or loss of access to money needed for care.[102][103]

Stop signs

- **A guaranteed approval, benefit amount, or faster decision.** Only VA decides eligibility and the award.
- **A filing fee, percentage of future benefits, or upfront payment for an initial claim.** Verify accreditation and fee authority; free accredited VSO assistance is available.
- **An urgent trust, annuity, investment, deed, or account transfer.** Pause for independent VA, Medicaid, tax, and elder-law review before changing ownership or control.
- **Passwords, blank signatures, redirected deposits, or secrecy.** Close the screen, preserve the solicitation, and verify through official channels.

Benefits-Helper Verification

Name / organization: _____

VA accreditation search checked: ☐ **Date:** _____ **Result saved:** ☐

Role: ☐ VSO ☐ Attorney ☐ Claims agent ☐ Not a claim representative

Appointment form: ☐ 21-22 ☐ 21-22a ☐ None **Written fee terms reviewed:** ☐

Products / commissions disclosed: ☐ **Independent review completed:** ☐

Claim Submission Log

Date	Item / pages	Method / recipient	Receipt / follow-up
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

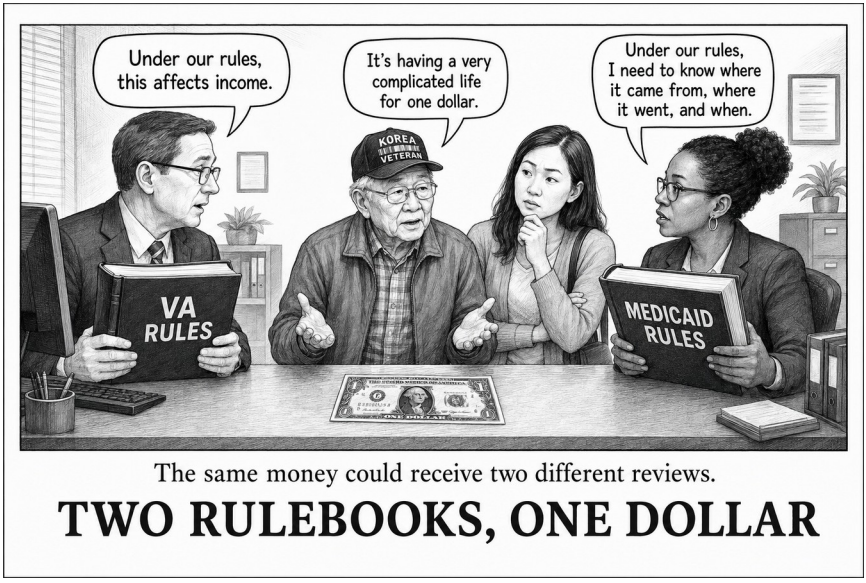
Bottom Line

Apply safely by protecting the potential date, submitting the correct forms, proving function and expenses, preserving a complete copy and receipt, verifying every representative, and refusing any shortcut that requires secrecy, surrender of passwords, or an unreviewed product transaction.

CHAPTER 22

Coordinating VA and Medicaid

Samuel placed one dollar between two rulebooks. The VA reviewer said, “Under our rules, this affects income.” The Medicaid reviewer wanted to know where it came from, where it went, and when. Samuel sighed. “It’s having a very complicated life for one dollar.”



VA Pension and Illinois Medicaid can examine the same household, care bill, transfer, trust, or deposit without reaching the same classification. Each program has its own purpose, definitions, measuring periods, application date, evidence demands, and consequences. Approval by one agency neither proves eligibility for the other nor supplies permission to rearrange assets.[104][105]

REMEMBER THIS
Coordinate the facts, not the conclusions. Give both programs the same complete history, then let each program apply its own rules in writing.

Translate every fact into both rulebooks

Grace stopped using shorthand such as “exempt,” “medical expense,” and “allowed transfer” without naming the program. She built a crosswalk instead. The goal was not to force identical answers; it was to prevent an answer from one system from being repeated inaccurately in the other.

Coordination crosswalk

Question	VA Pension lens	Illinois Medicaid LTC lens
What is measured?	Net-worth and income rules for Pension, with VA-specific exclusions and deductions.	Eligibility resources, income, transfers, and—after eligibility—the amount payable toward care.
How far back?	VA currently reviews certain asset transfers during the 3 years before a Pension claim.	Illinois LTC and specified waiver transfer rules use a 60-month review for covered transfers.
What can care costs do?	Qualifying unreimbursed medical expenses may reduce income for VA Pension purposes.	Medical expenses may affect spenddown or other calculations, but Medicaid applies its own rules and order.
What does an award change?	The VA decision establishes the benefit under VA rules.	The award must be reported; Medicaid determines its own treatment and contribution-to-care effect.

A 3-year review is not a shortened Medicaid review

VA’s current Pension rule reviews certain transfers during the three years before the claim and can impose a Pension penalty when its conditions are met. Illinois long-term-care Medicaid applies a 60-month rule to covered transfers for less than fair market value by an institutionalized person or spouse. The penalty tests, exceptions, calculation, and starting rules are not interchangeable.[104][106]

Build one dual-program ledger

- **Record the event once.** Date, amount, source, destination, owner before and after, fair-market value, purpose, and supporting document.
- **Add two analysis columns.** VA treatment / source and Medicaid treatment / source. Write “unresolved” until each answer is verified.
- **Keep annual figures dated.** Attach the effective period to every VA rate, Medicaid allowance, divisor, income standard, and resource standard; never paste an undated number into the plan.

DIFFERENT WINDOWS, SAME HISTORY
Disclose the full transaction history requested by each program. A transfer outside one program’s review window may remain important to the other.

Sequence applications without inventing a guaranteed order

Families often ask whether VA or Medicaid should be filed first. There is no universal answer. The safer sequence begins with the care setting, likely Medicaid pathway, current resources, transfer history, VA claim readiness, cash-flow deadline, and the dates each program can protect.

Run the sequence in five controlled steps

- **1. Identify the Medicaid route.** Institutional Medicaid, Supportive Living, or a home- and community-based waiver can involve different service, financial, and timing questions.[105]
- **2. Freeze risky transactions.** Before filing or moving property, list gifts, deed changes, joint accounts, trusts, annuities, loans, caregiver payments, and sales below market value.
- **3. Protect filing dates deliberately.** Record any VA intent-to-file date and the Medicaid application date separately. Neither date substitutes for the other.
- **4. Model cash flow in stages.** Show private-pay costs before either approval, after a possible VA award, after Medicaid eligibility, and after any patient-pay or room-and-board obligation.
- **5. Report decisions and changes.** Send award letters, effective dates, retroactive amounts, expense changes, and facility changes through each program's required channel; retain proof.

Do not subtract the same care bill twice by intuition

VA may use qualifying, paid, unreimbursed medical expenses when determining Pension income. Illinois Medicaid may use medical expenses in a spenddown analysis and, after long-term-care eligibility, generally applies income toward care after permitted deductions. Those are different calculations. Preserve the gross benefit, each expense, each reimbursement, and the program's written treatment rather than reporting only a family-created net number.[107][108]

Award-and-Change Record

Program: _____ **Claim / case number:** _____

Applied: _____ **Decision:** _____ **Effective date:** _____

Gross monthly amount: _____ **Retroactive payment:** _____

Other program notified: ☐ **Date / method:** _____ **Proof saved:** ☐

> **NO SILENT OFFSET**
> Report the VA award and the underlying expenses as requested. Do not assume Medicaid sees only the remainder after the family's VA calculation.

Know when coordinated expert review is mandatory

Grace could organize records and ask precise questions. She could not safely turn a VA strategy into a Medicaid strategy by analogy. The family paused before action whenever a decision could change ownership, access, timing, income, or the care-payment plan.

Stop before signing, transferring, or filing when:

- **A transfer is proposed or already occurred.** Gift, below-market sale, forgiven debt, new joint owner, beneficiary change, deed change, caregiver payment, or return of transferred property.
- **A product or entity is involved.** Trust, annuity, promissory note, life estate, business interest, prepaid contract, or adviser-sold "VA qualification" arrangement.
- **The applicant is married or the family is blended.** Spousal allowances, ownership, elective-share rights, support duties, beneficiaries, and estate plans can collide.
- **The award may be retroactive or the setting may change.** A lump sum, move to a nursing facility, Supportive Living admission, waiver enrollment, hospitalization, or death can alter reporting and cash flow.
- **The agencies disagree—or the explanation is oral.** Request the rule, calculation, and notice in writing; preserve appeal deadlines while obtaining coordinated advice.
- **There is not enough cash to survive the pending period.** A technically plausible sequence is not safe if it leaves the applicant unable to pay for care before decisions arrive.

Bring one brief to every adviser

Care setting / expected move date: _____

VA status / protected date: _____

Medicaid route / application status: _____

Transfers in last 60 months: ☐ None known ☐ Listed with documents

Proposed transaction: _____

Questions answered for both programs: ☐ Eligibility ☐ Income ☐ Transfer ☐ Timing ☐ Tax / estate

Bottom Line

VA Pension and Illinois Medicaid can work together, but they do not merge. Coordinate the timeline, disclose the same complete facts, keep separate calculations, report each decision, and obtain cross-program review before any transaction that cannot be easily undone.

PART VI

The Illinois Supportive Living Program

CHAPTERS 23–26

Identify the program correctly, verify the community and services, and coordinate the financial transition before relying on an SLP apartment.

CHAPTER 23

What SLP Is—and Is Not

The residence representative divided the bill in two. Grace pointed to supportive services. “Medicaid helps with this side.” Samuel pointed to room and board. “And this side still knows my address.”



Supportive Living assistance did not make housing free.

THE BILL HAS TWO SIDES

Illinois’s Supportive Living Program—SLP—is a Medicaid-supported, apartment-style alternative for people who otherwise may need nursing-facility care. It combines housing with personal, health-related, and supportive services while emphasizing choice, privacy, dignity, autonomy, and independence. But Medicaid payment for SLP services does not erase the resident’s housing, meal, personal-needs, or calculated income responsibilities.[109][110]

REMEMBER THIS
SLP is a care-and-housing model with two payment streams. Medicaid pays its approved share of covered services; the resident remains responsible for room and board and any assigned liability.

Use the formal program name, not the marketing adjective

A residence may describe itself as "supportive," "assisted," or "service enriched" without being certified in Illinois's Supportive Living Program. Grace checked the current HFS operational-site list rather than relying on a brochure, website phrase, or salesperson's promise.

SLP is

- **A Medicaid home- and community-based services pathway.** HFS administers the program as an alternative to traditional nursing-facility care for qualifying older adults and adults with physical disabilities.[109]
- **A residential setting.** Residents occupy apartment-style housing and direct their lives and services within program and site rules.
- **A defined service package.** Certified providers must deliver or coordinate required personal, health-related, safety, meal, housekeeping, and social supports.
- **An assessed fit.** The State screening and the provider's own assessment answer different questions: program-level need and whether that particular site can safely meet the person's needs.

SLP is not

- **Every assisted-living residence.** Only certified participating settings —and, within them, the population the site is designated to serve— belong to the program.
- **A nursing facility.** SLP is residential and supportive; its services do not convert the apartment into round-the-clock skilled nursing care.
- **Free housing.** Meals belong to room and board under the SLP payment structure, and the Medicaid resident has a monthly room-and-board charge.[110][111]
- **Automatic admission.** Medicaid eligibility, functional screening, site assessment, unit availability, contract terms, and move-in readiness remain separate.
- **A promise that needs can always be met.** A provider may admit or retain only residents whose needs can be met through the SLP service model.[110]

Four questions for the first telephone call

1. Are you currently certified and operational in SLP? ☐

2. Which population is this site designated to serve? ☐ 65+ ☐ Physical disability, 22–64

3. Are you accepting Medicaid applicants now? ☐

4. Is there an available appropriate apartment—or a wait list? ☐

One application can open into five separate doors



Door 1 — age or disability group

The statewide program identifies two populations: people age 65 or older, and people ages 22 through 64 with a physical disability determined by the Social Security Administration. Each SLP provider designates which population it serves; a site serving one group is not automatically an option for the other.[109][110]

Door 2 — Medicaid eligibility

The applicant must satisfy the applicable Illinois Medicaid rules, including nonfinancial and financial requirements. A private-pay resident is not automatically approved for Medicaid SLP payment merely because the person already lives at the site.[110]

Door 3 — nursing-facility level of care

For Medicaid SLP payment, the appropriate State-designated screening agency must find that the person needs nursing-facility level of care and that SLP is appropriate. The screening measures need; it does not require the person to live in a nursing home or make SLP a nursing facility.[109][110]

Doors 4 and 5 — participating site and placement

The site must be certified for the applicant's population, assess that it can meet the person's needs, and have an appropriate placement. Program eligibility does not reserve an apartment or override a lawful clinical or safety assessment.

Covered services must become an individual service plan

The SLP service menu is broad, but it is not self-executing. The provider must assess the resident, consult with the resident, and maintain a service plan that responds to actual needs while promoting independence and wellness.[109][110]

The core service menu

Health, care, and safety	Daily living and community
Intermittent nursing services	Meals and snacks
Medication oversight	Laundry and housekeeping
Personal care and ADL assistance	Maintenance
24-hour response/security staff	Social and recreational programming
Emergency call system	Health promotion and exercise
Well-being checks and ancillary services	Resident choice and service-plan participation

Translate the menu into frequency and responsibility

- **What exactly will staff do?** Name the assistance, cueing, supervision, medication task, response plan, or nursing follow-up.
- **When and how often?** Separate scheduled help from on-request response; identify night, weekend, and emergency coverage.
- **Who provides it?** Identify licensed nursing functions, direct-care tasks, outside clinicians, pharmacy roles, and family contributions.
- **What happens after a change?** Require reassessment after hospitalization, falls, cognitive decline, medication changes, or a new assistance need.

Service-Plan Reality Check

Need described: ☐ **Task assigned:** ☐ **Frequency stated:** ☐
Night response: ☐ **Medication role:** ☐ **Change trigger:** ☐
Resident agreed: ☐ **Family copy saved:** ☐ **Review date:** _____

A MENU IS NOT A SCHEDULE
A listed service becomes dependable only when the plan states the resident's need, the provider's task, the frequency, and the response when needs change.

Room and board remains the resident's responsibility

The cartoon's dividing line is financial, not merely administrative. Medicaid reimburses the approved SLP service component. The resident pays the room-and-board charge and may also owe a group care credit or patient liability based on countable income after allowed deductions.[111]

Read the monthly calculation in order

- **1. Start with countable income.** List Social Security, pensions, VA benefits, and other income exactly as the Medicaid notice classifies them.
- **2. Apply allowed deductions.** The current HFS explanation identifies items such as eligible medical expenses, health-insurance premiums, the personal-needs allowance, and SLP room and board.
- **3. Identify room and board.** Every SLP resident has a monthly charge; HFS ties Medicaid-resident amounts to SSI-related figures and issues updates when the controlling amounts change.[111][112]
- **4. Identify group care credit.** Remaining countable income may become the resident's service contribution. Medicaid pays the balance of the approved service rate.
- **5. Reconcile the residence bill to the State notice.** Room and board, group care credit, uncovered medical costs, deposits, and optional personal charges should not be blended into one unexplained number.

What the resident should receive and retain

- **A written resident contract.** Confirm housing terms, room-and-board charge, deposits, absence rules, optional charges, discharge provisions, and notice of increases.
- **A current eligibility or liability notice.** Save the effective month and the income and deduction figures used.
- **A protected personal amount.** HFS currently publishes the applicable personal-needs amount; because it can change, date the figure and verify the newest State guidance rather than relying on an old worksheet.

SLP Monthly Bill Reconciliation

Room and board: _____ Group care credit / patient liability: _____

Uncovered medical expenses: _____ Optional personal charges: _____

Total residence bill: _____ Medicaid notice agrees: ☐

Test the program's limits before choosing the apartment

A pleasant apartment and Medicaid eligibility do not answer whether SLP can safely meet the resident's present and foreseeable needs. The provider may admit or retain only people whose needs fit the services available through the program and that site.[110]

Limit questions families should ask

- **Nursing intensity.** Which needs require outside home health, clinic, physician, or hospital care? What conditions or frequency exceed intermittent nursing capacity?
- **Transfers and mobility.** How many staff can assist, what equipment is permitted, and what happens if the resident later needs more help?
- **Cognition and behavior.** Can the site support wandering risk, cueing, medication refusal, night disruption, or dementia—and is a certified dementia-care setting required?
- **Other waiver enrollment.** A Medicaid SLP resident may not simultaneously participate in another federal HCBS waiver program; resolve transitions before services stop.[109][110]
- **Temporary absence and higher care.** Ask what happens during hospitalization, rehabilitation, vacation, or nursing-facility admission and who continues paying room and board.
- **Discharge and appeal.** Obtain the criteria, notice process, relocation support, complaint route, and deadlines before a crisis.

SLP Fit and Payment Sheet

Decision point	Evidence / answer	Status
Age / disability population	_____	☐ Open ☐ Verified
Medicaid eligibility	_____	☐ Open ☐ Verified
Level-of-care screening	_____	☐ Open ☐ Verified
Site certification / available unit	_____	☐ Open ☐ Verified
Needs within site capacity	_____	☐ Open ☐ Verified
Room, board, liability, and cash flow	_____	☐ Open ☐ Verified

Bottom Line

SLP can preserve independence and make supportive services affordable for the right person in the right certified setting. It is not universal assisted living, free housing, a nursing facility, or an automatic placement. Verify every door, every service, every resident charge, and the site's capacity before relying on it.

CHAPTER 24

Finding and Comparing SLP Communities

Grace put the State provider list on speakerphone. “You’re listed as a Supportive Living community.” The admissions operator answered, “We are.” Samuel leaned toward the phone. “And do you have a Medicaid apartment available?” The operator paused. “That was a different question.”



The operator was right. An official listing establishes an important fact, but not every fact the family needs. A site may be certified and operational yet have no appropriate apartment, maintain a waitlist, serve a different age or disability group, or lack the dementia setting or staffing pattern Samuel needs. The family must verify the program, the placement, and the fit separately.[113]

REMEMBER THIS
Listed is not available. Available is not appropriate. Appropriate is not affordable until the Medicaid status, resident payment, and contract are confirmed in writing.

Build the search from the official list outward

HFS publishes a county-based locator and links to operational sites and a county table. Start there, date the search, and save the page used. Provider participation and site details can change; a private referral website or an old brochure should not be the family's source of truth.[113]

Search in widening circles

- **Home county.** List every operational site that serves the applicant's age or disability group. Do not assume the closest community has the right designation or an opening.
- **Adjacent counties.** Add communities within a realistic driving radius. County borders matter less than transportation, family access, physicians, dialysis, pharmacy delivery, and emergency travel.
- **A second geographic ring.** Use it when local waitlists are long or specialized dementia capability is scarce. Calculate the family's total travel burden, not mileage alone.

Ask one precise availability sequence

- **Program status.** Are you currently certified and operational in the Illinois Supportive Living Program? Which population is this site designated to serve?
- **Medicaid participation.** Are you accepting new residents whose SLP services will be paid through Medicaid—not merely residents who may apply later?
- **Apartment status.** Is an appropriate Medicaid apartment open now? If not, is the list for all apartments, Medicaid-designated apartments, or a particular care setting?
- **Waitlist mechanics.** What places a person on the list? Is an assessment, application, deposit, Medicaid approval, or complete medical packet required? How is priority determined?
- **Proof and follow-up.** Who maintains the list, when was it last updated, and when should the family call again? Get the contact name and confirm the next step by email.

A waitlist position is not a move-in promise. Record the date, stated position or status, prerequisites, deposits, refund terms, documents submitted, contact person, and next follow-up. Keep several communities active until an apartment, acceptance decision, payment plan, and move-in date are all confirmed.

Match the person to the certified population and care model

The HFS county pages distinguish ordinary operational SLP settings from designated dementia care settings and may identify sites serving adults ages 22–64 with physical disabilities. Those labels are not interchangeable. The same street address can contain different programs, units, contracts, and admission rules.[113]

When dementia is part of the picture

A general SLP may support a resident with mild cognitive impairment, but that does not make it a certified dementia care setting. Illinois's dementia-unit rules address residents age 65 or older with a physician-confirmed diagnosis, secure design, medication administration, dementia-appropriate activities, welfare checks, staff ratios, and dementia-specific training.[114]

- **Name the setting.** Ask whether the proposed apartment is in the HFS-listed dementia care setting, the general SLP population, or another licensed area of the campus.
- **Describe behavior, not only diagnosis.** Discuss wandering or exit seeking, nighttime wakefulness, agitation, medication refusal, unsafe cooking, falls, transfer needs, continence, and ability to summon help.
- **Test future fit.** Ask what change would trigger reassessment, a move within the campus, outside services, emergency evaluation, or discharge.
- **Verify the handoff.** If a resident later needs the dementia unit, is there a separate waitlist, assessment, contract, apartment charge, or Medicaid approval step?

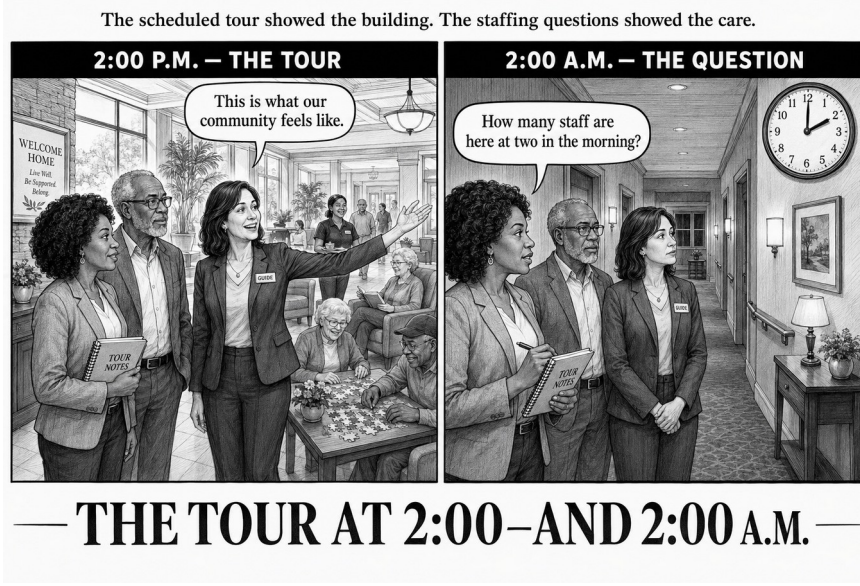
The capability interview

Give each community the same one-page needs summary: diagnoses; medications; mobility and transfer assistance; cognition and behaviors; ADLs; nighttime needs; recent falls or hospitalizations; dietary needs; and current outside services. Ask the nurse or assessor—not only sales staff—to state what the site can provide now, what requires an outside provider, and what it cannot safely provide.

REMEMBER THIS
A memory-care label is marketing unless it matches the actual certified setting, resident population, staff model, service plan, and apartment being offered.

Tour the staffing plan, not only the apartment

At two in the afternoon, Grace saw a bright lobby, scheduled activities, and several staff members. The useful question came later: who would answer Samuel at two in the morning?



Illinois requires sufficient licensed and certified staff to meet the population's needs and 24-hour response staff for scheduled, unpredictable, and emergency calls. The general SLP rule does not replace a site-specific answer about actual staffing. Dementia care settings have additional requirements, including at least one CNA for every ten dementia-unit residents on all shifts, awake CNAs assigned to that unit, and licensed-nurse availability on-site or on-call for medication needs.[115]

Ask by shift and by job

- **Presence.** How many CNAs, nurses, response staff, and supervisors are physically present on days, evenings, nights, weekends, and holidays?
- **Assignment.** Are any of those staff shared with another floor, building, general SLP population, independent-living area, or dementia unit?
- **Response.** Who answers call systems, how response is documented, and what happens when two residents need help simultaneously?
- **Change and absence.** How are call-offs, vacancies, agency staff, hospital returns, and temporary one-to-one needs covered?

Observe quietly: Are residents waiting for help? Do staff know residents by name? Are call devices reachable? Do dining, medication, toileting, and shift-change periods look organized? One visit is a sample, so return at a different time and compare what you see with the written answer.

Read the discharge rule before the move-in date

Families often ask, “Can she move in?” The safer question is, “What facts could make her move out?” Illinois requires the resident contract to state termination conditions. The SLP discharge rule addresses discussion, written notice, reasons, safe discharge planning, appeal rights, emergency situations, inability to meet needs, contract breaches, and nonpayment. [116]

Ask the community to explain these triggers

- **Care exceeds capacity.** Which mobility, transfer, nursing, behavioral, cognitive, or nighttime needs would the site say it cannot meet—even with available outside services?
- **Hospital or rehabilitation absence.** How long may the apartment be held, what remains payable, and what assessment is required before return?
- **Payment problem.** What happens during a pending Medicaid application, eligibility interruption, resident-liability dispute, or delayed State payment?
- **Campus transfer.** Can the resident move to another unit or level on the campus? Is that a right, a preference, or a new admission subject to availability and payment approval?
- **Notice and appeal.** Who receives notice, how many days apply, where is the hearing request sent, and who helps plan a safe alternative placement?

Do not accept “we age in place” as a discharge policy. Ask for the resident contract, admission criteria, discharge policy, temporary-absence policy, grievance process, resident-rights notice, current fee sheet, and any addendum governing the specific apartment. Compare the documents with the sales conversation and save the version actually signed.

REMEMBER THIS
The best time to understand discharge criteria is while the family still has choices. A written limit is not necessarily a bad limit; a hidden or misunderstood limit is dangerous.

If an involuntary discharge notice arrives, read it immediately. The rule generally calls for 30 days’ written notice unless delay could jeopardize health or safety, and it describes hearing rights and deadlines. Follow the actual notice, preserve the envelope, and seek prompt legal or ombudsman help when housing or care is at risk.[116]

SLP Community Tour and Comparison Sheet

Complete one sheet per community, then copy the scores into the comparison grid. Record who answered each clinical, financial, and contract question.

Community: _____ County: _____ Tour date/time: _____

Contact / role: _____ Follow-up date: _____ Waitlist status: _____

Verify before ranking

- **Program and apartment.** HFS listing checked: ☐ Population matches: ☐ Medicaid opening confirmed: ☐ Waitlist terms saved: ☐
- **Care fit.** Nurse reviewed needs: ☐ Dementia setting verified if needed: ☐ Night needs accepted: ☐ Outside-service gaps listed: ☐
- **Staffing.** Day/evening/night counts obtained: ☐ Shared assignments explained: ☐ Call-response process observed: ☐
- **Money and contract.** Room and board: _____ Liability estimate: _____ Deposits/fees: _____ Discharge and absence policies received: ☐
- **Daily life.** Meals observed: ☐ Residents engaged: ☐ Transportation fits: ☐ Family travel workable: ☐ Apartment accessible: ☐

Comparison factor	A	B	C
Verified Medicaid opening / waitlist	__/5	__/5	__/5
Current and future care fit	__/5	__/5	__/5
Night/weekend staffing confidence	__/5	__/5	__/5
Dementia capability, if needed	__/5	__/5	__/5
Discharge/return rules understood	__/5	__/5	__/5
Monthly resident cost understood	__/5	__/5	__/5
Family access and daily-life fit	__/5	__/5	__/5

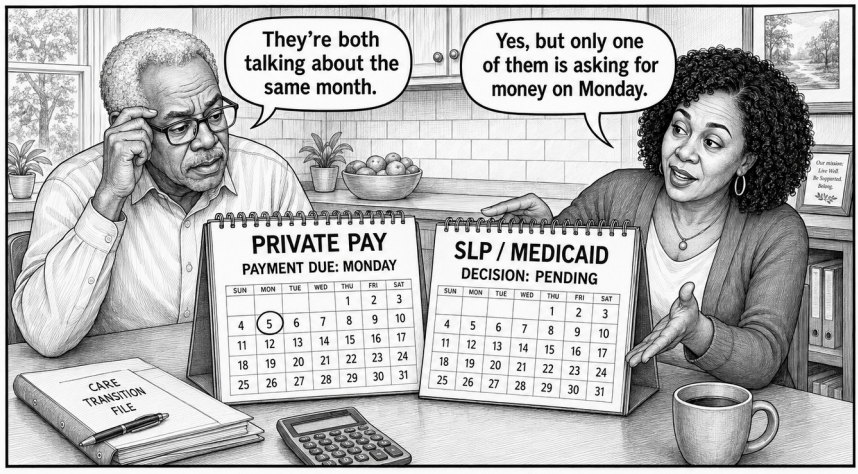
Bottom Line

Use the official list to find candidates, then verify the exact population, Medicaid apartment, waitlist, dementia designation, staff coverage, costs, and discharge terms. Tour more than once, give every site the same needs summary, and keep several options alive. The right SLP community is not simply the nearest or prettiest one. It is the setting that can document a safe, payable, durable fit.

CHAPTER 25

From Private Pay to SLP

Samuel looked from one calendar to the other. “They’re both talking about the same month.” Grace nodded. “Yes, but only one of them is asking for money on Monday.”



The care timeline and the approval timeline did not automatically meet.

— THE TWO CALENDARS —

A move from private payment to Medicaid-supported SLP is not a switch the family flips. It is a coordinated handoff among the Medicaid case, required screening, SLP provider, resident contract, billing office, and Samuel’s own income. Each track can have a different effective date. Until the controlling notices and provider records agree, the family must know who is paying every day and what happens if a decision is delayed.[117]

REMEMBER THIS

Pending is a status, not a payment source. Do not end a safe payment arrangement until the next arrangement’s eligibility, screening, provider acceptance, effective date, and resident obligation are documented.

Run the conversion as five linked tracks

Grace made one timeline rather than waiting for one office to finish before contacting the next. The workflow differs when a person already lives in an SLP as a private-pay resident versus moving from another setting, but the family still must coordinate the same essential decisions.

- **1. Confirm the exact SLP placement.** Verify that the apartment is within the certified program, the site serves Samuel's population, the provider will retain or admit him if Medicaid is approved, and the proposed care remains appropriate.
- **2. File the correct Medicaid application.** Save the complete application, requested coverage month, confirmation, case number, documents, authorized-representative information, and every follow-up request. Do not assume the residence filed what the family intended.
- **3. Complete the required screening.** For a resident who entered SLP as private pay and later converts to Medicaid, HFS instructs the provider to request a conversion screening from the Care Coordination Unit. The screening must be completed before conversion; Medicaid payment cannot start before that screening date.[118]
- **4. Complete the provider's assessment and contract work.** The State's functional decision does not replace the provider's current assessment, service plan, apartment acceptance, payment calculation, or resident contract.
- **5. Reconcile the effective dates.** Compare the application month, financial-eligibility months, screening date, SLP admission or conversion date, Medicaid payment start, private-pay end, and first resident-contribution month.

No mandatory private-pay waiting period

HFS reminded SLP providers in May 2026 that they may not require a potential resident to pay privately for a fixed period before being allowed to apply for Medicaid. A person must be able to apply. If someone is later determined Medicaid-eligible for a period in which excess private payment was collected, HFS states that the provider must refund amounts above the resident responsibility determined by DHS.[119]

TIMING RULE
Do not confuse a Medicaid application's possible eligibility months with the SLP payment start date. A late conversion screening can move the admission/payment start forward even when other eligibility requirements were met earlier.[118]

The rate changes; Samuel's income does not disappear

A certified SLP may charge private-pay residents a different rate from the Medicaid service rate. After Medicaid conversion, Medicaid pays its approved portion of covered SLP services, while the resident remains responsible for room and board and any group care credit—also called patient liability—calculated from countable income and allowed deductions.[120]

Read the monthly calculation in order

- **Countable income.** List Social Security, pensions, VA benefits, retirement distributions, and other income exactly as the Medicaid case treats them. The required minimum income is tied to the current SSI amount so room and board can be paid while the resident retains the personal-needs allowance.
- **Allowed deductions.** The current HFS resident explanation subtracts allowable medical expenses, health-insurance premiums, the personal-needs allowance, and SLP room and board from countable income.
- **Group care credit.** The remaining amount becomes the resident's contribution toward covered care. Medicaid pays the balance of the approved service rate. A resident whose income equals the applicable SSI amount may have no group care credit, but still owes room and board.
- **Resources and changing facts.** Excess countable resources can affect cost of care or eligibility. Report income, premium, resource, marital, address, and care changes through the required channel and preserve proof.

Current-rule reference — verified July 2026

HFS identifies a 2026 monthly SSI amount of $994 for a single person, SLP room and board of $874 for single occupancy and $625.50 per resident for double occupancy, and a $120 personal-needs allowance. These figures are included to explain the structure, not to serve as permanent numbers. Verify the current HFS notice for the actual conversion month.[120]

Samuel's transition estimate

Gross countable monthly income: _____

Less allowable medical expenses / premiums: _____

Less current room and board: _____ Less current PNA: _____

Estimated group care credit: _____ Provider estimate agrees: ☐

Private-Pay-to-SLP Transition Control Sheet

Do not choose a move or conversion date from the most optimistic calendar. Choose it from the last essential approval—and keep a safe alternative until the handoff is complete.

Controlling dates

Medicaid application filed: _____ Requested coverage month: _____

Conversion / DON screening requested: _____ Completed: _____

Provider assessment completed: _____ Apartment accepted: ☐

Financial approval received: _____ Effective month(s): _____

SLP conversion / payment start confirmed in writing: _____

Private-pay end date: _____ Final reconciliation due: _____

Money bridge

Cash available for pending period: _____ Weekly / monthly private-pay cost: _____

Room and board after conversion: _____ Estimated GCC: _____

Deposits / prepaid rent / credits expected back: _____ Timing: _____

Backup plans

- **Plan A — conversion on schedule.** Name the apartment, effective date, resident charge, documents still due, and person confirming each item.
- **Plan B — decision delayed.** Identify how long private pay can safely continue, whether the current apartment is held, the next billing date, and the reserve that must not be spent elsewhere.
- **Plan C — denial, lost placement, or changed needs.** Keep alternate SLP communities, a safe temporary-care option, appeal contacts, and emergency funding or family responsibilities current.
- **Stop-and-review triggers.** Seek qualified help before transferring assets, signing a new private-pay guarantee, relying on retroactive coverage, leaving a safe setting, or accepting a notice that conflicts with the provider's bill.

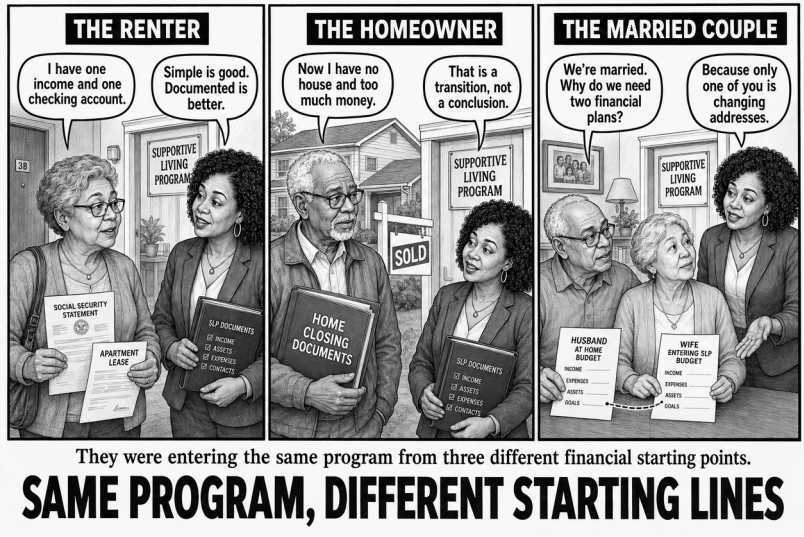
Bottom Line

A safe private-pay-to-SLP transition has no invisible day. The family can identify the care setting, payment source, resident obligation, responsible contact, and backup plan for every date on the calendar. File early enough to solve missing-record problems, complete the conversion screening before payment is expected to begin, keep enough cash for delay, and reconcile the final notices against the provider's ledger.

CHAPTER 26

SLP Case Studies

Three households reached the same program from different financial starting points. Louise rented and lived on modest Social Security. Walter sold the home that had held most of his wealth. Ruth entered SLP while Harold remained in their house. None of them could safely borrow another household's calculation.



These fictional examples use simplified facts and July 2026 reference figures to show the sequence of reasoning. They are not eligibility promises. Actual results depend on the Medicaid category, application and screening dates, ownership, transfers, deductions, spouse protections, provider calculations, and current notices.[121]

Use the same five questions every time

- **Who is applying?** Single person or married person with a spouse outside the setting?
- **What is owned today?** List legal ownership, access, equity, sale proceeds, joint resources, and recent transfers.
- **What income belongs to whom?** Start with gross income, then identify premiums, allowed deductions, and any spouse allowance.
- **Which dates control?** Application, screening, sale, private-pay period, conversion, eligibility, and provider payment start.
- **What could break the plan?** Missing proof, changed needs, lost apartment, delayed approval, unplanned gift, or a bill that does not match the notice.

Case 1 — Louise: one income, one lease, one careful calculation

Louise, seventy-two, rented a small apartment and had no spouse. Her gross Social Security was $1,380 per month. The deposit arriving in checking was lower because $202.90 was withheld for her documented Medicare premium. She had $8,400 in checking and savings, household goods, and no known transfers in the review period.

What the family verified

- **Program fit.** Louise met the age route, completed the required level-of-care screening, and was accepted for an available SLP apartment.
- **Resource position.** Her verified countable resources were below the assumed July 2026 AABD Medical disregard used for this example. The family still supplied every requested statement.[122]
- **Income floor.** Her gross income exceeded the current SSI-linked minimum described by HFS for an SLP resident.

Illustrative monthly calculation

Gross countable Social Security $1,380.00
Less documented health-insurance premium ..... −202.90
Less 2026 single-occupancy room and board ..... −874.00
Less 2026 SLP personal-needs allowance ..... −120.00
Illustrative group care credit $183.10

Louise would pay room and board plus the officially determined group care credit; Medicaid would pay the remaining approved SLP service amount. The provider's estimate was not final until it matched the DHS/HFS calculation and effective month.[121]

Why the simple case could still fail

- Using the bank deposit instead of gross Social Security would hide the premium and distort the calculation.
- Treating an apartment opening as Medicaid approval could create an uncovered private-pay period.
- Ignoring a closed account or unexplained deposit could delay verification even though Louise's current finances looked simple.

CASE LESSON
A short asset list is not a shortcut around documentation. Louise's case worked because every number had a source, every deduction had proof, and the provider's bill was reconciled to the official notice.

Case 2 — Walter: the house became cash

Walter, seventy-nine, entered SLP as a private-pay resident while his vacant house was listed. He had no spouse and did not expect to return. The house sold at arm's length for $260,000. After the mortgage, taxes, repairs agreed at closing, and documented selling expenses, $212,000 was deposited into his checking account.

The classification changed on closing day

Before the sale, the family's home analysis required occupancy, intent, equity, and long-term-care rules. After closing, Walter owned liquid proceeds he could legally access. Available nonexempt cash is a resource; the family could not continue calling it "the house" merely because that was its source.[122]

The safe sequence

- **Prove fair market value.** Keep the listing agreement, market analysis or appraisal, purchase contract, closing disclosure, deed, payoff, repair invoices, and net-proceeds deposit.
- **Report the sale and new account balance.** Update the Medicaid case and provider; do not wait for the next annual review.
- **Build a lawful spending plan.** Walter's funds paid his private care, debts, dental work, hearing equipment, clothing, burial arrangements within current rules, and other fair-market-value purchases for his benefit. Every payment produced a receipt or contract.
- **Protect the look-back record.** No gifts, bargain sales, undocumented family reimbursements, or casual joint-account changes were made while reducing resources.[123]
- **Apply at the right point.** The family coordinated the expected resource month, required screening, available apartment, application evidence, and enough private-pay cash to survive a delay.

Walter's proceeds ledger

Net proceeds received: _____ Date / account: _____

Private care paid: _____ Debts / medical needs: _____

Other lawful purchases: _____ Current verified balance: _____

CASE LESSON

Selling an exempt or potentially exempt asset can change its Medicaid treatment. The sale itself was not the mistake. Failing to plan, report, and document the proceeds would have been.

Case 3 — Ruth and Harold: two addresses, one coordinated plan



Ruth entered SLP while Harold remained in their home. Ruth received $2,000 gross monthly income; Harold received $1,200. Their combined countable resources and Harold's housing costs required the spousal-impoverishment analysis described in Chapters 16–18—not the single-person shortcut.[124]

Two connected calculations

- **Resources.** DHS first determined combined nonexempt resources and the amount Ruth could retain or transfer under the applicable community-spouse resource rules. The house remained separately analyzed because Harold occupied it.
- **Income.** Income was attributed under the spouse rules. The official calculation allowed Ruth to contribute $650 monthly to Harold in this fictional example, based on his income and recognized needs. The allowance counted only if it was actually transferred.

Illustrative Ruth calculation

Ruth's gross income $2,000 − premium $200 − room/board $874 − PNA $120 − spouse allowance $650 = illustrative GCC $156.

Omitting the spouse allowance would raise Ruth's apparent contribution by $650 and could break Harold's home budget. Simply dividing every account in half could also be wrong. Ownership, the snapshot/resource assessment, CSRA, income attribution, actual transfers, and current annual standards control.[124]

Bottom Line

Louise needed exact proof. Walter needed a proceeds plan. Ruth and Harold needed two calculations joined by one budget. SLP eligibility follows the facts, dates, and documents beneath the household label.

PART VII

Create the Family Plan

CHAPTERS 27–29

Turn the research into dated actions, assigned roles, documented decisions, and safeguards that preserve the older adult's voice.

CHAPTER 27

Your 30-Day Action Plan

Thirty days is enough time to create order, but not enough time to treat every task as equally urgent. Elena's relatives produced four good ideas in one minute. Her job was to identify which action protected Maria tonight, which preserved money and rights, and which could safely wait.



Urgency required a sequence—not simultaneous panic.

THIRTY DAYS, THIRTY OPINIONS

The plan below begins when a move is expected within thirty days. It does not assume that Medicaid, VA Pension, insurance, a home sale, or an SLP apartment will be approved or available in that period. Possible funding remains possible until an official decision, payment, or binding coverage determination says otherwise.[125]

Use three lanes—not one giant list

- **Safety lane.** What must happen today so the person is not discharged to an unsafe setting or left without medication, supervision, food, mobility help, or emergency response?
- **Placement lane.** Which settings can meet the assessed need, have an actual opening, disclose the real price, and accept the intended payment source?
- **Payment lane.** What pays the deposit and first months while benefits, insurance, property, or family arrangements are verified?

> **ACT NOW**
> Write the expected move date at the top of every working page. A task without an owner and deadline is still only an opinion.

Days 1–3: stabilize the person and freeze avoidable mistakes

The first seventy-two hours are for facts that affect immediate safety and irreversible decisions. Elena did not begin by transferring money or listing Maria's property. She began with the care team, the discharge date, and a written description of what Maria could and could not safely do.

1. Name the care need

- **Obtain the current assessment.** Record medication help, transfers, bathing, toileting, cognition, nighttime needs, behavior, mobility, diet, equipment, and emergency risks.
- **Ask what changes the recommendation.** A setting appropriate for scheduled assistance may not be appropriate for wandering, two-person transfers, complex nursing, or unpredictable behavior.
- **Identify the decision-maker.** Maria speaks for herself while she has capacity for the decision. A diagnosis, age, or family disagreement does not automatically replace her voice.

2. Put a temporary hold on irreversible transactions

- **Do not give away or retitle property.** Do not add names to deeds or accounts, make large gifts, sell below fair market value, or sign a family caregiver contract to solve a deadline.
- **Do not assume the house must be sold.** First identify ownership, occupancy, spouse protections, equity, likely net proceeds, tax questions, and the payment runway.
- **Do not count a pending benefit as cash.** Build the first-month plan from money that is legally accessible and documented now.

3. Open the control file

- **Identity and authority.** Photo identification, Social Security and Medicare information, insurance cards, powers of attorney, guardianship orders if any, releases, and emergency contacts.
- **Care and medication.** Current medication list, diagnoses, physician contacts, recent assessments, therapy notes, equipment, allergies, and discharge instructions.
- **Money and deadlines.** Gross income, accessible cash, recurring bills, bank statements, contract deadlines, quoted deposits, expected move date, and who receives official mail.

DO NOT DO THIS YET

Do not let a thirty-day deadline become the explanation for a transfer, joint account, deed change, or product purchase that the family cannot later document and defend.

Days 4–14: compare real openings and start the right processes

The second phase turns the safety description into a short list of settings. A directory listing is not an opening, Medicaid certification is not current availability, and an attractive apartment is not proof of clinical fit.

Run five calls in parallel

1. **Care team.** Confirm the current level of care, discharge assumptions, equipment, prescriptions, transportation, and who will send records.
2. **Communities.** Ask about an actual opening, assessed-care acceptance, total first-month price, ongoing charges, deposit and refund terms, discharge criteria, and the exact payment programs accepted.
3. **Benefits and insurance.** Request policy claim instructions, screen the correct Medicaid or SLP pathway, check possible VA status with an accredited representative, and preserve every tracking number.[126]
4. **Financial and legal advisers.** Escalate home, transfer, spouse, tax, contract, capacity, and authority questions before acting—not after the bank statement changes.
5. **Family.** Assign visits, calls, documents, transportation, packing, payment tracking, and daily communication. One coordinator maintains the master log.

Build the thirty-day private-pay bridge

Available cash: _____ Monthly gross income: _____
First-month residence charge: _____ Care add-ons: _____
Moving / equipment / medication: _____ Home carrying cost: _____
Documented family contribution: _____ Unfunded amount: _____

A bridge must cover the gap between the care start date and the date a dependable payment source actually begins. If the family cannot fund the likely delay, that is not a later problem; it changes today's placement choice.

Read before signing

- **Obtain the entire agreement.** Review services, add-on charges, rate changes, deposits, refunds, absence rules, discharge terms, arbitration, responsible-party language, and the effect of a delayed benefit decision.
- **Keep promises in writing.** Record who said what, on what date, and whether the statement appears in the contract or official program notice.

Days 15–30: close the gaps, prepare the move, and preserve a backup

By the third phase, the family should know the preferred setting, the known first-month cost, the unresolved approvals, and the date on which the current plan runs out of money or safety. The remaining work is controlled execution.

The 30-Day Control Sheet

Expected move date: _____

Current location and safe-through date: _____

Chosen setting / backup setting: _____

Assessment owner / completion date: _____

Contract reviewer / deadline: _____

First-month payment source: _____

Pending benefit or insurance claim / tracking number: _____

Medication, equipment, and transportation owner: _____

Home, lease, mail, utilities, and valuables owner: _____

Family update time and meeting leader: _____

Final forty-eight-hour check

- **Care.** Medication list reconciled; prescriptions and equipment available; transportation confirmed; receiving staff have the care information.
- **Money.** Amount due matches the signed agreement; payment source is accessible; receipts and account authority are clear; no benefit is treated as approved prematurely.
- **Documents.** Identification, insurance, powers of attorney, clinical records, contact list, signed contract, inventory, and submission log travel with the correct person—not in the moving truck.
- **Backup.** The family knows what happens if the opening, assessment, transport, payment, or discharge date changes.

> **REMEMBER THIS**
> A thirty-day plan protects safety, money, rights, and options while longer processes continue.

Bottom Line

Start with safety; connect the setting, price, payment, and dates; assign every task; and stop before irreversible transactions. Urgency requires sequence.

CHAPTER 28

Your One-to-Five-Year Plan

A long planning horizon is not permission to postpone. It is the family's chance to make choices while Maria can explain what she wants, sign documents she understands, compare housing without a discharge clock, and correct records before anyone needs them.



The plan is not one binder that remains correct forever. People move, diagnoses change, accounts close, agents become unavailable, policies lapse, beneficiaries change, and annual program figures expire. The durable system is a complete starting file plus a scheduled review.[127]

Use three planning horizons

- **Within one year.** Create authority, document care preferences, inventory finances and insurance, screen benefits, and correct missing or contradictory records.
- **Within one to three years.** Test housing and care options, strengthen the private-pay runway, review property and insurance decisions, and rehearse how help would begin.
- **Within three to five years.** Update the plan annually and after every major health, family, housing, legal, or financial change.

REMEMBER THIS
Planning early does not surrender independence. It gives the older adult more ways to define how help will be provided and who may provide it.

Build the documents before anyone needs emergency authority

Maria's drawer contained good documents from different decades. The problem was not that she had failed to plan. The problem was that no one knew which version controlled, whether the named agents were still willing, or whether the bank and physicians had usable copies.

The legal and authority review

- **Property power of attorney.** Ask Illinois counsel whether the form, powers, successor agents, effective terms, gifting authority, real-estate authority, and recordkeeping expectations match the current plan. An agent acts for the principal; the agent does not become the owner.[128]
- **Health care power of attorney and advance directives.** Name the decision-maker, alternates, care values, communication expectations, and where copies will be kept. Provide current copies to the appropriate clinicians.[129]
- **Capacity is decision-specific.** Do not treat age or a diagnosis as automatic incapacity. Seek qualified medical and legal help when the person's ability to understand a particular decision is genuinely in question.[130]
- **Other authority is separate.** Social Security representative payee, VA fiduciary or representative, trust authority, guardianship, and account access are not interchangeable with a general power of attorney.

The annual document inventory

- **Identity and family.** Identification, marriage and divorce records, military records, citizenship or immigration documents if relevant, and current family contacts.
- **Income and property.** Social Security, pension, retirement and investment accounts, bank accounts, deeds, vehicle titles, debts, taxes, digital assets, recurring payments, and closed-account history.
- **Insurance.** Medicare, supplement or Advantage coverage, prescription coverage, long-term-care insurance, life insurance, annuities, homeowner or renter coverage, auto, and umbrella liability.
- **Estate and beneficiary designations.** Will, trust if any, transfer-on-death or payable-on-death designations, retirement and life-insurance beneficiaries, burial arrangements, and professional contacts.

Document control record

Document: _____ Signed / issued: _____ Stored: _____ Agent or owner: _____ Copy provided to: _____ Reviewed by: _____ Next review: _____ Replaced version destroyed or marked obsolete: _____

Turn care preferences and housing choices into usable instructions

"I want to stay home" is a value, not yet a plan. It becomes actionable when the family knows what staying home means, what risks are acceptable, what help Maria will accept, what the home can support, and what event triggers another setting.

The care-preference conversation

- **Daily life.** Preferred schedule, food, faith, language, privacy, pets, social life, transportation, bathing, medication help, visitors, technology, and routines that protect identity.
- **Risk and supervision.** Falls, wandering, driving, cooking, medication, firearms, scams, emergency response, and who may be contacted when concerns arise.
- **Care setting.** Home modifications, in-home help, adult day services, independent living, assisted living, memory care, SLP, and nursing-facility preferences—including unacceptable conditions.
- **Decision triggers.** Define events that require reassessment: repeated falls, unsafe nighttime behavior, missed medication, caregiver exhaustion, hospitalization, two-person transfers, or a home budget that no longer works.

The housing review

- **Home condition.** Entrance, stairs, bathroom, bedroom, lighting, maintenance, taxes, insurance, utilities, neighborhood access, and realistic repair costs.
- **Ownership and family structure.** Deed, mortgage, joint owners, spouse occupancy, prior transfers, estate plan, accessibility, and who is legally responsible for expenses.
- **Four possible jobs.** Remain a home, be sold, produce rent, or support borrowing. Compare each job by net cash, risk, management burden, spouse impact, taxes, and benefit consequences.

Screen benefits and insurance before the crisis

- **Run a broad screen.** Medicaid pathways, Medicare cost assistance, VA Pension and compensation, long-term-care insurance, life-insurance provisions, tax reductions, transportation, nutrition, caregiver support, and local aging services.
- **Use funding labels.** Available now, approved for a dated future period, or merely possible. Do not build the dependable budget from the third column.
- **Review contracts, not product names.** Benefits, exclusions, elimination periods, covered settings, premiums, cash value, loans, beneficiary effects, and claim procedures belong in the annual review.

Schedule the annual financial checkup

Choose one month each year. Maria chose the month after tax filing, when statements were available and no holiday competed for attention. Elena maintained the checklist; Maria remained the decision-maker; Mark verified that the backup plan still worked from a distance.

Annual Plan Review

- **People.** Addresses, telephone numbers, agents, successor agents, caregivers, advisers, physicians, emergency contacts, and who should receive information.
- **Health and function.** Diagnoses, medications, mobility, cognition, daily activities, driving, emergency events, caregiver strain, and current assessment needs.
- **Income and expenses.** Gross income, deductions, taxes, insurance premiums, housing cost, debt, care cost, subscriptions, automatic payments, and the private-pay gap.
- **Assets and ownership.** Balances, titles, deeds, equity, joint ownership, closed accounts, digital property, valuables, insurance values, burial arrangements, and recent transfers.
- **Benefits and policies.** Eligibility screens, annual limits, claim status, premium changes, coverage, beneficiaries, and contact information.
- **Housing and care.** Home safety, repairs, likely care tiers, preferred communities, geographic alternatives, availability, transportation, and what would trigger a move.
- **Documents and access.** Current signed versions, storage, copies, passwords and digital-access plan, release forms, agent acceptance, and obsolete versions.

One-to-Five-Year Planning Record

This year's three actions:
1. _____
2. _____
3. _____

One-to-three-year decision to prepare: _____
Three-to-five-year risk to monitor: _____
Next annual review date: _____ Meeting owner: _____

Bottom Line

Create the file while Maria can direct it. Review authority, insurance, benefits, housing, care preferences, and finances yearly. Maintenance—not binder thickness—keeps a plan useful.

CHAPTER 29

The Family Meeting

A family meeting is not a vote to take over someone's life. It is a method for hearing the older adult, defining authority, assigning work, recording decisions, and exposing conflicts before they become secret transactions or emergency accusations.



Maria's answer established the first rule: help should expand her ability to decide and participate for as long as possible. Authority changes only when a valid document, program rule, clinical determination, or court order says it changes—and only for the decisions within that authority. [128]

Invite roles, not factions

- **Decision-maker.** The older adult, or the legally authorized decision-maker for the issue when authority is active.
- **Facilitator.** Keeps the agenda moving, states disagreements neutrally, and prevents one voice from controlling the meeting.
- **Recorder.** Documents facts, decisions, owners, deadlines, unresolved questions, objections, and the location of supporting records.
- **Subject-matter participants.** Care professionals, benefits representatives, accountant, insurance professional, care manager, or attorney participate only where their expertise is relevant.

GROUND RULE
No surprise documents, no unexplained transfers, no private side agreements, and no decision attributed to Maria unless Maria actually made or authorized it.

Separate capacity, authority, and family influence

Families often collapse three questions into one: Can Maria decide? Who may act? What does the family prefer? Those are different questions, and a loud family preference does not create legal authority.

Capacity belongs to the decision

- **Begin with a presumption of participation.** Speak directly to Maria, provide hearing or language support, choose a good time of day, explain one choice at a time, and allow questions.
- **Do not diagnose at the table.** Memory loss, disagreement, disability, age, or an unwise choice does not by itself establish incapacity. Obtain qualified assessment for the particular decision when needed.[130]
- **Record support—not only conclusions.** Note what information was provided, how Maria expressed her choice, what assistance she used, and whether a professional evaluation exists.

Authority belongs to the document or law

- **Property agent.** Read the power, limitations, effective terms, successor provisions, and duties. Illinois agents who act must act for the principal's benefit and keep records of receipts, disbursements, and significant actions.[128]
- **Health care agent.** Confirm the current form, its activation terms, the treatment question, and the older adult's expressed wishes. Give the provider the correct copy.
- **Program-specific roles.** Representative payee, VA fiduciary or accredited representative, trustee, guardian, authorized representative, and responsible party have different sources and limits of authority.

Use a decision statement

Decision: _____
Decision-maker and source of authority: _____
Maria's stated preference: _____
Information reviewed: _____
Professional question still open: _____

REMEMBER THIS
A power of attorney is not a crown, a joint account is not a power of attorney, and family closeness is not authority. Name the role before anyone acts.

Make transparency ordinary—and recognize when conflict becomes danger

Transparency does not require publishing every private detail to every relative. It requires the right records, available to the person and authorized reviewers, with enough independent oversight to deter mistakes, conflicts, and exploitation.

Four duties for anyone managing money

1. **Act for the person's benefit.** Follow known expectations and lawful instructions; do not turn access into personal entitlement.
2. **Manage carefully.** Pay obligations on time, protect property, verify prices, and use qualified help when a decision exceeds the agent's skill.
3. **Keep money separate.** Avoid commingling, informal loans, cash without receipts, and accounts whose ownership contradicts the plan.
4. **Keep complete records.** Maintain statements, receipts, contracts, invoices, approvals, explanations, and a transaction ledger.[131]

Warning signs that require attention

- **Money or mail changes.** Missing funds, unusual withdrawals, unpaid ordinary bills, changed beneficiaries, new joint owners, unexplained gifts, missing statements, or sudden property transfers.
- **Control and isolation.** Someone prevents private conversations, answers every question, withholds the telephone or mail, frightens the person, or insists on secrecy.
- **Documents do not match the story.** Signatures, dates, receipts, contracts, account titles, or explanations conflict—or the person cannot explain a transaction supposedly made for her benefit.
- **Pressure replaces consent.** Urgency, threats, guilt, romance, caregiving dependence, or promises of eligibility are used to obtain money, signatures, or authority.[132]

Respond in proportion to the risk

- **Immediate danger:** call 911.
- **Suspected abuse, neglect, exploitation, or self-neglect in the community:** contact Illinois Adult Protective Services at 1-866-800-1409; anonymous reports are accepted.[132]
- **Long-term-care concern:** contact the regional Long-Term Care Ombudsman or appropriate regulator; the Ombudsman is resident-directed and is not an emergency responder.[133]
- **Possible theft or fraud:** preserve records, notify the relevant financial institution, and seek legal and law-enforcement guidance without confronting a suspected exploiter in a way that increases danger.

Use an agenda, a decision log, and a closing check

A predictable structure lowers conflict because everyone knows when concerns will be heard and how a decision becomes final. Elena circulated the facts before the meeting; Mark submitted questions; Maria chose which private details could be shared.

Family Meeting Agenda

1. **Maria's priorities.** What outcome matters most to her today? What information may be shared, and with whom?
2. **Changed facts.** Care, safety, housing, income, assets, insurance, benefits, contracts, deadlines, and family availability.
3. **Decisions.** Name the decision-maker, authority, options, risks, professional advice, and deadline for each item.
4. **Assignments.** One owner, one deliverable, one deadline, one storage location, and one backup for every task.
5. **Conflicts and safeguards.** Disclose personal interests, family payments, gifts, reimbursements, account access, and any objection that belongs in the record.
6. **Closing confirmation.** Read back decisions, unresolved questions, next meeting date, and who will distribute the record.

Decision and Assignment Log

Meeting date: _____ Facilitator: _____ Recorder: _____
Decision / task: _____
Decision-maker / authority: _____
Owner: _____ Deadline: _____ Evidence location: _____
Maria's preference or objection: _____
Conflict disclosed: _____
Follow-up result / date: _____

Before the meeting closes

- **Confirm that Maria was heard directly and privately where appropriate.**
- **Confirm that no participant is relying on an authority the person does not hold.**
- **Confirm that payments, reimbursements, gifts, and conflicts are documented.**
- **Confirm that suspected exploitation or immediate safety concerns were not deferred to the next meeting.**

Bottom Line

Keep Maria at the center, define roles, record decisions, and make transparency routine. Suspected exploitation or danger requires the proper protective channel—not another family discussion.

CONCLUSION

Monday Came

Monday arrived exactly as Elena feared it would. The bank did not stay open late. Medicaid did not become one simple program. The insurance policy did not summarize itself. Mark did not stop asking questions from two thousand miles away. Maria did not become less opinionated because the family had made a spreadsheet.

But the family was no longer solving one enormous problem called assisted living. They had separated care from setting, price from payment, current cash from possible benefits, ownership from family assumptions, and immediate safety from long-term planning.

Elena could now say what she knew:

- **Maria's current needs had been assessed and written down.**
- **The chosen setting had been asked to explain its services, prices, and discharge rules.**
- **The family knew how many months the dependable money could cover.**
- **Every pending benefit had a status, a date, an owner, and a backup plan.**
- **The house, accounts, policies, and family contributions had been analyzed by their real legal and financial characteristics—not their nicknames.**
- **Maria's preferences and decision-making role appeared in the plan, not merely in the family's memories.**

That did not guarantee a perfect outcome. It created something more useful: a plan the family could explain, update, and defend.

THE BOOK'S CENTRAL RULE
Define the care, verify the setting, calculate the real price, identify the lawful payment sources, and connect every decision to the dates and documents that control it.

A plan is a practice

Families rarely finish long-term-care planning. They reach a stable point, then revise when health, housing, income, ownership, programs, or people change. The binder is not the plan. The practice is the plan.

The practice looks ordinary:

- **One person opens every notice on the day it arrives.**
- **One log records every submission and telephone call.**
- **Every large transaction has a purpose, authority, fair-value explanation, and receipt.**
- **Every annual figure is labeled with the date it was verified.**
- **Every family helper knows the limits of the role.**
- **Every change in care triggers a new setting and cost review.**
- **Every meeting returns to the older adult's goals and rights.**

On the next Monday, Maria sat at the kitchen table while Elena replaced an expired insurance summary in the family file. Mark joined by video. Maria listened to the agenda, corrected two facts, rejected one apartment, and approved the next three calls.

"Who is in charge?" Mark asked, smiling because he knew the answer.

Maria looked over her glasses.

"You are all in charge of helping me remain in charge."

That sentence did not solve Medicaid, VA Pension, SLP, the house, or the next care crisis. It told the family how to solve them without losing Maria inside the process.

FINAL REMEMBER THIS

The goal is not to make every decision today. The goal is to make the next decision safely, preserve the evidence, and leave the family with more lawful options tomorrow.

APPENDIX A

The Master Family Action File

Use this appendix as the cover sheet for the working file. It is an index, not a substitute for the documents themselves. Record where each item is stored and who may access it.

1. Person, authority, and communication

Preferred name / pronouns / language: _____

Primary address / mailing address: _____

Primary decision-maker for current issue: _____

Property agent / successor: _____

Health care agent / successor: _____

Representative payee, VA fiduciary, trustee, guardian, or other role: _____

Attorney / accountant / insurance professional / care manager: _____

Primary family coordinator / backup: _____

Who may receive information: _____

Where signed authority documents are stored: _____

2. Immediate safety and care

Current setting and safe-through date: _____

Primary physician / pharmacy: _____

Medication list location: _____

Current assessment / level of care: _____

Mobility, transfer, bathing, toileting, cognition, nighttime, diet, and behavior needs: _____

Emergency plan / equipment / transportation: _____

FILE RULE
Mark obsolete documents clearly. Keep a dated index so a helper can identify the current version without guessing.

The financial and property index

Record gross amounts, legal ownership, access, and the date verified. Do not use this page to store passwords or full account numbers.

Income

Source / gross / deduction / net / owner / frequency / proof location

_____
_____
_____
_____
_____

Accounts and investments

Institution / type / owner / balance date / access / statement location

_____
_____
_____
_____
_____

Real estate and vehicles

Property / title / occupancy / debt / equity / expense / evidence

_____
_____
_____
_____

Insurance, annuities, burial, and other property

Contract / owner / beneficiary / value / premium / claim status / location

_____
_____
_____
_____

CONTROL TOTAL
Reconcile this index to current statements, deeds, titles, policies, tax records, and the transfer history before relying on the total.

The care, housing, and payment plan

Care and setting

Current assessed needs / date: _____

Preferred setting / backup setting: _____

Three nonnegotiable care or quality requirements: _____

Trigger for reassessment or move: _____

Chosen community / actual availability contact: _____

Discharge criteria reviewed / location: _____

Price and runway

Base monthly price / care add-ons / recurring extras: _____

First-month charges / home carrying costs / moving costs: _____

Dependable monthly income: _____

Documented monthly gap: _____

Accessible private-pay funds: _____

Conservative runway / recalculation date: _____

Benefits and claims

Program / claimant / pathway / status / filing date / tracking / next deadline

_____

_____

_____

_____

_____

FUNDING LABELS
Use only three: available now, approved for a stated period, or possible. Do not move a possible benefit into the dependable budget.

The annual review and professional-help triggers

Annual review date: _____ Leader: _____ Next review: _____

Review after any change in:

- **Health, cognition, function, medication, hospitalization, or caregiver capacity.**
- **Address, marital status, household, agent, beneficiary, ownership, account access, or professional adviser.**
- **Income, taxes, debt, insurance, property value, care price, home expense, or benefit status.**
- **Law, annual program limits, forms, agency procedures, provider participation, or contract terms.**

Stop and obtain individualized help before:

- **Gifting, transferring, retitling, selling below market, adding a joint owner, changing a deed, or returning transferred property.**
- **Buying or surrendering an annuity or insurance policy, borrowing against the home, creating or funding a trust, or changing major beneficiaries.**
- **Signing a family caregiver agreement, paying a relative, making a large reimbursement, or using cash without a complete record.**
- **Relying on spouse protections, hardship, transfer exceptions, estate-recovery protections, or a claim that property is exempt.**
- **Signing an admission contract, responsible-party clause, arbitration agreement, or discharge settlement that is not understood.**
- **Acting when capacity, authority, conflict of interest, exploitation, or immediate safety is disputed.**

Annual review decisions

Changed fact: _____
Decision / reason: _____
Owner / deadline: _____
Professional consulted: _____
Evidence stored: _____

CLOSING CHECK
The file is ready when a trustworthy substitute can locate the current documents, explain the current plan, identify pending deadlines, and account for every material transaction.

APPENDIX B

Official Resources

Websites, forms, telephone numbers, and agency procedures change. The entries below were reviewed in July 2026. Verify them before relying on them, and keep a dated printout or screenshot of any rule or form used for a major decision.

Illinois aging, care coordination, and protection

- **Illinois Department on Aging Senior HelpLine — 1-800-252-8966; Illinois Relay 711.** Information, local care coordination, legal-service referrals, transportation, nutrition, caregiver resources, and Long-Term Care Ombudsman contacts.
- **Illinois Adult Protective Services — 1-866-800-1409, twenty-four hours.** Reports of suspected abuse, neglect, exploitation, or self-neglect involving adults age sixty or older and adults age eighteen through fifty-nine with disabilities living in the community. Call 911 for immediate danger.[132]
- **Illinois Long-Term Care Ombudsman Program.** Resident-directed advocacy, rights information, and complaint assistance for current, prospective, and former long-term-care residents. Obtain the regional contact through the Senior HelpLine.[133]
- **Illinois Department of Public Health Central Complaint Registry — 1-800-252-4343.** Complaints involving nursing facilities and other IDPH-regulated providers; verify the current online complaint route.
- **HFS Supportive Living complaint and benefits information — 1-800-226-0768.** Confirm the current complaint, resident-rights, and program contact route for SLP.

Illinois Medicaid and Supportive Living

- **Illinois Application for Benefits Eligibility — ABE.illinois.gov.** Online applications for Medicaid and certain other benefits. Save the tracking number and complete submission record.[126]
- **Illinois DHS Help Line — 1-800-843-6154.** Application help, local Family Community Resource Center location, and case questions.
- **Illinois HFS — hfs.illinois.gov.** Medicaid guidance, long-term-care information, Supportive Living Program materials, provider lists, resident fact sheets, notices, and forms.

ROUTE BEFORE CALLING
Write the person's county, age, disability or care route, current setting, program name, application status, case number, and exact question. This reduces the "other agency" loop.

Federal benefits, insurance, and consumer protection

Medicare and Social Security

- **Medicare — 1-800-MEDICARE (1-800-633-4227); Medicare.gov.** Coverage, plan, claim, discharge-planning, and advance-care-planning information. Medicare generally does not cover most long-term custodial care.[125][134]
- **Social Security Administration — 1-800-772-1213; SSA.gov.** Retirement, survivor and disability benefits, benefit verification, representative-payee information, and account services.

Veterans benefits

- **U.S. Department of Veterans Affairs — 1-800-827-1000; VA.gov.** Pension, compensation, forms, claim status, and accredited-help information.
- **VA accreditation search — VA.gov/ogc/apps/accreditation.** Verify whether a VSO representative, claims agent, or attorney is VA-accredited. VA-recognized VSOs provide representation on claims without charge.[135]

National aging and consumer resources

- **Eldercare Locator — 1-800-677-1116; eldercare.acl.gov.** Connection to local aging agencies, services, and Adult Protective Services resources nationwide.
- **Consumer Financial Protection Bureau — consumerfinance.gov.** Managing Someone Else's Money guides, fiduciary duties, trusted-contact information, and financial-exploitation resources.[131]
- **Federal Trade Commission — ReportFraud.ftc.gov; 1-877-FTC-HELP.** Consumer fraud and scam reporting; use IdentityTheft.gov for identity-theft recovery planning.
- **IRS — IRS.gov.** Official tax publications and current rules for home sales, retirement distributions, caregiver employment, medical expenses, and other tax questions. Use a qualified tax professional for individual advice.

VA SAFETY
Use the VA accreditation search before sharing records or discussing a fee. Separate benefits assistance from any proposed annuity, trust, insurance, or investment sale.

Professional and facility questions

This book cannot choose a professional or residence. It can help the family ask questions that reveal credentials, scope, conflicts, and accountability.

Questions for an elder-law attorney

- **Do you regularly handle the exact Illinois Medicaid, SLP, spouse, transfer, estate-recovery, capacity, or admission issue involved?**
- **Who is the client, what does the engagement cover, what facts or documents are missing, and what decisions should stop until review is complete?**
- **Will advice be provided in writing, and how are fees, conflicts, and communication with family members handled?**

Questions for financial, tax, and insurance professionals

- **What license or credential applies, how are you paid, what product or referral compensation exists, and are you acting as a fiduciary for this recommendation?**
- **What tax, liquidity, surrender, beneficiary, Medicaid, VA, spouse, and estate consequences require separate qualified review?**
- **Can the recommendation be explained without assuming benefit approval or a future market return?**

Questions for a community or care provider

- **What needs can you safely serve today, what staffing and response exist by shift, and what change could cause transfer or discharge?**
- **What is the complete first-month and recurring price, which services are included, how can rates change, and what happens during absence or delayed benefit approval?**
- **Which public programs does this specific site currently participate in, is an appropriate apartment actually available, and who confirms financial eligibility?**
- **What complaint, grievance, ombudsman, regulator, appeal, and emergency routes apply?**

CREDENTIAL RULE
A familiar title is not verification. Confirm the person, license or accreditation, scope of practice, compensation, conflicts, and responsibility for the written advice.

PLAIN-ENGLISH GLOSSARY

A–C

These definitions explain how terms are used in this book. A program, contract, statute, or policy may supply a more technical definition for a particular decision.

Activities of daily living (ADLs). Basic personal functions such as bathing, dressing, eating, toileting, transferring, and mobility. Programs and policies may define or group them differently.

Agent. A person authorized to act for another under a power of attorney. The agent has duties and limited authority; the agent does not become the principal or owner.

Aid and Attendance. A higher payment level within VA Pension for a qualifying claimant who meets specified care-need criteria. It is not a freestanding benefit.

Annuity. A contract that may convert money into payments. Ownership, access, transfer, tax, beneficiary, Medicaid, and VA consequences require contract-specific review.

Applicant. The person seeking eligibility or benefits. A family member helping with forms is not automatically the applicant or owner of the applicant's property.

Asset / resource. Money or property considered under a program's financial rules. The exact definition, exclusions, valuation date, and accessibility test depend on the program.

Assisted living. A residential setting offering housing and supportive services. The name alone does not establish staffing, licensure, dementia capability, Medicaid payment, or suitability.

Capacity. The ability to understand and make a particular decision. Capacity can be decision-specific and may require professional evaluation; age or diagnosis alone is not the answer.

Community spouse. Under spousal-impoverishment rules, the spouse who remains outside the institution or covered long-term-care setting.

Community Spouse Resource Allowance (CSRA). The amount of qualifying resources protected for the community spouse under the applicable spousal rules and date-specific standards.

Compensation. In this book's VA chapters, a benefit tied to service-connected disability. It is different from needs-based VA Pension.

Countable. Included in a particular program's calculation. A countable item for one program may be treated differently by another.

D–G

Deduction. An allowed amount subtracted in a program calculation when the required conditions and proof are met. An expense is not automatically a deduction.

Dementia care setting. An SLP or other residential setting designed or certified to serve a specified population with dementia. Verify the site's actual certification, services, staffing, and availability.

Discharge criteria. Contractual, clinical, regulatory, or program conditions under which a residence may require transfer or end residency. Read them before admission.

Elimination period. A long-term-care insurance waiting period that must be satisfied under the policy before covered benefits begin.

Equity. Fair market value minus legally enforceable debt secured by the property. Program rules may impose additional valuation or home-equity requirements.

Estate recovery. A process through which a state seeks recovery from a Medicaid recipient's estate for specified benefits after death, subject to program scope, timing, survivor protections, and hardship rules.

Exempt / noncountable. Excluded from a particular eligibility calculation under current rules. It does not necessarily mean protected from sale, liens, estate recovery, tax, contracts, or later changes.

Fair market value. The price or compensation reasonably obtainable in an arm's-length transaction. Documentation matters when property is sold, transferred, or services are purchased.

Financial eligibility. The income, resource, transfer, and related financial tests for a program. It is separate from medical, functional, provider, or placement eligibility.

Five-year look-back. The sixty-month Medicaid review period used for certain transfers connected to long-term-care coverage. It is a records review, not a blanket prohibition on spending.

Gross income. Income before taxes, premiums, garnishments, or other deductions. Many benefit calculations begin with gross rather than the bank deposit.

Group care credit (GCC). In Illinois SLP, the resident's calculated contribution toward covered supportive services after applicable income deductions. The official calculation and effective month control.

H–L

Health care agent. The person authorized under a health care power of attorney to make health decisions within the document's terms when the authority applies.

HECM. A Home Equity Conversion Mortgage, the federally insured reverse-mortgage program. Counseling, occupancy, property charges, loan terms, and repayment events matter.

Home and Community-Based Services (HCBS) waiver. A Medicaid pathway providing specified services in community settings for an eligible population. Waiver coverage, eligibility, providers, and availability are program-specific.

Housebound. A VA Pension payment level with statutory criteria. It is not simply a description of someone who prefers to remain home.

Institutionalized spouse. Under spousal-impoverishment rules, the spouse receiving qualifying institutional or covered long-term-care services.

Instrumental activities of daily living (IADLs). More complex tasks such as medication management, shopping, transportation, finances, meal preparation, and housekeeping.

Intent to file. A VA process that may preserve a potential effective date if the correct complete claim follows within the required period. It is not an award or completed claim.

Joint account. An account with more than one named owner or access holder. Legal ownership, contribution, withdrawal rights, survivorship, program presumptions, and documentation require careful review.

Level of care. A medical or functional threshold used by a program or provider. Meeting it does not by itself establish financial eligibility or an available placement.

Lien. A legal claim or encumbrance against property. A lien, estate-recovery claim, eligibility decision, and notice are not the same thing.

Long-term care. Ongoing help with personal, cognitive, supervisory, or health-related needs across home, community, residential, or nursing settings. Medicare generally does not cover most custodial long-term care.

Look-back. A retrospective review of transactions. Medicaid and VA use different review periods, definitions, consequences, and exceptions.

M–P

Maintenance needs allowance. An income allowance that may be available to a qualifying community spouse under date-specific spousal rules, including recognized housing-cost components.

Medicaid. A joint federal-state program with multiple eligibility pathways and covered services. “Medicaid” alone does not identify the pathway, setting, agency, or rule needed.

Medical eligibility. The health, disability, functional, or level-of-care test for a program. It is separate from the financial test.

Net proceeds. Money remaining after sale price is reduced by documented debt, taxes, commissions, repairs, closing costs, and other legitimate sale expenses.

Net worth. For VA Pension, a defined calculation involving assets and annual income under current rules and date labels. It is not interchangeable with Medicaid resources.

Notice. An official written agency or provider communication stating a request, decision, effective date, reason, deadline, or appeal right. Read the entire notice on the day received.

Patient liability. An amount a Medicaid recipient must contribute toward long-term-care costs after applicable post-eligibility deductions. Terminology varies by setting and program.

Penalty period. A period of ineligibility for specified long-term-care payment resulting from an uncompensated transfer under the applicable rules and calculation.

Personal-needs allowance (PNA). A protected monthly amount retained by a long-term-care resident for personal use under the applicable program and year.

Power of attorney. A legal instrument authorizing an agent to act within stated powers and conditions. Property and health care powers are separate; authority and duties depend on the document and law.

Principal. The person who grants authority to an agent under a power of attorney.

Private-pay bridge. The documented funds and timing plan used to cover care while a benefit, property, insurance, or placement process remains unresolved.

R–V

Representative payee. A person or organization appointed by Social Security to manage Social Security or SSI payments for a beneficiary. A power of attorney alone does not create this role.

Resource assessment / snapshot. A date-specific inventory and valuation of a couple's resources used in the spousal-impoverishment process. It is distinct from the application date.

Reverse mortgage. A loan secured by home equity that generally becomes due upon specified events. The borrower must continue meeting occupancy and property-charge requirements.

Room and board. Housing and food charges. In Illinois SLP, Medicaid pays approved supportive services, while the resident remains responsible for room and board under current program rules.

Snapshot date. The date on which resources are measured for a specified spousal assessment. Confirm the legally controlling event and evidence rather than choosing the date informally.

Spend-down. A term used in more than one Medicaid context. In asset planning, it means reducing resources through lawful expenditures or other permitted actions—not simply giving money away.

Supportive Living Program (SLP). Illinois' Medicaid waiver program providing specified supportive services in certified residential settings for eligible populations. It is not ordinary assisted living with automatic Medicaid payment.

Transfer. A sale, gift, retitling, added ownership interest, waived right, or other act affecting value or control. Program treatment depends on timing, compensation, purpose, authority, and exceptions.

Uncompensated transfer. A transfer for which the person did not receive fair market value or other recognized compensation, subject to the applicable program's rules and exceptions.

Unreimbursed medical expense. A qualifying expense actually paid and not reimbursed, used under applicable VA or tax rules when evidence and other conditions are satisfied.

VA-accredited representative. A VA-recognized VSO representative, accredited claims agent, or accredited attorney authorized to assist with VA claims within applicable rules.

VA Pension. A needs-based benefit for qualifying wartime Veterans or surviving spouses who satisfy service, status, age or disability, income, net-worth, and other requirements.

FINAL ENDNOTES

These notes will move to the final endnotes section during full-manuscript production. Sources were reviewed in July 2026.

1. Centers for Medicare & Medicaid Services, "Long-Term Care," Medicare.gov and "Skilled Nursing Facility Care," Medicare.gov.
2. Illinois Department on Aging, "Community Care Program".
3. Illinois Department of Public Health, "Assisted Living," including laws and rules.
4. Illinois Department of Healthcare and Family Services, "Illinois Supportive Living Program" and "Resident Fact Sheet".
5. Illinois Department of Healthcare and Family Services, "Supportive Living Program—Eligibility Criteria and Access".
6. Illinois Department on Aging, "Long-Term Care Ombudsman Program".
7. CareScout, "Cost of Care Survey 2025: Ranked Median Costs by State". The survey, conducted July through November 2025 and published in 2026, reports an Illinois assisted-living-community median of $6,219 per month for a private one-bedroom unit. A median is a benchmark, not a quote or guarantee of included services.
8. Illinois General Assembly, 210 ILCS 9/90, "Contents of Service Delivery Contract", and Joint Committee on Administrative Rules, 77 Ill. Adm. Code 295.2030, "Establishment Contracts".
9. Social Security Administration, "Get benefit verification letter".
10. Illinois General Assembly, Illinois Power of Attorney Act, 755 ILCS 45.
11. U.S. Department of Veterans Affairs, "Request your military service records (including DD214)".
12. Social Security Administration, "Life Expectancy Calculator".
13. U.S. Bureau of Labor Statistics, "CPI Inflation Calculator". The CPI-U is a broad consumer-price measure; the illustrative care-cost assumptions in Chapter 5 are not CPI forecasts.
14. Social Security Administration, "What you could get from Survivor benefits".
15. Pension Benefit Guaranty Corporation, "Find unclaimed retirement benefits".
16. Illinois Department of Healthcare and Family Services, "Applying for Medicaid" (including ABE applications for SNAP, cash assistance, and Medicare Savings Programs).
17. Illinois Department on Aging, "Senior Health Insurance Program".
18. Internal Revenue Service, Topic No. 423, "Social Security and Equivalent Railroad Retirement Benefits".
19. Internal Revenue Service, Topic No. 410, "Pensions and Annuities".

20. Internal Revenue Service, "Retirement Topics—Required Minimum Distributions".
21. Internal Revenue Service, Publication 523, "Selling Your Home" (2025).
22. Internal Revenue Service, Publication 527, "Residential Rental Property" (2025).
23. Consumer Financial Protection Bureau, "What is a home equity loan?"
24. Consumer Financial Protection Bureau, "What is a home equity line of credit (HELOC)?"
25. U.S. Department of Housing and Urban Development, Home Equity Conversion Mortgage counseling and borrower requirements.
26. Illinois Department of Healthcare and Family Services, "Long Term Services & Information for Couples."
27. Centers for Medicare & Medicaid Services, "Estate Recovery."
28. Illinois Department of Healthcare and Family Services, "Guide to the Medicaid Estate Recovery Program."
29. National Association of Insurance Commissioners, "A Shopper's Guide to Long-Term Care Insurance."
30. National Association of Insurance Commissioners, "Life Insurance."
31. National Association of Insurance Commissioners, "Glossary of Insurance Terms" (Life Settlements).
32. Internal Revenue Service, Publication 926, "Household Employer's Tax Guide" (2026), and "Tax situations when taking care of a family member."
33. Illinois Department of Healthcare and Family Services, "Highlights of New Eligibility Requirements for Long Term Care."
34. Illinois Department of Healthcare and Family Services, "Applying for Medicaid" (including ABE applications for SNAP, cash assistance, and Medicare Savings Programs).
35. U.S. Department of Veterans Affairs, "Aid and Attendance benefits and Housebound allowance."
36. Illinois Department of Healthcare and Family Services, Fiscal Year 2025 Annual Report (HFS as the state Medicaid agency).
37. Illinois Department of Healthcare and Family Services, "After Applying for Medicaid."
38. Illinois Department of Healthcare and Family Services, "Long Term Services & Information for Couples" (application and needs-screening overview; annual figures must be verified separately).
39. Illinois Department of Healthcare and Family Services, "Home and Community Based Services Waiver Programs."
40. Illinois Department of Healthcare and Family Services, "Illinois Persons who are Elderly waiver."

41. Illinois Department of Healthcare and Family Services, "Illinois Supportive Living Program."
42. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 120.380–120.385 (resources, exemptions, disregards, spenddown, and long-term-care factors).
43. Illinois Department of Human Services, "PM 07-02-01: Asset Limits" (current AABD medical standard).
44. Illinois Department of Healthcare and Family Services, HFS 2378H, "Instructions for Mail-In Application for Medical Benefits," including resource and documentation schedules.
45. Illinois Department of Human Services, "PM 02-07-03-n: Asset Verification System."
46. Illinois Department of Healthcare and Family Services, "Guide to the Medicaid Estate Recovery Program."
47. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 120.61 and 120.384 (income/resource spenddown and long-term-care eligibility).
48. Illinois Department of Healthcare and Family Services, "Highlights of New Eligibility Requirements for Long Term Care," and 89 Ill. Adm. Code 120.388 (transfers on or after January 1, 2007, fair market value, look-back, and penalties).
49. Illinois Department of Healthcare and Family Services, HFS 2378H, "Instructions for Mail-In Application for Medical Benefits," Form C—Transfer of Resources and Income.
50. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 120.381 (burial spaces, burial funds, and funeral/burial contracts). Verify current indexed amounts and contract requirements before purchase.
51. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 120.388(f)(3) (family and friend care, preexisting documentary evidence, contemporaneous records, and prevailing rates). See also IRS Publication 926 for possible household-employment tax obligations.
52. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 120.388(a)–(k), "Property Transfers Occurring On or After January 1, 2007" (assets, fair market value, uncompensated value, 60-month look-back, penalty timing, calculation, and multiple transfers).
53. Illinois Department of Healthcare and Family Services, HFS 2378H, "Instructions for Mail-In Application for Medical Benefits," Form C—Transfer of Resources and Income (five-year records and transfer questions).
54. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 120.388(s), "Records Production."

55. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 120.388(f)(3), family and friend care, preexisting documentation, contemporaneous records, and prevailing rates.
56. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 120.388(m), exceptions and rebuttals, including qualifying home transfers, spouse and disability provisions, fair-market-value intent, and transfers exclusively for another reason.
57. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 120.388(m)(6) and (r), returned assets and hardship waivers; subsection (i)(2) references appeal rights under 89 Ill. Adm. Code 102.80.
58. Illinois Department of Healthcare and Family Services, "Long Term Care" and HFS 3191, "Long Term Services and Information for Couples" (medical-benefits application and needs-screening requirements); HFS, "Home and Community Based Services Waiver Programs" (financial and program-specific eligibility).
59. Illinois Department of Healthcare and Family Services, "Applying for Medicaid" and "Manage My Illinois Medicaid" (ABE, telephone applications, case numbers, document upload, case status, and appeals portal).
60. Illinois Department of Healthcare and Family Services, HFS 2378H, "Instructions for Mail-In Application for Medical Benefits" (identity, income, resources, transfers, management of finances, and verification questions).
61. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 102.80–102.83 (appealable actions, filing period, and continuation rules); Illinois Department of Healthcare and Family Services, "Manage My Illinois Medicaid" and "Appeal Request Form" (appeal channels).
62. Centers for Medicare & Medicaid Services, "Estate Recovery"; Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 102.210(b), scope of Illinois estate claims.
63. Illinois Department of Healthcare and Family Services, "Guide to the Medicaid Estate Recovery Program" (notice process, estate examples, first $25,000, survivor protections, hardship, and estate expenses).
64. Illinois Joint Committee on Administrative Rules, 89 Ill. Adm. Code 102.210(a) and (f), estate definition and qualified long-term-care partnership treatment.
65. Illinois Department of Healthcare and Family Services, "Estate Recovery" (liens, claims, protected occupancy, release, and June 2, 2022 restriction on new liens); Public Act 102-1037.

66. Illinois Department of Healthcare and Family Services, “Hardship Waiver,” “Glossary of Terminology,” and Guide to the Medicaid Estate Recovery Program; 89 Ill. Adm. Code 102.210(g)–(h).
67. Illinois Department of Human Services, PM 07-02-22, “Community Spouse Resource Allowance,” covered long-term-care pathways and community-spouse status.
68. Illinois Department of Human Services, PM 15-04-04, “Use of Income (NH, SLF, and DoA HCBS Waiver),” duty to report income and resources of either or both spouses.
69. Illinois Department of Human Services, MR #26.05 and PM 07-02-22, 2026 CSRA and circumstances permitting a higher allowance by court order or appeal.
70. Illinois Department of Human Services, WAG 07-02-22, HFS 3190 determination notice, protected ownership transfer, and first-redetermination verification.
71. Illinois Department of Human Services, PM and WAG 15-04-04-a, “Community Spouse Maintenance Needs Allowance,” gross-income subtraction, actual contribution, SSI limitation, court orders, and fair-hearing decisions.
72. Illinois Department of Human Services, MR #26.05, “Community Spouse Maintenance Needs Allowance and Community Spouse Resource Allowance Limit Increases,” effective January 1, 2026 and annual adjustment statement; Illinois HFS Provider Notice dated January 22, 2026.
73. 42 U.S.C. § 1396r-5(c)(1), federal computation and requested assessment of resources at the beginning of the first continuous period of institutionalization, documentation, copies, retention, notice, and possible assessment fee.
74. 42 U.S.C. § 1396r-5(c)(2)-(4), (e), and (f), attribution at initial eligibility, separate treatment after eligibility, notice and fair-hearing rights, and permitted transfer to or for the sole benefit of the community spouse.
75. Illinois Department of Human Services, PM 07-02-22, “Community Spouse Resource Allowance,” covered pathways, total nonexempt resources, current CSRA, post-determination resources, disclosure duty, notices, and appeal exceptions.
76. Illinois Department of Human Services, WAG 07-02-22, HFS 3190 decision attachment, application-based transfer period, ownership verification, and current worked examples.
77. Illinois Department of Human Services, PM 07-02-04-a, “Homestead Property,” occupancy-based exemption, persons whose residence preserves the exemption, excess-equity exception, and valuation guidance.

78. Illinois Department of Human Services, WAG 25-03-02(2), 2026 CSRA, CSMNA, AABD Medical resource standard, and home-equity limit.

79. Illinois Department of Human Services, PM 07-02-22, “Community Spouse Resource Allowance,” covered pathways, total nonexempt resources, current maximum, ownership transfer, later-acquired resources, and appeal exceptions.

80. Illinois Department of Human Services, PM 07-02-20-b, “Allowable Transfers,” transfers between spouses, limits imposed by the CSRA, and allowable transfers of specified property.

81. Illinois Department of Human Services, PM 15-04-04-a, “Community Spouse Maintenance Needs Allowance,” 2026 maximum, gross-income subtraction, available non-SSI income, actual contribution, court orders, and fair-hearing results.

82. U.S. Department of Veterans Affairs, “Veterans Pension Eligibility,” character-of-discharge, wartime-service, income and net-worth, age, disability, nursing-home, SSDI, and SSI requirements.

83. U.S. Department of Veterans Affairs, “How Are Pension Benefits and Disability Compensation Different?” needs-based pension, service-connected compensation, and the rule against simultaneous receipt.

84. U.S. Department of Veterans Affairs, “VA Aid and Attendance Benefits and Housebound Allowance,” pension prerequisite, functional criteria, non-duplication, and Forms 21-2680 and 21-0779.

85. U.S. Department of Veterans Affairs, “Current Veterans Pension Rates,” MAPR formula, income for VA purposes, unreimbursed medical expenses, net-worth treatment, annual adjustment, and 36-month transfer review.

86. U.S. Department of Veterans Affairs, “Survivors Pension,” claimant, relationship, wartime-service, financial requirements, application form, and intent-to-file information.

87. U.S. Department of Veterans Affairs, “Get Help from an Accredited Representative or VSO,” accredited representatives, free VSO representation, and permitted fee distinctions.

88. U.S. Department of Veterans Affairs, “Eligibility for Veterans Pension,” current service-length, wartime-period, discharge, age, disability, nursing-home, SSDI, and SSI requirements.

89. U.S. Department of Veterans Affairs, “Current Pension Rates for Veterans,” MAPR formula and amounts effective December 1, 2025, five-percent medical-expense thresholds, net-worth limit effective December 1, 2025 through November 30, 2026, asset treatment, and transfer review.

90. U.S. Department of Veterans Affairs, “Aid and Attendance Benefits and Housebound Allowance,” pension prerequisite, functional criteria, non-duplication, and Forms 21-2680 and 21-0779.

91. 38 C.F.R. § 3.272(g), unreimbursed medical-expense exclusion, payment and household requirements, five-percent threshold, annualization period, and reasonable estimates.

92. 38 C.F.R. § 3.278, deductible medical expenses, ADLs, IADLs, custodial care, providers, insurance, transportation, institutional care, in-home care, and care-facility conditions.

93. U.S. Department of Veterans Affairs, VA Form 21P-8416, “Medical Expense Report,” current form purpose and unreimbursed-expense requirement.

94. U.S. Department of Veterans Affairs, “How to Apply for a Discharge Upgrade,” and Veterans Pension eligibility guidance on VA Character of Discharge review.

95. U.S. Department of Veterans Affairs, “How Are Pension Benefits and Disability Compensation Different?” needs-based Pension, service-connected compensation, and non-simultaneous payment.

96. 38 C.F.R. § 3.275, asset definition and exclusions for a primary residence and personal effects in VA Pension net-worth determinations.

97. U.S. Department of Veterans Affairs, “How to Apply for a VA Pension as a Veteran,” required information, Form 21P-527EZ, filing methods, automatic online intent, and accredited help; and the current Form 21P-527EZ page, related Form 21P-0969, and supporting forms.

98. U.S. Department of Veterans Affairs, “Your Intent to File a VA Claim” and VA Form 21-0966 guidance, potential effective date, one-year filing period, one active intent, separate benefit types, automatic online intent, and authorized-signature requirements.

99. U.S. Department of Veterans Affairs, “Aid and Attendance Benefits and Housebound Allowance,” clinician-completed Form 21-2680 and nursing-home Form 21-0779.

100. U.S. Department of Veterans Affairs, VA Form 21P-8416, “Medical Expense Report,” paid household medical or dental expenses and non-reimbursement requirements.

101. U.S. Department of Veterans Affairs, “Get Help from a VA Accredited Representative or VSO,” representative types, accreditation search, free VSO service, appointments, and Forms 21-22 and 21-22a.

102. Veterans Benefits Administration, “Fraud Prevention,” pension-poaching warnings, asset maneuvers, credentials, passwords, blank forms, direct deposit, and personal-information protections.

103. Federal Trade Commission, “Veterans Pension and Disability Benefits” and 2025 consumer alert, free initial claim assistance, accreditation verification, pressure tactics, benefit assignments, and product-linked asset schemes.

104. U.S. Department of Veterans Affairs, current Veterans Pension rate and eligibility guidance, net-worth calculation, applicable deductible expenses, and the 3-year asset-transfer review and penalty framework.

105. Illinois Department of Healthcare and Family Services, “Long Term Care,” application and needs-screening roles; and 89 Ill. Adm. Code 120.61, long-term-care eligibility and post-eligibility treatment of income.

106. 89 Ill. Adm. Code 120.388, property transfers occurring on or after January 1, 2007, including the 60-month review, covered long-term-care services, uncompensated value, penalties, and exceptions.

107. U.S. Department of Veterans Affairs, VA Form 21P-8416 guidance, paid unreimbursed medical expenses, documentation retention, and facility or in-home-care support.

108. 89 Ill. Adm. Code 120.10 and 120.61, Medicaid financial eligibility, spenddown, and application of income toward long-term-care costs after allowed deductions.

109. Illinois Department of Healthcare and Family Services, “Supportive Living Program,” eligible populations, eligibility criteria, access, service menu, managed-care coordination, and restriction on concurrent federal HCBS waiver participation.

110. 89 Ill. Adm. Code Part 146, especially Sections 146.200, 146.205, 146.220, 146.225, and 146.230, program description, definitions, resident participation, reimbursement, room and board, provider capacity, and covered services.

111. Illinois Department of Healthcare and Family Services, “Supportive Living Program Resident Fact Sheet,” services, resident cost-of-care liability, room and board, group care credit, personal-needs allowance, rates, and current operational cautions.

112. Illinois Department of Healthcare and Family Services, current SLP provider notices concerning annual SSI-related room-and-board and personal-needs updates; figures must be verified for the applicable month.

113. Illinois Department of Healthcare and Family Services, “Locate a SLP Provider,” current operational-site and county listings, including population and dementia-care-setting identifiers (reviewed July 2026).

114. 89 Ill. Adm. Code Part 146, Subpart E, especially Sections 146.600, 146.610, 146.630, 146.640, and 146.660, dementia care setting population, design, services, welfare checks, staffing, and training.

115. 89 Ill. Adm. Code 146.235 and 146.660, general SLP staffing, 24-hour response staff, licensed and certified staffing, and additional dementia care unit staffing requirements.

116. 89 Ill. Adm. Code 146.240 and 146.255, resident-contract termination terms, involuntary-discharge grounds, notice, safe-discharge planning, and appeal information.

117. Illinois Department of Healthcare and Family Services, “Supportive Living Program,” program structure, resident responsibility for room and board, and covered supportive services; 89 Ill. Adm. Code Parts 120 and 146.

118. Illinois Department of Healthcare and Family Services, Provider Notice dated December 4, 2025, “Supportive Living Program Resident Private Pay to Medicaid Conversion Screenings,” required conversion screening and payment-start limitation.

119. Illinois Department of Healthcare and Family Services, Provider Notice dated May 8, 2026, “Supportive Living Program Medicaid Admission Requirements,” prohibition on mandatory private-pay periods before Medicaid application and reconciliation of excess private payments.

120. Illinois Department of Healthcare and Family Services, “Supportive Living Program Resident Fact Sheet,” 2026 room-and-board notice, 2026 COLA notice, and current resident cost-of-care calculation, minimum income, personal-needs allowance, and group care credit guidance (reviewed July 2026).

121. Illinois Department of Healthcare and Family Services, “Supportive Living Program Resident Fact Sheet,” 2026 room-and-board and COLA notices, resident income floor, personal-needs allowance, and group care credit calculation (figures reviewed July 2026).

122. 89 Ill. Adm. Code 120.380–120.385, available resources, resource disregard, homestead equity and occupancy considerations, and long-term-care financial eligibility.

123. 89 Ill. Adm. Code 120.388, transfers on or after January 1, 2007, 60-month review, fair-market-value requirements, allowable transfers, returned assets, and penalty rules.

124. 89 Ill. Adm. Code 120.61 and 120.379, Supportive Living Facilities within long-term-care income rules, post-eligibility deductions, spousal income attribution, resource assessment, CSRA, community-spouse income allowance, and fair-hearing rights.

125. Centers for Medicare & Medicaid Services, “Your Discharge Planning Checklist,” Publication 11376, information patients and caregivers should obtain before discharge; and Medicare.gov, long-term-care coverage limitations.

126. Illinois Department of Healthcare and Family Services, “Applying for Medicaid,” online, telephone, in-person, and paper routes; ABE tracking number and document-preparation guidance (reviewed July 2026).

127. Illinois Department on Aging, “Senior HelpLine,” statewide information, local service referrals, care coordination, legal services, transportation, nutrition, and caregiver resources (reviewed July 2026).

128. 755 ILCS 45/2-7 and 755 ILCS 45/3-3, Illinois agent duties, principal expectations and best interests, records of receipts, disbursements, and significant actions, and the statutory property power-of-attorney notice.

129. 755 ILCS 45/4-7 and Illinois Power of Attorney for Health Care provisions, notice to providers, copies in the medical record, and consultation with an available health care agent when authority applies.

130. 755 ILCS 40/20 and 755 ILCS 40/25, Illinois Health Care Surrogate Act, presumption of decisional capacity absent actual notice to the contrary, decision-making process, and inquiry concerning available agents or surrogates.

131. Consumer Financial Protection Bureau, “Managing Someone Else’s Money” and “What Is a Fiduciary?” duties to act in the person’s interest, manage carefully, keep property separate, maintain records, and recognize exploitation warning signs.

132. Illinois Department on Aging, “Report Abuse” and Adult Protective Services materials, eligible populations, anonymous and good-faith reporting, confidentiality, immediate-danger distinction, and the statewide 24-hour hotline (reviewed July 2026).

133. Illinois Department on Aging, Long-Term Care Ombudsman Program and facility-reporting guidance, resident-directed advocacy, confidentiality, complaints, regulator routes, and emergency-response limitation (reviewed July 2026).

134. Medicare.gov, “Advance Care Planning” and “Long-Term Care Coverage,” advance directives, health care proxy, living will, annual wellness planning, and Medicare’s general exclusion of most long-term custodial care.

135. U.S. Department of Veterans Affairs, “Accredited Representatives,” VA-recognized VSO, agent, and attorney roles, accreditation search, appointment forms, and fee rules (reviewed July 2026).